

Patient Face Sheet

TRANSCEND HOSPICE 18 LLC

PATIENT INFORMATION	
Patient Name: KATHERINE A HOWRIGAN	MR #: 101996
Admission Date: 10/14/2025	Admission #: 1 Current Status: Transferred in from Another Hospice
Primary Diagnosis (ICD10): G30.1 (Alzheimer's disease with late onset) Related Conditions (ICD10): R52 (Pain, unspecified), R60.9 (Edema, unspecified), R19.7 (Diarrhea, unspecified), F33.9 (Major depressive disorder, recurrent, unspecified), I10 (Essential (primary) hypertension), R11.0 (Nausea), R11.10 (Vomiting, unspecified), R06.03 (Acute respiratory distress), K59.00 (Constipation, unspecified), F41.9 (Anxiety disorder, unspecified) Unrelated Conditions (ICD10):	
Level of Care: Routine Home Care	Disaster Code: Disaster Code 2
General Information:	
Birth Date: 6/30/1934	SSN: 048-26-0963
Race: White	Gender: Female
Ethnicity: Not of Hispanic, Latino/a, or Spanish origin	
Marital Status: Single	Is DNR?: Yes
Other Information: Veteran	Primary Language: English
Allergies: NKA	Religion: Catholic
Funeral Home: Address:	
Patient Contact Information:	
Mobile: (817) 903-2377	
Referral Information:	
Referral Source:	Called In By: Mona Jones
LOCATION INFORMATION	
Current Location:	
Location Type: Hillside Assisted Living	
Institution Name (facility patients):	
Address: 553 Big Bend Dr, Keller TX 76248-2525	
CAREGIVER INFORMATION	
Primary Caregiver	
Caregiver Name: Therese Cox	Relationship to Patient: Daughter
Address: 5317 Hibbs Dr, Fort Worth TX 76137-4901	
Mobile: (817) 689-9918	
Secondary Caregiver	
Caregiver Name: Rose	Relationship to Patient: Other (non-family member)
Address:	
Mobile: (817) 903-2377	
PHYSICIAN INFORMATION	
Attending Physician Name:	NPI #:
Address:	
Hospice Physician Name: RESTREPO, DIEGO	NPI #: 1063664076
FINANCIAL INFORMATION	
Payer Name: Palmetto GBA	
Coverage ID: 1X83VK6VY96	Coverage Type: Medicare Part A
Service(s): Hospice (Primary)	

Medication List

HOWRIGAN, KATHERINE (MR# 101996)

Generated On: 1/2/2026 1:16 PM

Allergies: NKA

Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
Regular Medications:									
10/15/2025		ABH GEL	1/25/1 mg/ml	Apply to inner wrist Syringe	topical	Every 4 hours as needed for anxiety	anxiety	Yes	Active
10/15/2025		acetaminophen 32 MG/ML Oral Suspension	160 mg/5ml	Take 30ml Oral Suspension	Oral Liquid	Every 4 hours as needed for pain	Moderate pain	Yes	Active
10/15/2025		ammonium lactate 120 MG/ML Topical Cream	12%	Apply to skin Topical Cream	Topical	After shower daily	Dry skin	Yes	Active
10/15/2025		ascorbic acid 100 MG / biotin 0.15 MG / calcium pantothenate 5 MG / folic acid 1 MG / niacin 20 MG / pyridoxine 10 MG / riboflavin 1.7 MG / thiamine mononitrate 1.5 MG / vitamin B12 0.006 MG Oral Capsule	100-0.15-5-1-20-10-1.7-1.5-0.006 mg	Take 1 capsule Oral Capsule	po	Daily	vitamin	No	Active
10/15/2025		calcium ascorbate 500 MG Oral Tablet	500 mg	Take 1 tablet Oral Tablet	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cholecalciferol 0.025 MG Oral Capsule	0.025 mg	Take 1 tablet Oral Capsule	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cobamamide 0.1 MG / vitamin B12 5 MG Sublingual Tablet	0.1-5 mg	Take 1 tablet Sublingual Tablet	Sublingual	Daily	Vitamin b12 deficiency	No	Active
10/15/2025		diclofenac sodium 0.01 MG/MG Topical Gel [Voltaren]	1%	Apply to affected area Topical Gel	topical	Every 12 hours	mild pain	Yes	Active
10/15/2025		dorzolamide 20 MG/ML / timolol 5 MG/ML Ophthalmic Solution [Cosopt]	2-0.5%	1 drop each eye Ophthalmic Solution	Ophthalmic	Twice daily	Dry eye	Yes	Active
10/15/2025		levothyroxine sodium 0.075 MG Oral Tablet	0.075 mg	Take 1 tablet Oral Tablet	Oral Pill	Before breakfast	Hypothyroidism	No	Active
10/15/2025		loratadine 10 MG Oral Tablet [Claritin]	10 mg	Take 1 tab;et Oral Tablet	po	Daily	antihistamine	Yes	Active
10/17/2025	10/24/2025	sulfamethoxazole 800 MG / trimethoprim 160 MG Oral Tablet [Bactrim]	800-160 mg	Take 1 tablet Oral Tablet	po	Twice daily	infection	Yes	Discontinued
Comfort Kit Medications:									
10/15/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/15/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as	for excess secretions -	Yes	Active

						needed	#15mL / RF: 11		
10/15/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/15/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/15/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/15/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/15/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/15/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

INITIAL COMPREHENSIVE ASSESSMENT (WITH HOPE)

COMPREHENSIVE ASSESSMENT

Date/Time In:	10/15/2025 1:20:00 PM
Date/Time Out:	10/15/2025 3:30:00 PM
Modality:	In-person visit
Describe onset of illness, progression of symptoms, functional status, response to treatments, etc: including any recent hospitalizations, ER visits, weight loss and infections:	Hospice Admission Skilled Nursing Note
	Patient Name: Katherine Howrigan
	Age: 91
	Sex/Race: Caucasian Female
	Admission Date: 10/15/2025
	Primary Diagnosis: Senile Degeneration of the Brain
	Comorbidities: Dementia, Osteoporosis, Osteoarthritis, Hypothyroidism, Essential Tremors, Glaucoma, Alopecia
	Level of Care: Routine Hospice Care
	Advance Directives: OOHDNR
	POA/Decision Maker: Daughter
	Allergies: NKDA
	FAST: 6e
	Scales:
	PPS: 30%
	KPS: 30%
	LMAC: 18.5 cm
	Subjective
	SN admitted the patient to hospice services at her group home. The patient's daughter provided the patient's medical history and current health status. She expressed the family's wish to focus on comfort care and to avoid further hospitalizations.
	Reports indicate that over the past three months, Ms. Howrigan has exhibited progressive cognitive, functional, and physical decline, including increased weakness, poor endurance, and decreased nutritional intake.
	The daughter reports the patient sleeps approximately 16–18 hours per day with frequent daytime napping. Appetite remains poor—she consumes about 25% of her meals, consisting of small portions that she rarely finishes—no reports of nausea or vomiting.
	Objective
	General: Elderly female, frail and cachectic in appearance, seated in her wheelchair on the porch at the time of assessment. Appears calm, no acute distress noted.
	Neurological: Alert and oriented ×1. Follows simple commands inconsistently. Speech is clear but limited. Displays mild essential tremors.
	Functional Status:
	ADLs: Requires maximum assistance with 5/6 ADLs (previously 4/6).
	Feeding: Self-feeds with utensils but requires prompting to do so.
	Continence: Incontinent of bowel and bladder.
	Nutrition/Weight: Appetite poor; intake 25% of three small meals per day. Evidence of weight loss and generalized muscle wasting was observed.

Skin/Wounds: Skin clean, dry, and intact (CDI). No open areas or pressure injuries noted.

Falls: One fall reported in the past six months; no injury sustained.

Cardiopulmonary: LSCTA; respirations even and unlabored on room air; diminished breath sounds in the lower lobes—occasional shortness of breath with exertion.

GI/GU: Abdomen soft and non-tender; bowel sounds active ×4 quadrants—incontinent of both bowel and bladder.

Musculoskeletal: Limited mobility; primarily wheelchair-bound; unable to ambulate independently.

Pain: Denies pain at the time of the visit. Tylenol is available and used PRN for mild discomfort.

Mood/Behavior: Cooperative and calm; no anxiety or agitation observed. The patient refused blood pressure measurement, which the daughter states is typical behavior. SN will notify the primary RN to obtain a wrist BP cuff for future visits.

Assessment

91-year-old Caucasian female with end-stage senile degeneration of the brain, compounded by multiple comorbidities. The patient exhibits severe cognitive and functional decline, poor nutritional intake, weight loss, and increased dependence on ADLs. Prognosis poor.

Plan

Hospice Admission: The patient was admitted at the routine level of care for comfort management and symptom control.

Pain Management: Continue Tylenol PRN for mild discomfort; reassess as needed.

Wound Care: None required at present; monitor for pressure areas due to immobility.

Nutrition: Encourage small, frequent meals and nutritional supplements as tolerated.

Fall Precautions: Hospital bed, briefs, chux, and gloves provided. Reinforce safety during transfers and repositioning.

Respiratory Care: Monitor for dyspnea; provide comfort measures and oxygen PRN if indicated.

Incontinence Care: Continue using briefs to maintain skin integrity and hygiene.

Psychosocial Support: Chaplain and Social Work services to be initiated for patient and family support.

Education: Family educated on comfort and safety measures, disease progression, symptom management, DME, hospice philosophy, skin care, comfort kit use, medication regimen, and fall precautions.

Follow-up: SN reviewed the findings with the IDT and Dr. Restrepo. Patient deemed hospice appropriate. Initial POC reviewed with the patient's daughter and MPOA—all in agreement.

? Scales upon Admission: PPS/KPS 30 | LMAC 18.5 cm | DNR | Allergies: NKDA

CO-MORBIDITIES

What other diagnoses does patient have that may contribute to a decreased life expectancy when combined with the primary diagnosis?

- ☒ Generalized Muscle Weakness
- ☒ Malnutrition

GENERAL INFORMATION

Race: Caucasian
Religion: Catholic
Primary Language: English
Does the patient or primary caregiver need an interpreter? No
Is the patient or the patients spouse a veteran? Yes
Did they serve during war time? Yes
Receiving VA benefits? Yes

PSYCHOSOCIAL / SPIRITUAL / BEREAVEMENT ASSESSMENT

Patient/Responsible Party's Current Response to Diagnosis/Prognosis: (Check all that apply)

• Denial

- ☒ Patient
- ☒ Representative

• Anger

- ☒ Patient
- ☒ Representative

• Bargaining

- ☒ Patient
- ☒ Representative

• Acceptance

- ☒ Patient
- ☒ Representative

• Spiritual Support

Needs spiritual support? Yes

• Psychosocial Support

Needs psychosocial support? Yes

• Patient/Family Goals

- ☒ Patient and family will receive physical, spiritual, and emotional support

PAIN/COMFORT ASSESSMENT

Has patient experienced pain the past 7 days? No
Pain Assessment Scale utilized: FLACC
Acceptable level of pain: 3
Best: 3
Worst: 8

☒ Generalized pain/undefined location

Pain Frequency: Intermittent
Pain Duration: Less than an hour

Non-Verbal Signs of Pain

☒ N/A

What makes the pain better?

☒ Medication

What causes pain or makes it worse?

☒ Not taking pain medication

What other symptoms are associated with the pain?

☒ Insomnia

Current treatments for pain:

☒ NSAIDS

Patient is on a scheduled opioid, currently taking a PRN opioid or has been instructed to begin taking an opioid:	No
How much breakthrough pain medication has patient taken in the past 24 hours?	0
Is current pain regime effective?	Yes
Patient/family pain management goals	Relieve pain/discomfort as much as possible even if it will cause increased sedation
Patient/caregiver educated on pain regime and side effects of pain medications:	Yes, and verbalized understanding
Patient educated on non-medication treatments of pain?	No
Non-medication treatments were initiated:	No

HEAD TO TOE ASSESSMENT

Vital Signs

Type of temperature taken:	Temporal:
Temporal temperature: 99.9	
Respiratory rate (respirations per minute): 16	
Respiratory pattern:	Regular
SaO2	95
Pulse Type:	Radial
Heart rate: 95	
Arm circumference taken:	Right
Arm circumference: 18.5	

Nutrition

☒ Diet Orders (specify):

☒ General (as tolerated)

Average percentage of meals consumed: 25
Patient Feeding: Patient able to feed self independently
☒ Evidence of Dehydration
☒ Skin dry

- ☒ Evidence of Cachexia
 - ☒ Poor appetite/anorexia
 - ☒ Clavicular/orbital wasting

Neurological Status

Orientation

- ☒ Self

Behavior

- ☒ Calm
- ☒ Confused

Energy

- ☒ Fatigue/Weakness

Number of hours spent sleeping in a day: 16

Responsiveness

Responsiveness: Verbal stimuli

Intellect

Intellect: Dementia/Alzheimer's

Speech

- ☒ Slurred
- ☒ Nonsensical

Neuromusculoskeletal Status

- ☒ Shuffling gait/uncoordinated gait

Muscle Tone

- ☒ Muscle wasting

Functional Assessment

Functional Assessment Score: 6E

Gastrointestinal Status

Last BM Yesterday

Bowel Sounds positive in all 4 Quadrants

- ☒ Normal
 - ☒ RUQ
 - ☒ RLQ
 - ☒ LUQ
 - ☒ LLQ

- ☒ Incontinence

Genitourinary Status

☒ Urinary incontinence

Integumentary Status

☒ Skin intact

Pressure Ulcer Risk Assessment

Sensory perception: ability to respond meaningfully to pressure-related discomfort: 1
Moisture: degree to which skin is exposed to moisture: 2
Activity: degree of physical activity: 3
Mobility: ability to change and control body position: 2
Nutrition: usual food intake pattern: 2
Friction and shear: 2
Total score: 12

CARDIOPULMONARY ASSESSMENT

Was patient screened for shortness of breath 1. Yes
Date of first screening for shortness of breath 10/17/2025
Did screening indicate the patient had shortness of breath 0. No

☒ Respiratory Status WNL

☒ Decreased

☒ RLL

☒ LLL

FUNCTIONAL STATUS

Walking/Locomotion Wheelchair
Type: Standard
Bed Mobility; Independent
Transfers: Assist of 1
Dressing: Dependent
Eating: Independent
Toilet use: Dependent
Personal hygiene: Dependent
Bathing Dependent
Palliative performance scale (PPS): 30% - Totally Bedbound

Fall Risk Assessment (NHPCO Guidelines)

• History of a fall within the last 6 months

☒ 0: No falls

• Mobility

☒ 3: Confined to bed or chair

• Elimination

☒ 3: Incontinent all or most of the time

• Mental Status

☒ 4: Confused at all times

• Medical conditions which could impair the ability to ambulate

- ☒ 1: Chronic debilitating illness
- ☒ 1: Weakness or paralysis of the extremities
- ☒ 1: Dementia

Total: 13

☒ 10 or more points = High risk

HOME SAFETY ASSESSMENT

☒ Patient lives in a facility that is deemed safe by the State

ASSESSMENT OF PATIENT/CAREGIVER TO SAFELY ADMINISTER MEDICATIONS

☒ N/A (Medication administered by facility staff)

HOPE Admission Assessment

Based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?	0. No
Living Arrangements	3. Congregate home (alf, residential home care)
Availability of Assistance	1. Around-the-clock (24 hours a day with few exceptions)
Interpreter requested	0. No

Preferences

- Was patient asked about CPR 1. Yes
- Date asked about CPR 10/15/2025
- Was patient asked about treatments other than CPR 1. Yes
- Date asked about treatment other than CPR 10/15/2025
- Was patient asked about hospitalization 1. Yes
- Date asked about hospitalization 10/15/2025
- Was patient asked spiritual/existential concerns 1. Yes
- Date asked about spiritual/existential concerns 10/15/2025

Active Diagnoses

Principal Diagnosis 02. Dementia (including Alzheimer's disease)

Comorbidities and Co-existing Conditions

Pain

- Was patient screened for pain 1. Yes
- Date of first screening for pain 10/15/2025
- Patient's pain severity was 1. Mild
- Type of standardized pain tool used 3. Patient visual
- Is pain an active problem for the patient? 0. No

Symptom Impact

- Was a symptom impact screening completed? 1. Yes
- Date of symptom impact screening 10/15/2025

Pain	1. Slight
Shortness of breath	0. Not at all
Anxiety	1. Slight
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	0. Not at all
Agitation	1. Slight

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

- Was scheduled opioid initiated or continued? 0. No
- Was PRN opioid initiated or continued? 0. No
- Was bowel regimen initiated or continued? 0. No

TEACHING/EDUCATION PROVIDED

Education Provided To:

- ☒ Caregiver

Topics

- ☒ Palliative care, hospice philosophy, hospice benefit
- ☒ Scope of services, role of team members
- ☒ Call us First Program
- ☒ Diagnosis
- ☒ Prognosis
- ☒ Disease process
- ☒ Safe & effective use of equipment & supplies
- ☒ Dying process, signs & symptoms of approaching death
- ☒ Covered medications, supplies, equipment
- ☒ How to re-order medications and supplies
- ☒ Medication usage, side effects, drug interactions
- ☒ Basic home safety, emergency preparedness
- ☒ Resides in a facility. Follow facility plan
- ☒ Infection control, disposal of hazardous waste, universal precautions
- ☒ Skin care, wound care, dressing changes
- ☒ Energy Conservation techniques
- ☒ Plan of care established with primary care giver
- ☒ Symptom management
- ☒ Catheter care
- ☒ Bowel regimen
- ☒ Pain management
- ☒ What to do at the time of death

- ☒ Anticipatory grief, grief, bereavement
- ☒ Funeral planning
- ☒ Advance directives
- ☒ Community resources
- ☒ Signs & symptoms of infection
- ☒ Discharge planning

COMMUNICATION WITH OTHER TEAM MEMBERS

- ☒ Hospice Medical Director
- ☒ Clinical Director/PCC
- ☒ Assigned RN Case Manager
- ☒ Assigned LPN
- ☒ Social Worker
- ☒ Volunteer Coordinator
- ☒ Bereavement Coordinator
- ☒ Spiritual Counselor

Date: 10/15/2025

Electronically signed by: VIRGINIA ALLAN RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

PSYCHOSOCIAL ASSESSMENT

PSYCHOSOCIAL ASSESSMENT

Date/Time in: 10/16/2025 3:00:00 PM
Date/Time out: 10/16/2025 3:59:00 PM
Others present during the visit caregiver

Pain

Pain Rating: 2
Pain scale used: Verbal Scale

Advance Directives

☒ Do not resuscitate

Communication and Comments

Comments related to plan of care and overall impressions: Understanding Goals
Due to advanced Alzheimer's disease, Ms. Howrigan is unable to demonstrate meaningful understanding of her diagnosis or prognosis. She is aware of her surroundings intermittently but is unable to articulate insight into her medical condition. Family members demonstrate a clear understanding of the progressive and terminal nature of Alzheimer's disease and express concern regarding cognitive decline, safety, and maintaining comfort. End-of-life goals focus on comfort-oriented care, minimizing agitation and distress, preserving dignity, and allowing the patient to remain in a familiar and supportive assisted living environment for the remainder of her life.

Hospice, Death, and Dying

The patient is unable to meaningfully participate in discussions regarding hospice, death, or dying due to cognitive impairment. Family members verbalize a solid understanding of hospice philosophy, including comfort-focused care, symptom management, and emotional support. They understand that hospice services aim to manage symptoms such as pain, anxiety, agitation, and functional decline rather than prolong life. While the family acknowledges that the disease will continue to progress, discussions regarding the dying process remain emotionally challenging. Continued education and anticipatory guidance are recommended.

Pain

Pain Rating: Unable to self-report due to cognitive impairment
Pain Scale Used: PAINAD observational scale
Location/Description: No nonverbal indicators of pain observed during assessment. Patient appeared calm, with relaxed posture and no grimacing, guarding, or vocalizations noted.
Nurse Notified: Yes – nurse notified; no signs of pain observed at time of visit.

Advance Directives

Power of Attorney: Family reports power of attorney established with adult child; documentation not available at time of assessment.

Living Will: Family reports advance directives were completed prior to cognitive decline; specific details not fully known.

Do Not Resuscitate: Not in place; family continues to discuss resuscitation preferences.

Organ/Body Donor: Not discussed.

State Guardianship: Not applicable.

Surrogate: Adult child serves as primary surrogate decision-maker.

Funeral Plans

No funeral or final arrangements have been made. Family reports emotional difficulty addressing planning at this time. Education and support offered as appropriate.

Funeral Home Name: N/A

Address: N/A

Social / Cultural

Ms. Howrigan is a 91-year-old female with a long-established family and community history. She completed high school and was primarily a homemaker, later engaging in part-time work. No military service reported. Prior to cognitive decline, she enjoyed reading, attending church services, and spending time with family. She has experienced multiple losses, including the death of her spouse and peers. No immigration history reported. Cultural values emphasize family involvement, faith, and respect for elder care.

Living Situation / Support System

The patient resides in an assisted living facility where staff provide 24-hour supervision, assistance with activities of daily living, medication administration, and safety monitoring. She appears comfortable in this setting and benefits from consistent routines and familiar caregivers. Family visits regularly and remains involved in care decisions. Hospice staff provide additional medical, emotional, and psychosocial support.

Family Dynamics / Impact of Loss

Family dynamics appear supportive and involved. Family members express sadness and anticipatory grief related to the patient's decline. The patient's death is expected to have a significant emotional impact on family members, particularly adult children who have been actively involved throughout the illness. Continued emotional support and grief counseling are recommended.

Psychosocial

At the time of assessment, Ms. Howrigan presents with advanced cognitive impairment. She is oriented to person inconsistently and is disoriented to place and time. Verbal communication is limited. Mood appears generally calm with occasional periods of restlessness reported by staff. Affect is appropriate. The patient relies heavily on environmental cues, routine, and caregiver reassurance for emotional stability.

Family members demonstrate appropriate coping but verbalize emotional strain, sadness, and fatigue associated with prolonged caregiving and decision-making responsibilities. Anticipatory grief is evident. The family is receptive to hospice involvement, education, and psychosocial support. Continued monitoring for caregiver stress and emotional needs is recommended.

Financial

The patient's financial needs are managed through family resources and benefits. No financial concerns identified at this time. Assisted living costs are currently being met.

Concrete Needs / Referrals

Spiritual Counselor/Caregiver: Offered; family open to future visits

Family Counseling: Recommended for anticipatory grief and emotional support

Community Social Services: Not requested at this time

Meals on Wheels: Not applicable due to assisted living placement

Lifeline: Not applicable due to 24-hour supervision

Transportation: Provided by family or facility

Funeral Planning/Final Arrangements: Education offered; family not ready to proceed

Communication and Comments

Communication with Team Members:

Psychosocial assessment findings documented and communicated to the interdisciplinary team via charting.

Assisted living staff updated regarding hospice goals, symptom monitoring, and psychosocial observations.

Comments Related to Plan of Care:

Plan to continue psychosocial support focused on maintaining comfort, routine, and emotional reassurance for the

patient while supporting family and facility staff through education and anticipatory grief counseling. Social work to remain involved for ongoing assessment, support, and coordination of care.

Date: 10/16/2025

Electronically signed by: ASHLEY DAVIS LMSW

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

SPIRITUAL ASSESSMENT

SPIRITUAL ASSESSMENT

Date/Time in: 10/20/2025 12:20:00 PM
Date/Time out: 10/20/2025 1:20:00 PM
Location/address: Hillside Assisted Living
Name/relationship of person providing information: Therese Cox (Daughter)

Response to Terminal Prognosis

- *Acceptance*
 - ☒ Patient
 - ☒ Representative

Pain Assessment

Pain rating: 0
Pain scale used: Non-Verbal Scale
Location/description of pain: The patient showed no signs of pain during this assessment visit.

Religious Community

- *Identifies with a faith tradition*
 - ☒ Patient
 - Specify:** The patient identifies as a Catholic although she is not able to attend mass due to her loss of independence.

Comments: This chaplain was able to complete the spiritual assessment today with Katherine at her Assisted living home in Keller Texas. Upon arrival the patient was sitting in her wheel chair finishing lunch and drinking a boost. I was able to speak with the patients daughter prior to this assessment visit and she was able to render the spiritual and life history of the patient to this chaplain due to the patients inability to communicate. The patient suffers from Alzheimer's and is not able to communicate well due to its advanced stage. The patient appears well dressed and groomed. I was able to encourage the patient to drink most of her boost but she was not interested in her lunch food. The patient grew and and raised her family in Texas. She has been married and has two children, a son and a daughter. She worked with the Navy as a flite attendant and later as an executive secretary. She does practice her faith in the Catholic Church but is unable to attend services due to her advanced DX. The patients daughter told me the local Catholic church does come by the ALF and offer communion to the patient periodically.

Indicators of Spiritual Strengths

- *Awareness of/sustained by the transcendent*
 - ☒ Patient
- *Has a sense of meaning, purpose, belonging*
 - ☒ Patient
- *Spiritually at peace*
 - ☒ Patient
- *Belief in life after death/eternal life*
 - ☒ Patient

• Practices prayer, worship, and/or meditation

☒ Patient

• Finds strength in spiritual rituals and/or symbols

☒ Patient

• Finds strength in sacred passages

☒ Patient

Comments related to spiritual strengths:

The patient does identify as a Christian who practices her faith in the LDS church but does not attend due to her DX.

Indicators of Spiritual Concerns/Spiritual Distress

Teaching and other interventions provided during visit (include patient/family's response to teaching/interventions):

The patient was not able to life reflect with me during this visit due to her Dx. I was able to share scripture with the patient from Psalm 23. I was able to sing a few hymns to the patient during this visit and played music from my phone. I was able to talk about Gods love and care for us during this visit and pray for the patient prior to its end.

Communication and Comments

Communication with other team members (include name, title, date, time, and topic(s) discussed):

None at this time.

Comments related to Plan of Care and overall impressions:

This chaplain was able to complete the spiritual assessment today with Katherine at her Assisted living home in Keller Texas. Upon arrival the patient was sitting in her wheel chair finishing lunch and drinking a boost. I was able to speak with the patients daughter prior to this assessment visit and she was able to render the spiritual and life history of the patient to this chaplain due to the patients inability to communicate. The patient suffers from Alzheimer's and is not able to communicate well due to its advanced stage. The patient appears well dressed and groomed. I was able to encourage the patient to drink most of her boost but she was not interested in her lunch food. The patient grew and and raised her family in Texas. She has been married and has two children, a son and a daughter. She worked with the Navy as a flite attendant and later as an executive secretary. She does practice her faith in the Catholic Church but is unable to attend services due to her advanced DX. The patients daughter told me the local Catholic church does come by the ALF and offer communion to the patient periodically. The patient was not able to life reflect with me during this visit due to her Dx. I was able to share scripture with the patient from Psalm 23. I was able to sing a few hymns to the patient during this visit and played music from my phone. I was able to talk about Gods love and care for us during this visit and pray for the patient prior to its end. This chaplain will see the patient 1xm for spiritual support.

Plan for next visit:

Next Month

Date: 10/20/2025

Electronically signed by: VERNON MORRISON

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

BEREAVEMENT RISK ASSESSMENT

Date/Time In 10/20/2025 1:30:00 PM
Date/Time Out 10/20/2025 1:45:00 PM
Risk Assessment Grid

Caregiver	Therese Cox (primary - Daughter)
1: Possible alcohol/substance abuse	☐
1: Neglect of appearance	☐
1: History of mental or emotional health concerns	☐
1: History of marked dependence on patient	☐
1: History of suicide ideation or intent	☐
1: Age less than 18	☐
1: Absence of meaningful spiritual support	☐
1: Death means loss of constant companion or emotional support	☒
1: Death will result in loss of financial provisions/financial support	☒
1: Death means change or loss of role or function	☒
1: Unable to share feelings	☐
1: Difficulty making decisions	☐
1: Ambivalent relationship with the patient	☐
1: Death means loss of meaning/purpose in life	☐
1: Communication difficulties	☐
1: Cultural issues present	☐
1: Inflexible thinking	☐
1: Dependent family members/children living in patient's home	☐
1: Unfinished business with patient	☐
1: Sense of hopelessness	☐
1: Signs of spiritual distress	☐
1: Estranged or isolated from support system	☐
1: Inadequate or negative coping skills	☐
1: Concurrent life crisis	☐

Patient: KATHERINE HOWRIGAN

Medical Record: #101996

HOSPICE PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Initial Certification of Terminal Illness

Start of Care Date: 10/15/2025
Benefit Period: 60 Days
Certification Period - From: 9/3/2025
Certification Period - To: 11/1/2025
Primary Diagnosis: Alzheimer's disease with late onset
ICD-10 Code: G30.1

Written Certification: Based on my clinical judgment, I certify that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Physician Narrative: Explain the clinical findings that support a life expectancy of six months or less. If applicable, include an explanation of why the findings of the face-to-face encounter support a life expectancy of six months or less.
Note: This narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients.

Narrative: Patient 91-year-old female with terminal diagnosis of Alzheimer's. Patient comorbidities of pain edema diarrhea major depressive disorder constipation. Patient admitted to hospice with the following declines a past 3 to 6 months. Patient presents with a PPS of 30% fast score of 6E left MAC of 18.5 cm. Over the past 3 months patient has exhibited progressive cognitive functional and physical decline including increased weakness poor endurance and decreased nutritional intake. Patient sleeps 16 to 18 hours/day and consumes 25% of meals consist of small portions but she rarely finishes. Patient presents as an elderly frail cachectic in appearance. Patient is alert and oriented x 1. Patient requires assistance with 5-6 ADLs previously 4-6 ADLs. Patient is incontinent bowel and bladder. Patient increased weakness with 1 fall in the past 6 months without injury. Patient is primarily wheelchair-bound unable to ambulate independently. Patient hospice appropriate life expectancy less than 6 months.

Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD

Date: 10/15/2025

Patient: KATHERINE HOWRIGAN

Medical Record: #101996

HOSPICE PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Initial Certification of Terminal Illness

Start of Care Date: 10/15/2025
Benefit Period: 60 Days
Certification Period - From: 9/3/2025
Certification Period - To: 11/1/2025
Primary Diagnosis: Alzheimer's disease with late onset
ICD-10 Code: G30.1

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Physician Narrative: Explain the clinical findings that support a life expectancy of six months or less. If applicable, include an explanation of why the findings of the face-to-face encounter support a life expectancy of six months or less.
Note: This narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients.

Narrative: Patient 91-year-old female with terminal diagnosis of Alzheimer's. Patient comorbidities of pain edema diarrhea major depressive disorder constipation. Patient admitted to hospice with the following declines a past 3 to 6 months. Patient presents with a PPS of 30% fast score of 6E left MAC of 18.5 cm. Over the past 3 months patient has exhibited progressive cognitive functional and physical decline including increased weakness poor endurance and decreased nutritional intake. Patient sleeps 16 to 18 hours/day and consumes 25% of meals consist of small portions but she rarely finishes. Patient presents as an elderly frail cachectic in appearance. Patient is alert and oriented x 1. Patient requires assistance with 5-6 ADLs previously 4-6 ADLs. Patient is incontinent bowel and bladder. Patient increased weakness with 1 fall in the past 6 months without injury. Patient is primarily wheelchair-bound unable to ambulate independently. Patient hospice appropriate life expectancy less than 6 months.

Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD

Date: 10/15/2025

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

VERBAL PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Verbal Physician Initial Certification of Terminal Illness

Start of Care Date:

10/15/2025

Benefit Period:

60 Days

Certification Period - From:

9/3/2025

Certification Period - To:

11/1/2025

Primary Diagnosis:

Alzheimer's disease with late onset

ICD-10 Code:

G30.1

Hospice Physician Initial Certification of Terminal Illness

Verbal Certification:

On

10/15/2025

Dr.

DIEGO F RESTREPO MD

certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Verbal Certification Received By:

Electronically signed by: TRACY KING RN

Date:

10/15/2025

Physician Signature / Date:

Attending Physician Initial Certification of Terminal Illness

Verbal Certification:

On

10/15/2025

Dr.

DIEGO F RESTREPO MD

certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Verbal Certification Received By:

Electronically signed by: TRACY KING RN

Date:

10/15/2025

Physician Signature / Date:

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE RECERTIFICATION

Date/Time in: 12/4/2025 10:00:00 AM
Date/Time out: 12/4/2025 10:30:00 AM

LCD/RECERTIFICATION

HEAD TO TOE ASSESSMENT

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 98

• Respiratory Rate

Respiratory rate (respirations per minute): 18
Oxygen liters per minute: 98

• Heart Rate

Type of heart rate: Regular
Radial pulse rate: 76

• Pedal Pulses

- ☒ Present
- ☒ Right
- ☒ Left

Pain Assessment

Pain Rating Type: Non-Verbal Scale
Pain Rating: 0

Gastrointestinal Status

☒ Bowel incontinence

Genitourinary Status

☒ Urinary incontinence

Pressure Ulcer Risk Assessment

Sensory perception: ability to respond meaningfully to pressure-related discomfort	1
Moisture: degree to which skin is exposed to moisture:	1
Activity : degree of physical activity	1
Mobility: ability to change and control body position	1
Nutrition: usual food intake pattern	1
Friction and shear:	1
Total score:	6

CARDIOPULMONARY ASSESSMENT

Respiratory Status

- ☒ Decreasing activity tolerance
 - *Breath Sounds*
 - ☒ Diminished
 - *Breathing Pattern*
 - ☒ Regular

Current Respiratory Medications and Treatments

- ☒ Inhaler(s)

FUNCTIONAL STATUS

- *Bed Mobility*
Score: 1
- *Transfer*
Score: 1
- *Walking/Locomotion*
Score: 1
- *Dressing*
Score: 1
- *Eating*
Score: 1
- *Toilet use*
Score: 1
- *Personal Hygiene*
Score: 1
- *Bathing*
Score: 1

Functional Assessment

Functional assessment staging scale (FAST): 7d - Cannot Sit Up
Karnofsky performance scale (KPS)-all patients: 30% - Severely Disabled
Palliative performance scale (PPS)-all patients: 30% - Totally Bedbound

Fall Risk Assessment (NHPCO Guidelines)

- Total: 6
- ☒ 6-10 points = Moderate risk

CO-MORBIDITIES: What other diagnoses does patient have that may contribute to a decreased life expectancy when combined with the primary diagnosis?

☒ **Dementia**

Narrative: Patient: 91-year-old Caucasian female
Primary Diagnosis: Alzheimer's disease

Comorbidities: Pain, Edema, Diarrhea, Major depressive disorder, recurrent, Essential (primary) hypertension, Nausea, Vomiting, Acute respiratory distress, Constipation, Anxiety disorder. Location: Avalon Memory Care, Keller, TX (Assisted Living Facility)

Clinical Summary:

Since admission, the patient has demonstrated continued functional and cognitive decline. She exhibits increased confusion, generalized weakness, and failure to thrive. Somnolence has increased, with the patient sleeping most of the day and being awake during nighttime hours.

Speech is garbled, and she communicates with word salad. She is unable to make her needs known, and staff must anticipate and meet her needs. The patient is chair-bound and requires total assistance with all 6/6 ADLs. She is incontinent of bowels and bladder. She is unable to hold her head upright, requiring repositioning with pillows for support and comfort.

Appetite remains fair to poor, depending on meal type; she consumes less than 50% of offered meals and requires assistance with feeding. Weight loss is evidenced by temporal muscle wasting. Skin assessment reveals poor turgor, pallor, and a frail appearance.

The family is actively involved in the patient's care and receives updates after each visit. Staff education is ongoing regarding safety, nutrition, skin integrity, and disease progression. The plan of care remains effective for symptom control and comfort maintenance.

PPS: 30% LMAC: 26 cm FAST: 7D

Plan: Continue current hospice plan of care focusing on comfort, pressure injury prevention, nutritional support, infection control, and caregiver education.

Date: 12/4/2025

Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN

Medical Record: #101996

HOSPICE PHYSICIAN RECERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Recertification of Terminal Illness

Start of Care Date: 10/14/2025
Benefit Period: 60 Days
Certification Period - From: 1/1/2026
Certification Period - To: 3/1/2026
Primary Diagnosis: Alzheimer's disease with late onset
ICD-10 Code: G30.1

Written Certification: Based on my clinical judgment, I certify that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Physician Narrative: Explain the clinical findings that support a life expectancy of six months or less. If applicable, include an explanation of why the findings of the face-to-face encounter support a life expectancy of six months or less.
Note: This narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients.

Narrative: Patient 91-year-old female with terminal diagnosis of Alzheimer disease. Patient comorbidities of pain edema diarrhea major depressive disorder hypertension vomiting anxiety constipation. Patient presents with found declines a certification. Patient presents with PPS of 30%. Patient has had a decrease in fast score from 6E-7C. Patient's left MAC is 18.5 cm. Patient sleeping 16 to 18 hours/day. Patient consuming 25% of meals and reports patient has increased dependence on activities of daily living and reduced engagement with surroundings. Patient is alert oriented x 1 patient presents with mild tremors. Patient with hallucinations requiring Haldol. Patient needing assistance with 5-6 ADLs previously 4-6 ADLs. Patient is primarily wheelchair-bound unable to ambulate independently. Patient is incontinent bowel and bladder. Patient was started on Macrobid for UTI this certification. Patient hospice appropriate life expectancy less than 6 months.

Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD

Date: 12/17/2025

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE RECERTIFICATION

Date/Time in: 12/17/2025 10:05:00 AM
Date/Time out: 12/17/2025 10:59:00 AM

LCD/RECERTIFICATION

HEAD TO TOE ASSESSMENT

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 99.5

• Respiratory Rate

Respiratory rate (respirations per minute): 16

• Heart Rate

Type of heart rate: Irregular
Radial pulse rate: 83

• Pedal Pulses

- ☒ Present
- ☒ Right
- ☒ Left
- ☒ Diminished
- ☒ Right
- ☒ Left

Pain Assessment

Pain Rating Type: Non-Verbal Scale
Pain Rating: 0
New, controlled, or uncontrolled? Controlled

Nutritional Status

• Diet Orders

- ☒ General as tolerated
- ☒ Decreasing appetite
- ☒ Evidence of cachexia

Neurological Status

• Consciousness

- ☒ Alert

• *Oriented to*

☒ Person

• *Intellect*

☒ Confusion

☒ Dementia

☒ Long term memory loss

☒ Short term memory loss

• *Energy*

☒ Fatigue/weakness

☒ Increased sleeping

• *Emotions*

☒ Cooperative

• *Speech*

☒ Clear

☒ Nonsensical

Neuromusculoskeletal Status

• *Balance, coordination and mobility*

☒ Shuffling gait/uncoordinated gait

• *Muscle tone*

☒ Muscle wasting

Gastrointestinal Status

☒ Bowel incontinence

Genitourinary Status

☒ Urinary incontinence

☒ Signs/symptoms of urinary tract infection(s)

INTEGUMENTARY STATUS

☒ Skin intact

Pressure Ulcer Risk Assessment

Sensory perception: ability to respond meaningfully to pressure-related discomfort	1
Moisture: degree to which skin is exposed to moisture:	2
Activity : degree of physical activity	2
Mobility: ability to change and control body position	2
Nutrition: usual food intake pattern	2
Friction and shear:	2

Total score:

11

CARDIOPULMONARY ASSESSMENT

Respiratory Status

- ☒ Decreasing activity tolerance
- Breath Sounds

☒ Diminished

☒ Clear
- Breathing Pattern

☒ Regular

Edema

- ☒ Lower extremities
- Score: +1

FUNCTIONAL STATUS

- Bed Mobility

Score: 2
- Transfer

Score: 3
- Walking/Locomotion

Score: 3
- Dressing

Score: 3
- Eating

Score: 1
- Toilet use

Score: 3
- Personal Hygiene

Score: 3
- Bathing

Score: 3

Functional Assessment

Functional assessment staging scale (FAST): 7c - No Ambulation

Karnofsky performance scale (KPS)-all patients: 30% - Severely Disabled

Palliative performance scale (PPS)-all patients: 30% - Totally Bedbound

Fall Risk Assessment (NHPCO Guidelines)

• *History of a fall within the last 6 months*

☒ 2: 1 or 2 falls

• *Mobility*

☒ 3: Confined to bed or chair

• *Elimination*

☒ 3: Incontinent all or most of the time

• *Mental Status*

☒ 4: Confused at all times

☒ 4: Uncooperative or unable to follow directions

• *Medical conditions which could impair the ability to ambulate*

☒ 1: Chronic debilitating illness

☒ 1: Weakness or paralysis of the extremities

☒ 1: Dementia

☒ 1: Anti-hyperintensives

Total: 20

☒ 10 or more points = High risk

CO-MORBIDITIES: What other diagnoses does patient have that may contribute to a decreased life expectancy when combined with the primary diagnosis?

☒ Coronary Artery Disease

☒ Cachexia

☒ Dementia

Narrative: Hospice Recertification Skilled Nursing Note

Patient Name: Katherine Howrigan
Age: 91
Sex/Race: Caucasian Female
Date of Visit: 12/17/2025
Admission Date: 10/15/2025
Primary Hospice Diagnosis: Senile Degeneration of the Brain
Related Comorbidities: Dementia, Osteoporosis, Osteoarthritis, Hypothyroidism, Essential Tremors, Glaucoma, Alopecia
Level of Care: Routine Hospice Care
Advance Directives: OOHDNR
POA/Decision Maker: Daughter
Allergies: NKDA

Scales This Visit

FAST: 7c from a 6e

PPS: 30%

KPS: 30%

LMAC: 18.5 cm

Subjective

SN completed a hospice recertification visit at the patient's group home. The patient's daughter provided updates regarding the patient's current status and ongoing decline. Family continues to desire comfort-focused care only, with no hospitalizations or aggressive interventions.

Over the past several months, the patient has demonstrated continued cognitive, functional, and physical decline. The daughter reports worsening weakness, poor endurance, and significantly decreased oral intake. The patient now sleeps approximately 16–18 hours per day, including frequent daytime naps.

Appetite remains poor, with intake averaging approximately 25% of meals. Meals consist of small portions that are rarely finished. No nausea or vomiting reported. Family reports increased dependence on activities of daily living and reduced engagement with surroundings.

Objective

General: Elderly female, frail and cachectic in appearance, seated in a recliner in the living room at the time of assessment. Calm, no acute distress noted.

Neurological: Alert and oriented ×1. Inconsistently follows simple commands. Speech is clear but limited. Mild essential tremors noted. The patient was changed from Lorazepam to Haloperidol due to hallucinations.

Functional Status:

ADLs: Requires maximum assistance with 5/6 ADLs (previously 4/6).

Mobility: Primarily wheelchair-bound; unable to ambulate independently.

Feeding: Able to self-feed with utensils but requires frequent prompting.

Continence: Incontinent of bowel and bladder.

Nutrition/Weight: Poor appetite with intake of approximately 25% of three small meals daily. Visible muscle wasting and ongoing weight loss were observed. LMAC remains at 18.5 cm.

Skin/Wounds: Skin clean, dry, and intact. No open areas or pressure injuries noted at this time.

Falls: Two falls were reported within the past 6 months; no injuries were sustained.

Cardiopulmonary: Lungs clear to auscultation; respirations even and unlabored on room air. Diminished breath sounds at the bilateral bases. Occasional shortness of breath with exertion.

GI/GU: Abdomen soft and non-tender; bowel sounds present in all four quadrants. Incontinent of bowel and bladder. The patient has been experiencing s/s of a UTI. She has been started on Macrobid 100 mg twice daily x 10 days.

Pain: Patient denies pain at the time of the visit. Acetaminophen is available PRN for mild discomfort.

Mood/Behavior: Cooperative and calm. No agitation or anxiety observed. Patient refused blood pressure measurement; daughter reports this is baseline behavior. SN to notify primary RN to obtain wrist BP cuff for future assessments.

Assessment

91-year-old female with end-stage senile degeneration of the brain demonstrating continued and progressive decline, as evidenced by worsening functional dependence, decreased oral intake, ongoing weight loss with muscle wasting, increased sleep time, and further decline in ADLs. FAST score remains at 6e with significant nutritional compromise and poor endurance. Overall prognosis remains poor, with a life expectancy of six months or less if the disease follows its expected course.

Patient continues to meet hospice eligibility criteria.

Plan

Continued Hospice Care: Patient remains appropriate for routine hospice level of care.

Pain Management: Continue acetaminophen PRN; monitor for changes in pain or discomfort.

Nutrition: Encourage small, frequent meals and nutritional supplements as tolerated.

Skin Integrity: Continue routine skin monitoring due to immobility and incontinence.

Fall Precautions: Maintain safety measures; DME in place, including hospital bed, briefs, chux, and gloves.

Respiratory Care: Monitor for dyspnea; provide comfort measures and oxygen PRN if indicated.

Incontinence Care: Continue use of briefs; reinforce pericare to maintain skin integrity.

Psychosocial Support: Continue Social Work and Chaplain involvement to support the patient and family.

Education: Family educated on disease progression, symptom management, comfort measures, DME use, medication regimen, fall precautions, and hospice philosophy.

IDT Coordination: Findings reviewed with IDT and Dr. Restrepo. Recertification and plan of care reviewed with daughter/MPOA, who remains in agreement with hospice goals.

Scales: PPS 30% | KPS 30% | LMAC 18.5 cm

Code Status: OOH DNR

Allergies: NKDA

Date: 12/17/2025

Electronically signed by: VIRGINIA ALLAN RN ADON

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE PHYSICIAN RECERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Recertification of Terminal Illness

Start of Care Date: 10/14/2025
Benefit Period: 60 Days
Certification Period - From: 1/1/2026
Certification Period - To: 3/1/2026
Primary Diagnosis: Alzheimer's disease with late onset
ICD-10 Code: G30.1

Written Certification: Based on my clinical judgment, I certify that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Physician Narrative: Explain the clinical findings that support a life expectancy of six months or less. If applicable, include an explanation of why the findings of the face-to-face encounter support a life expectancy of six months or less.
Note: This narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients.

Narrative: Patient 91-year-old female terminal diagnosis of Alzheimer disease. Patient will reason of pain edema diarrhea MDD acute respite distress constipation anxiety. Patient presents with the following declines a certification period patient presents with a PPS of 30% with a current FAST score of 7C previously at 6e. Patient's MAC is 18.5 cm. Patient is now sleeping 16 to 18 hours/day. Patient consuming 25% of meals consist of small portions that are rarely finished. Patient is oriented x 1 patient now requires assistance with 5-6 ADLs previously 4-6 ADLs. Patient's primary wheelchair bound unable to ambulate independently. Patient remains incontinent bowel and bladder. Patient hospice appropriate life expectancy less than 6 months

Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD
Date: 12/31/2025

Beneficiary: Howrigan Katherine A
DOD:

Medicare ID: 1X83VK6VY96

Sex: Female

DOB: 06/30/1934

Beneficiary Information

Beneficiary Last Name:*

Howrigan

Beneficiary First Name:**

Katherine

Medicare ID: *

1X83VK6VY96

Beneficiary Birth Date:**

06/30/1934

Beneficiary Name Suffix:

Optional Fields for Requesting Historical Data Using Date Range

Date From:

Date To:

Eligibility

Part A Eligibility

Beneficiary insured due to age OASI (Old-Age and Survivors Insurance)

Effective Date:

06/01/1999

Termination Date:

Part B Eligibility

Beneficiary insured due to age OASI (Old-Age and Survivors Insurance)

Effective Date:

06/01/1999

Termination Date:

Part B Immunosuppressive Drug Eligibility

Effective Date:

Termination Date:

Inactive Periods

Effective Date:

Termination Date:

Reason:

Beneficiary Address

Address Line 1:

5317 Hibbs Drive

Address Line 2:

City:

Fort Worth

State:

TX

Zip:

76137-4901

End Stage Renal Disease (ESRD)

Coverage Period Effective Date:

Coverage Period End Date:

Dialysis Start Date:

Dialysis End Date:

Transplant Effective Date:

Additional Information**MBI End Date**

End Date:

Audiology Screening

HCPSC Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
92550	07/01/2023		257	20%
92552	07/01/2023		257	20%
92553	07/01/2023		257	20%
92555	07/01/2023		257	20%
92556	07/01/2023		257	20%
92557	07/01/2023		257	20%
92562	07/01/2023		257	20%
92563	07/01/2023		257	20%
92565	07/01/2023		257	20%
92567	07/01/2023		257	20%
92568	07/01/2023		257	20%
92570	07/01/2023		257	20%
92571	07/01/2023		257	20%
92572	07/01/2023		257	20%
92575	07/01/2023		257	20%
92576	07/01/2023		257	20%
92577	07/01/2023		257	20%
92579	07/01/2023		257	20%
92582	07/01/2023		257	20%

HCPCS Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
92583	07/01/2023		257	20%
92584	07/01/2023		257	20%
92587	07/01/2023	07/01/2023	257	20%
92588	07/01/2023	07/01/2023	257	20%
92601	07/01/2023		257	20%
92602	07/01/2023		257	20%
92603	07/01/2023		257	20%
92604	07/01/2023		257	20%
92620	07/01/2023		257	20%
92621	07/01/2023		257	20%
92622	07/01/2023		257	20%
92623	07/01/2023		257	20%
92625	07/01/2023		257	20%
92626	07/01/2023		257	20%
92627	07/01/2023		257	20%
92640	07/01/2023		257	20%
92651	07/01/2023		257	20%
92652	07/01/2023		257	20%
92653	07/01/2023		257	20%

MDPP Coverage

Period 1

Start Date:
10/13/2025
End Date:

MDPP Inactive Periods

Start Date:

End Date:

MDPP Deductible

Period 1

Start Date:

10/13/2025

End Date:

Deductible Amount:

0

MDPP Co-Insurance

Period 1

Start Date:

10/13/2025

End Date:

Co-Insurance Amount:

0

MDPP Usage Information

HCPCS Code

Billing NPI

Date Of Service

Deductibles/Caps

Acupuncture

Eligibility begins:

01/21/2020

Technical Sessions Remaining

20
Eligibility begins:
01/21/2020
Professional Sessions Remaining
20

Part B Deductible

Start Date:
01/01/2025
End Date:
12/31/2025
Deductible Amount:
\$257

Part B Remaining Deductible

Start Date:
01/01/2025
End Date:
12/31/2025
Deductible Amount:
\$0

Co-insurance Details

Start Date:
01/01/2025
End Date:
12/31/2025
Co-insurance Amount:
20%

Blood Deductible

Calendar Year:
2025
Number Of Units Remaining:
3

Occupational Therapy Cap

Calendar Year:

2025
Amount Used:
\$0

Physical And Speech Therapy Cap

Calendar Year:
2025
Amount Used:
\$0

Pulmonary Rehabilitation Services

Calendar Year:
2025
Professional Sessions Remaining:
72
Technical Sessions Remaining:
72

Cardiac Rehabilitation Services

Calendar Year:
2025
Professional Sessions Used:
0
Technical Sessions Used:
0

Intensive Cardiac Rehabilitation Services

Calendar Year:
2025
Professional Sessions Used:
0
Technical Sessions Used:
0

Part B Free Services List of STC Codes (listSTCCodes.do)

STC Codes:
42,5,67,80,AJ,CO
Value:

0%

Start Date:

01/01/2025

End Date:

12/31/2025

Mental Health Co-insurance (if different from the plan level Co-insurance)
(listSTCCodes.do)**List of STC Codes**

STC Codes:

Value:

Start Date:

End Date:

Preventive

COVID

COVID-19 Immunization

Immunization Date	HCPC Code	Rendering Medicare NPI
11/03/2021	0004A	1689653040

Pneumococcal Vaccine (PPV)

Pneumococcal Vaccine

Date of Service	HCPC Code	Rendering Medicare NPI
10/25/2009	90732	1053472001

Flu

Flu Vaccination

Vaccination Date	HCPC Code	Rendering Medicare NPI
No data available		

Cognitive Assessment and Care Plan Services

Cognitive Assessment and Care Plan Services

Date of Service	HCPC Code	Rendering Medicare NPI
No data available		

Smoking Cessation

Number of Sessions in Benefit Period:
8
Initial Session Date:
Benefit Period Sessions Remaining:
8

Preventive

HCPCS Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
74263	01/01/2025	01/01/2025	0	0%
76706	07/01/2007	07/01/2007	0	0%
76977	06/01/1999	06/01/1999	0	0%
77067	04/01/2014	04/01/2014	0	0%
77078	06/01/1999	06/01/1999	0	0%
77080	06/01/1999	06/01/1999	0	0%
77081	06/01/1999	06/01/1999	0	0%
80061	01/01/2005	01/01/2005	0	0%
82270	01/01/2007	01/01/2007	0	0%
82465	01/01/2005	01/01/2005	0	0%
82947	01/01/2005	01/01/2005	0	0%
82950	01/01/2005	01/01/2005	0	0%

HCPCS Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
82951	01/01/2005	01/01/2005	0	0%
83036	01/01/2024	01/01/2024	0	0%
83718	01/01/2005	01/01/2005	0	0%
84478	01/01/2005	01/01/2005	0	0%
G0473	01/01/2015	01/01/2015	0	0%
G0118	01/01/2002	01/01/2002	257	20%
G0117	01/01/2002	01/01/2002	257	20%
G0105	06/01/1999	06/01/1999	0	0%
G0104	12/01/2017	12/01/2017	0	0%
G0101	01/01/2014	07/01/2001	0	0%
G0447	11/29/2011	11/29/2011	0	0%
G0446	11/08/2011	11/08/2011	0	0%
G0445	11/08/2011	11/08/2011	0	0%
G0444	10/14/2011	10/14/2011	0	0%
G0442	10/14/2011		0	0%
G0148	07/01/2001	07/01/2001	0	0%
G0147	07/01/2001	07/01/2001	0	0%
G0145	07/01/2001	07/01/2001	0	0%
G0144	07/01/2001	07/01/2001	0	0%
G0143	07/01/2001	07/01/2001	0	0%
G0439	01/01/2011		0	0%
G0438	01/01/2011		0	0%
P3000	07/01/2001	07/01/2001	0	0%
G0130	06/01/1999	06/01/1999	0	0%
G0328	01/01/2004	01/01/2004	0	0%
G0123	07/01/2001	07/01/2001	0	0%
G0121	12/01/2017	12/01/2017	0	0%

HCPCS Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
Q0091	01/01/2014	07/01/2005	0	0%

Hepatitis Screening

Date Of Service	HCPCS Code	Provider or Rendering NPI	Deductible	Co-insurance
-----------------	------------	---------------------------	------------	--------------

HIV/PrEP - Pre-Exposure Prophylaxis for HIV Prevention

Date of Service	HCPCS Code	Provider or Rendering NPI	Deductible	Co-insurance
-----------------	------------	---------------------------	------------	--------------

No data available

Plan Coverage

Medicare Advantage

Plan Type:

Enrollment Date:

Disenrollment Date:

Contract Number:

Plan Name:

Address Line 1:

Phone Number:

Address Line 2:

City:

State:

Zip Code:

Bill Code :

VBID Model Hospice Benefit Component Links

[Hospice Benefit Component participating plans \(https://innovation.cms.gov/innovation-models/vbid-hospice-benefit-participating-plans\)](https://innovation.cms.gov/innovation-models/vbid-hospice-benefit-participating-plans)

[Directions for submitting claims for plans supporting the Hospice Benefit Component of the Model \(https://innovation.cms.gov/innovation-models/vbid-hospice-benefit-billing-payment\)](https://innovation.cms.gov/innovation-models/vbid-hospice-benefit-billing-payment)

Medicare Part D

Enrollment Date:

01/01/2024

Disenrollment Date:

Contract Number:

S2135

Contract Name:

BLUE CROSS BLUE SHIELD ASSOCIATION

Plan Number:

802

Plan Name:

FEHB Basic Option

Address Line 1:

200 E. Randolph St., Suite 1800

Phone Number:

2029421270

Address Line 2:

City:

Chicago

State:

IL

Zip Code:

60601

Website

www.fepblue.org

Drug Plan :

OT

Enrollment

Y

Medicare Secondary Payer

Medicare Secondary Payer

Effective Date:

Termination Date:

Insurer Name:

Policy Number:

Patient Relationship:

Group Number:

Type of Primary Insurance:

Diagnosis Codes:

Address Line 1:

Address Line 2:

City:

State:

Zip Code:

Source Code:

Maintenance Date:

Ongoing Responsibility For Medicals:

Hospice/HomeHealth

Home Health Care

Patient Status:

NOA Indicator:

HHEH Start Date:

HHEH End Date:

HHEH DOEBA Date:

HHEH DOLBA Date:

Provider Number:

Provider Number Type:

Contractor Number:

Contractor Name:

HH Certification Start Date(s):

10/19/2022

HH Recertification Start Date(s):

Hospice

Hospice Episodes:

Effective Date	Termination Date	Start Date (DOEBA)	End Date (DOLBA)	Hospice Days Used	Provider Number	Provider Number Type
09/03/2025	11/01/2025	09/03/2025	09/30/2025	28	1750650917 (https://npiregistry.cms.hhs.gov/search)	NPI
07/05/2025	09/02/2025	07/05/2025	09/02/2025	60	1750650917 (https://npiregistry.cms.hhs.gov/search)	NPI
05/06/2025	07/04/2025	05/06/2025	07/04/2025	60	1750650917 (https://npiregistry.cms.hhs.gov/search)	NPI

Notices of Election (NOE):

Date	Election Receipt Date	Provider Number	Provider Number Type	Revocation Code	Election Revocation Date
03/07/2025	04/01/2025	1306884770	NPI	0	
12/08/2023	12/12/2023	1033705132	NPI	1	03/06/2025

Part A Deductible

Start Date:

01/01/2025
End Date:
12/31/2025
Deductible Amount:
\$1676

Part A Base Deductible Remaining

Start Date:
01/01/2025
End Date:
12/31/2025
Remaining Deductible:
\$1676

Deductible Remaining By Spell

DOEBA Date	DOLBA Date	Deductible Amt
No data to display		

Inpatient Spell Dates

Includes Hospital and Skilled Nursing Facility (SNF) Dates
No data to display

Inpatient Base Summary

Start Date	End Date	Full Days Allowed	Full Copayment Days Allowed	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full Inpatient Days	Full Inpatient Copay Days	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

SNF Base Summary

Start Date	End Date	SNF Full Days Allowed	SNF Full Copayment Days Allowed	SNF Copay Amount
01/01/2025	12/31/2025	20	80	\$209.5

SNF Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full SNF Days	Full SNF Copay Days	SNF Copay Amount
01/01/2025	12/31/2025	20	80	209.5

Lifetime Reserve Days

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Lifetime Days Allowed:

Lifetime Days Remaining :

Calendar Year:

Copayment Amount Per Day:

Lifetime Psychiatric Benefit Data

Lifetime Psychiatric Base Days:

190

Lifetime Psychiatric Remaining:

190

Part A Free Services List of STC Codes (listSTCCodes.do)

STC Codes:

42,45

Value:

0%

Start Date:
01/01/2025
End Date:
12/31/2025

Qualified Medicare Beneficiary (QMB) Program Eligibility

Beneficiaries who are enrolled in the Qualified Medicare Beneficiary (QMB) program are dually eligible for both Medicare and Medicaid. Beneficiaries who are enrolled in the QMB program are not liable for Medicare co-insurance or deductible payments. Note that QMB status may fluctuate for a minority of Beneficiaries. If eligibility results indicate the Beneficiary QMB enrollment has terminated, please verify the patient's QMB status through State online Medicaid eligibility systems or other documentation, including Medicaid Identification cards and documents issued by the State proving the patient qualifies for the QMB program.

Medicaid Enrollment

State:
Enrollment Date:
Termination Date:

Part A Deductible

Start Date:
End Date:
Deductible Amount:

Inpatient Base Summary

Start Date:
End Date:
Full Days Allowed:
Full Copay Days Allowed:
Copay Amount:

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date:
DOLBA Date:
Full Inpatient Days:
Full Inpatient Copay Days:
Copay Amount:

SNF Base Summary

Start Date:

End Date:

SNF Full Days Allowed:

SNF Full Copay Days:

Copay Amount:

SNF Days Remaining (May Also Include Spells)

DOEBA Date:

DOLBA Date:

Full SNF Days:

Full SNF Copay Days:

Copay Amount:

Lifetime Reserve

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Start Date:

End Date:

Copayment Amount:

Part B Deductible

Start Date:

End Date:

Deductible Amount:

Part B Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:

Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Katherine Howigan

have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

Admission Information: The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

After Hours Access/Inpatient Care: A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

Clinical Records: It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

Financial Authorization

I authorize benefits to be made on my behalf

☒ Bill Medicare 100% - Medicare Beneficiary Identifier (MBI) 1X83-VK6-VY96 Effective Date: 06/01/1999

☐ Bill Medicare 100% - Medicaid Beneficiary Number _____ Effective Date: _____

☐ Bill Primary Insurance _____ % Insurance Co. _____

Beneficiary/Pre-Auth Numbers _____ Effective Date: _____

☐ Bill Secondary Insurance _____ % Insurance Co. _____

Beneficiary/Pre-Auth Numbers _____ Effective Date: _____

Benefits are: ☐ Self-pay ☐ Charity or ☐ Other _____

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

Hospice Informed Consent/Election of Hospice Benefit

Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services:

☐ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST
☐ Other: _____

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://txhhs.force.com/complaint/s/>; or by calling 1-800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714-9030 or faxed to: 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complfrm.html>, or by email to ConsumerProtection@tdi.texas.gov.

I acknowledge I have received a copy along with a verbal explanation of the following:

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D Information
- Home Hazardous Waste Disposal information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations.

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: _____ Phone: _____

Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

Hospice Informed Consent/Election of Hospice Benefit

Required Signatures for the Hospice Informed Consent

(Part One of Two Signatures)

Beneficiary/Patient Signature

Date

1. Thurse Corp (P.O.A.)

10-13-25

Authorized Beneficiary Representative Signature (If beneficiary is unable to sign)

Date

Daughter
Relationship to Patient

Cognitive decline due to advanced dx
Reason patient is unable to sign

[Signature]
Hospice Agency Representative Signature

10-13-25

Date

Election of the Medicare/Medicaid Hospice Benefit

(Part Two of Two Requires Signatures)

1. Katherine Howrigan

choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 10-14-25 (start of care date).

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent.

Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5.00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: www.acentraqio.com, phone number 888-315-0636, TTY number 711.

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice).
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
 - Provided by the designated Hospice.
 - Provided by another hospice under arrangements made by the designated Hospice
 - Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services.

Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☒ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is:

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo NPI (if known) 1063664076

Office Address: _____

Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☐ ~~N/A~~ Patient is non-Medicare

☒ N/A I was transferred from Trinity Hospice on 10-14-25

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice)

Horizon Hospice with the effective date of election to begin on 10-14-25

I am electing the (1st, 2nd, 3rd, 4th, etc.) 11th benefit period.

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive.

Beneficiary/Patient Signature

Date

X Shirley Corp (POA.)

10-13-25

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

Daughter

Relationship to Patient

Cognitive decline due to advanced dx

Reason patient is unable to sign

Theresa Jones

Hospice Agency Representative Signature

10-13-25

Date

Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs
Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: _____. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or

☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☒ I am not requesting the addendum. I have been made aware that I can request it at any time.

x Theresa Coy

Signature of Beneficiary/Patient's Authorized Representative

10-13-25
Date

Daughter / cognitive decline due to advanced dx 10-13-25
Representative's Relationship/Reason Patient Unable to Sign Date

Mona
Signature of Witness/Hospice Representative

10-13-25
Date

The Hospice furnished the addendum on:

Date

Primary Care Person Responsibilities

I understand that I am accepting the role as the primary care person for Katherine Howrigan. I further understand this will involve receiving support from this Hospice. The commitment and responsibility of the Hospice is described below. I have a commitment to keep my loved one at home during his/her terminal illness.

I also have a commitment to provide for his/her care as much and as long as I am able to do so.

I understand the Hospice's goal is not to cure a patient's terminal illness, but to reduce symptoms such as pain and nausea and to provide psycho-social and spiritual support to both the patient and the family.

I. I understand the Hospice's obligations are as follows:

- A. The Hospice staff will provide instructions to me as needed.
- B. The Hospice staff will provide emotional and spiritual support to help me cope with this illness and eventual death. This support will be provided by hospice staff and may include volunteers. Hospice staff will also provide support to me after death through a bereavement program.
- C. The Hospice staff will provide patient care by scheduled appointments. Visits will be made to the home and/or nursing facility.

II. I understand the Hospice is available 24 hours a day, and I may call after hours for emergencies.

III. I understand that because the patient and family are cared for as one, the Hospice medical record will contain information about me as well as the patient. Every effort will be made to keep this information confidential. Information about me will be exchanged with the patient.

X Shirley Cox, P.O.A.
Caregiver Signature

10-13-25
Date

[Signature]
Hospice Signature

10-13-25
Date

Patient Choice Statement
(To be completed for all new patients/guardians)

I, Katherine Howrigan (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice.

I have selected Home Hospice (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

x Katherine Howrigan (P.O.A.)
Signature of Patient/Guardian

10-13-25
Date

x [Signature]
Signature of Hospice Representative

10-13-25
Date

Hospice Wound Photography Consent

I, Latherine Howrigan hereby consent and so authorize the taking
(Name of Patient)

of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time.

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: 10-14-25

X Theresa Cox 10-13-25
Patient Signature (or other authorized signature) Date

asmonif 10-13-25
Witness Signature Date

Daughter
Relationship, if other than patient

Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

X Theresa Corp

Beneficiary/Patient/Authorized Representative Signature

10-13-25
Date

[Signature]

Hospice Agency Representative Signature

10-13-25
Date



—HOSPICE—

Homage Hospice
8204 Elmbrook Drive, Suite 276
Dallas, Texas 75247

Office Hours:
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

Medical Records Release Form

Patient Name Katherine Howigan Birth Date 6/30/1934 SS# 048-26-0963
Address 820 W. McLeroy Blvd
City Saginaw State Texas Zip 76179
Home Phone _____ Business Ph _____ Cell 817-689-9919

Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

Medical Records Requested

For the past 1 year:

Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List.

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees and/or agents. For the purpose hereof "Medical Records" includes the following:
Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Homage Hospice in writing. A photocopy of Fax # _____ of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

Mikes Cox
Patient/Authorized Signature

10-13-25
Date

Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Katherine Howigan

have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

Admission Information: The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

After Hours Access/Inpatient Care: A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

Clinical Records: It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

Financial Authorization

I authorize benefits to be made on my behalf

☒ Bill Medicare 100% - Medicare Beneficiary Identifier (MBI) 1X83-VK6-VY96 Effective Date: 06/01/1999

☐ Bill Medicare 100% - Medicaid Beneficiary Number _____ Effective Date: _____

☐ Bill Primary Insurance _____ % Insurance Co. _____

Beneficiary/Pre-Auth Numbers _____ Effective Date: _____

☐ Bill Secondary Insurance _____ % Insurance Co. _____

Beneficiary/Pre-Auth Numbers _____ Effective Date: _____

Benefits are: ☐ Self-pay ☐ Charity or ☐ Other _____

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

Hospice Informed Consent/Election of Hospice Benefit

Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services:

☐ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST
☐ Other: _____

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://txhhs.force.com/complaint/s/>; or by calling 1-800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714-9030 or faxed to: 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complfrm.html>, or by email to ConsumerProtection@tdi.texas.gov.

I acknowledge I have received a copy along with a verbal explanation of the following:

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D Information
- Home Hazardous Waste Disposal information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations.

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: _____ Phone: _____

Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

Hospice Informed Consent/Election of Hospice Benefit

Required Signatures for the Hospice Informed Consent

(Part One of Two Signatures)

Beneficiary/Patient Signature

Date

1. Therese Corp (P.O.A.)

10-13-25

Authorized Beneficiary Representative Signature (If beneficiary is unable to sign)

Date

Daughter
Relationship to Patient

Cognitive decline due to advanced dx
Reason patient is unable to sign

[Signature]
Hospice Agency Representative Signature

10-13-25

Date

Election of the Medicare/Medicaid Hospice Benefit

(Part Two of Two Requires Signatures)

1. Katherine Howrigan

choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 10-14-25 (start of care date).

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent.

Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5.00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: www.acentraqio.com, phone number 888-315-0636, TTY number 711.

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice).
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
 - Provided by the designated Hospice.
 - Provided by another hospice under arrangements made by the designated Hospice
 - Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services.

Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☒ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is:

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo NPI (if known) 1063664076

Office Address: _____

Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☐ ~~N/A~~ Patient is non-Medicare

☒ N/A I was transferred from Trinity Hospice on 10-14-25

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice)

Horizon Hospice with the effective date of election to begin on 10-14-25

I am electing the (1st, 2nd, 3rd, 4th, etc.) 11th benefit period.

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive.

Beneficiary/Patient Signature

Date

X Shirley Corp (POA.)

10-13-25

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

Daughter

Relationship to Patient

Cognitive decline due to advanced dx

Reason patient is unable to sign

Theresa Jones

Hospice Agency Representative Signature

10-13-25

Date

**Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and
Drugs**
Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: _____. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or

☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☒ I am not requesting the addendum. I have been made aware that I can request it at any time.

x Theresa Coy

Signature of Beneficiary/Patient's Authorized Representative

10-13-25
Date

Daughter / cognitive decline due to advanced dx 10-13-25
Representative's Relationship/Reason Patient Unable to Sign Date

Mona
Signature of Witness/Hospice Representative

10-13-25
Date

The Hospice furnished the addendum on: _____

Date

Primary Care Person Responsibilities

I understand that I am accepting the role as the primary care person for Katherine Howrigan. I further understand this will involve receiving support from this Hospice. The commitment and responsibility of the Hospice is described below. I have a commitment to keep my loved one at home during his/her terminal illness.

I also have a commitment to provide for his/her care as much and as long as I am able to do so.

I understand the Hospice's goal is not to cure a patient's terminal illness, but to reduce symptoms such as pain and nausea and to provide psycho-social and spiritual support to both the patient and the family.

I. I understand the Hospice's obligations are as follows:

- A. The Hospice staff will provide instructions to me as needed.
- B. The Hospice staff will provide emotional and spiritual support to help me cope with this illness and eventual death. This support will be provided by hospice staff and may include volunteers. Hospice staff will also provide support to me after death through a bereavement program.
- C. The Hospice staff will provide patient care by scheduled appointments. Visits will be made to the home and/or nursing facility.

II. I understand the Hospice is available 24 hours a day, and I may call after hours for emergencies.

III. I understand that because the patient and family are cared for as one, the Hospice medical record will contain information about me as well as the patient. Every effort will be made to keep this information confidential. Information about me will be exchanged with the patient.

X Shirley Cox, P.O.A.
Caregiver Signature

10-13-25
Date

[Signature]
Hospice Signature

10-13-25
Date

Patient Choice Statement
(To be completed for all new patients/guardians)

I, Katherine Howrigan (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice.

I have selected Home Hospice (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

x Katherine Howrigan (P.O.A.)
Signature of Patient/Guardian

10-13-25
Date

x [Signature]
Signature of Hospice Representative

10-13-25
Date

Hospice Wound Photography Consent

I, Latherine Howrigan hereby consent and so authorize the taking
(Name of Patient)

of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time.

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: 10-14-25

X Theresa Cox 10-13-25
Patient Signature (or other authorized signature) Date

asmonif 10-13-25
Witness Signature Date

Daughter
Relationship, if other than patient

Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

X Theresa Corp

Beneficiary/Patient/Authorized Representative Signature

10-13-25
Date

[Signature]

Hospice Agency Representative Signature

10-13-25
Date



—HOSPICE—

Homage Hospice
8204 Elmbrook Drive, Suite 276
Dallas, Texas 75247

Office Hours:
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

Medical Records Release Form

Patient Name Katherine Howigan Birth Date 6/30/1934 SS# 048-26-0963
Address 820 W. McLeroy Blvd
City Saginaw State Texas Zip 76179
Home Phone _____ Business Ph _____ Cell 817-689-9919

Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

Medical Records Requested

For the past 1 year:

Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List.

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees and/or agents. For the purpose hereof "Medical Records" includes the following:
Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Homage Hospice in writing. A photocopy of Fax file of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

Mikes Cox
Patient/Authorized Signature

10-13-25
Date

Medicare Secondary Payer Screening Form

Patient Name: Howrigan Katherine SOC: 10152025 Date: 10152025

PART I

- Are you receiving Black Lung (BL) Benefits?
☒ No Go to 2.
☐ Yes BL is primary payer only for claims related to BL. Date benefits began: _____
- Are the services to be paid by a government research program?
☒ No Go to 3.
☐ Yes Government Research Program will pay primary benefits for these services.
- Has the Department of Veterans Affairs (DVA) authorized and agreed to pay for your care at this facility?
☒ No Go to 4.
☐ Yes DVA is primary for these services.
- Was the illness/injury due to a work-related accident/condition?
☒ No Go to Part II
☐ Yes Go to *Insurance Information Form* WC is primary payer only for related claims for work-related

PART II

- Was illness/injury due to a non-work-related accident?
☒ No Go to Part III
☐ Yes Go to *Insurance Information Form*

PART III

- Are you entitled to Medicare based on:
☒ Age- Go to PART IV
☐ Disability- Go to PART V
☐ End-Stage Renal Disease (ESRD)- Go to PART VI.
 Complete ALL "PARTS" associated with the patient's selections.

PART IV - AGE

If the patient answers "no" to 2, Medicare is primary unless answered "yes" to questions in part I or II.

- Are you or your spouse currently employed?
 Patient ☒ No - Never worked ☐ No - Retired Date: _____ ☐ Yes Go to 2.
 Spouse ☒ No - Never worked ☐ No - Retired Date: _____ ☐ Yes Go to 2.
- Do you have group health plan (GHP) coverage based on your own or a spouse's current employer who employs 20 or > employees?
 Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
 Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*

PART V - DISABILITY

If the patient answers "no" to all, Medicare is primary unless answered "yes" to questions in part I or II.

- Do you have group health plan (GHP) coverage based on your own, a spouse or family member's current employer who employs 100 or > employees?
 Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
 Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
 Family ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*

PART VI - ESRD

If the patient answers "no" to all in 1, Medicare is primary unless answered "yes" to questions in part I or II.

- Do you have group health plan (GHP) coverage based on your own, a spouse or family member?
 Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
 Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
 Family ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
- Have you received a kidney transplant?
☒ No
☐ Yes Date of Transplant: _____
- Have you received maintenance dialysis treatments?
☒ No
☐ Yes Dialysis Start Date: _____ If app, Self-Dialysis Training Program Start Date: _____
- Are you within the 30-month coordination period that starts MM/DD/CCYY? (The 30-month coordination period starts the first day of the month an individual is eligible for Medicare (even if not yet enrolled in Medicare) because of kidney failure (usually the fourth month of dialysis). If the individual is participating in a self-dialysis training program or has a kidney transplant during the 3-month waiting period, the 30-month coordination period starts with the first day of the month of dialysis or kidney transplant.)
☒ No Medicare is Primary
☐ Yes
- Are you entitled to Medicare on the basis of either ESRD and age or ESRD and disability?
☒ No
☐ Yes
- Was your initial entitlement to Medicare (including simultaneous or dual entitlement) based on ESRD?
☒ No Initial entitlement based on age or disability
☐ Yes GHP Primary during the 30 month coordination period.
- Does the working aged or disability MSP provision apply (i.e., is the GHP already primary based on age or disability entitlement)?
☒ No Initial entitlement based on age or disability
☐ Yes GHP Primary during the 30 month coordination period.

Hospice Signature: [Signature] Date: 10152025

Patient Documents for Howrigan, Katherine A

MRN: 1280577

Consolo Unique Patient ID: 1280577

DOB: 6/30/34

Patient Home Address: 824 W McLeroy Blvd. Saginaw, TX 76179

Facility Address: 824 W McLeroy Blvd. Saginaw, TX 76179 **Facility Phone:** 8178651438

From: Aimee Jenkins <ajenkins@trinityhospice.com>

Local time: Wed Oct 15 2025 13:01:30 GMT+0000 (Coordinated Universal Time)

Agency: Echo Hospice, LLC

Phone: 4697264402

Fax: 4695653781

To: Homage Hospice

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Patient Details for Howrigan, Katherine A (Patient ID 1280577, MRN 1280577)

Start Of Care	03/31/2025	Status	Pending - Discharged
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Start Of Care

03/31/2025

General Patient Details

<u>First Name</u> Katherine	<u>Middle Name</u> A	<u>Last Name</u> Howrigan	<u>Date of Birth</u> 06/30/1934
<u>Home Phone</u> 817-689-9918	<u>SSN</u> 048-26-0963	<u>Religion</u> Catholic	<u>Gender</u> Female

Home Address

<u>Address 1</u> 824 W McLeroy Blvd.	<u>City</u> Saginaw	<u>State</u> TX	<u>County</u> Tarrant, TX
<u>Zipcode</u> 76179			

Mailing Address

<u>Mailing Address 1</u> 824 W McLeroy Blvd.	<u>Mailing City</u> Saginaw	<u>Mailing State</u> TX	<u>Mailing Zipcode</u> 76179
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Patient Care Directives

Code Status: DNR, Disaster Aculty: CLASS3, Disaster Priority Tree: CLASS3

Military Service Info

<u>Veteran</u> Yes	<u>Military Service Branch</u> U.S. Navy	<u>Military Service Era</u> Korea (1966-1974)
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Primary Care Giver

<u>Primary Care Giver</u> Cox, Therese	<u>Relationship</u> Daughter	<u>Address</u> 5317 HIBBS DR FORT WORTH, TX 76137-4901	<u>Home Phone</u> 817-689-9918
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Diagnosis/Insurance Information

Diagnoses/Codes

Primary: G31.1 -
Senile degeneration of
brain, not elsewhere
classified **Secondary:**
F02.80 - Dementia in
other diseases
classified elsewhere,
unspecified severity,
without behavioral
disturbance, psychotic
disturbance, mood
disturbance, and
anxiety **Tertiary:**
K59.01 - Slow transit
constipation **LCD**
Worksheets: CGS:
Non-Cancer Diagnosis
- Dementia due to
Alzheimer's Disease
and Related Disorders
Other Comorbid:
Dementia,
Osteoporosis,
Osteoarthritis,
hypothyroidism,
essential tremors,
glaucoma, alopecia.

Level Of Care

ROUTINE

Primary Hospice Policy

Medicare Part A
1X83VK6VY96

Location Information

Location

Facility

Facility

Edition Senior Living of
Saginaw

Facility Type

Assisted Living

Facility Phone

817-865-1438

Facility Fax

8175461116

County

Tarrant

Address

824 W McLeroy Blvd.
Saginaw, TX 76179

Referral Details

Referral Account

Community Referral

Medical Director

Name

Adams, John

NPI Number

1548262900

Address

906 W. Randol Mill Rd
Arlington, TX 76012

Phone

817-261-4906

Fax

817-664-9625

DME Provider Information

Name

DME Express

Phone

888-355-3208

Staff Assignments

Copy CSV

Discipline	Name	Primary?	Visits	PRN Visits	Frequency Notes
Bereavement	Peoples, Kelly	Yes			
Chaplain	Parish, David	Yes	2 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	
Hospice Aide	Smith, Shenna	Yes	5 visit(s) Weekly		
MD Director	Adams, John	Yes			
Skilled Nurse	Ndah, Bih	Yes	1 visit(s) Weekly	2 PRN visit(s) every 7 day(s)	
Social Worker	Mosley, Marissa	Yes	2 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	

Showing 1 to 6 of 6 entries

← Previous 1 Next →

Screened Allergies

Quick Filter

▼ ▲

Description	Reaction Severity	Reaction/Side Effects	Code Type	Name Type	Code Category	
> No Known Drug Allergy			PAR Class	Primary Name	Active Ingredient	

Page: 1

▼ Rows per page: 25

▼ Displaying 1 - 1 of 1

< >

Unscreened Allergies

See Screened Allergies

Medications

Patient: Howrigan, Katherine A **DOB:** 06/30/1934 **SSN:** 0963

Medication	Order Date	Dosing	Covered
Acetaminophen Extra Strength Instructions 15 ml ORAL 2 times a day for Pain Non-Acute Administer 15mL PO BID for Chronic Pain	04/29/2025	Acetaminophen Extra Strength [1000 MG/30ML] ml Reasons Pain Non-Acute	Covered ✓
Ammonium Lactate Instructions 1 Cream TOPICAL 1 times a day apply to skin after shower daily	03/31/2025	Ammonium Lactate [12 %] Cream Reasons	Covered
ATIVAN / BENADRYL / HALDOL Instructions 1 Cream TOPICAL As Needed for Agitation/Restlessness APPLY 1ML TO TO INNER WRIST EVERY 4 HOURS AS NEEDED FOR AGITATION/RESTLESSNESS	04/29/2028	ATIVAN / BENADRYL / HALDOL [1 dose] Cream Reasons Agitation/Restlessness	Covered ✓
Bactrim DS Instructions 1 Tablet ORAL Give 1 tablet by mouth twice daily for 10 days	10/14/2025	Bactrim DS [800-160 MG] Tablet Reasons	Covered ✓
Cosopt PF Instructions 1 Drop(s) OPHTHALMIC 1 times a day 1 gtt ea eye at bedtime	03/31/2025	Cosopt PF [2-0.5 %] Drop(s) Reasons	Covered
Diclofenac Sodium Instructions 1 ml TOPICAL apply small amount to lower back and hips twice daily for pain	08/01/2025	Diclofenac Sodium [1 %] ml Reasons	Covered
Hydrocortisone Instructions 1 Cream TOPICAL 2 times a day As Needed for Itching	03/31/2025	Hydrocortisone [1 %] Cream Reasons Itching	Covered
Ibuprofen Instructions 1 Tablet ORAL 3 times a day As Needed for Pain	03/31/2025	Ibuprofen [800 MG] Tablet Reasons Pain	Covered ✓
Latanoprost Instructions 1 Solution OPHTHALMIC instill 1 drop to both eyes every night	08/01/2025	Latanoprost [0.005 %] Solution Reasons	Covered
	08/01/2025	Levothyroxine Sodium [75 MCG] Tablet	Covered

Levothyroxine

Sodium

Instructions

1 Tablet ORAL take one tablet by mouth daily

Reasons

Medication

Loperamide HCl

Order Date

03/31/2025

Dosing

Loperamide HCl [2 MG] Tablet

Covered

✓

Instructions

1 Tablet ORAL every 4 hours As Needed for
Diarrhea

Reasons

Diarrhea

Medication

Loratadine

Order Date

03/31/2025

Dosing

Loratadine [10 MG] Tablet

Covered

Instructions

1 Tablet ORAL 1 times a day

Reasons

Medication

MiraLax

Order Date

08/01/2025

Dosing

MiraLax [17 GM] Packet

Covered

✓

Instructions

1 Packet ORAL mix one capful in an 8oz of
liquid of choice and drink by mouth every day
as needed for constipation

Reasons

Medication

MiraLax

Order Date

04/29/2025

Dosing

MiraLax [17 GM/SCOOP] Powder

Covered

Instructions

1 Powder ORAL As Needed for Constipation

Reasons

Constipation

Medication

Multivitamin

Order Date

03/31/2025

Dosing

Multivitamin

Covered

Instructions

1 Tablet ORAL 1 times a day

Reasons

Medication

Oyster Shell

Order Date

03/31/2025

Dosing

Oyster Shell Calcium [500 MG] Tablet

Covered

Calcium

Instructions

1 Tablet ORAL 1 times a day

Reasons

Medication

Senna

Order Date

03/31/2025

Dosing

Senna [8.6 MG] Tablet

Covered

✓

Instructions

1 Tablet ORAL 1 times a day As Needed for
Constipation 1 tab po qd

Reasons

Constipation

Medication

Turns

Order Date

03/31/2025

Dosing

Turns [500 MG] Tablet Chewable

Covered

✓

Instructions

1 - 2 Tablet Chewable ORAL every 4 hours As
Needed for Indigestion

Reasons

Indigestion

Medication

vitamin B12

Order Date

03/31/2025

Dosing

vitamin B12 [1000 mcg] Tablet

Covered

Instructions

1 Tablet ORAL 1 times a day

Reasons

Medication

Vitamin D3

Order Date

03/31/2025

Dosing

Vitamin D3 [25 MCG (1000 UT)] Capsule

Covered

Instructions

1 Capsule ORAL 1 times a day

Reasons

Allergies

Patient: Howrigan, Katherine A DOB: 06/30/1934 SSN: 0963

Description	Severity	Side Effects	Comments
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Unscreened Allergies

See Screened Allergies

Signatures

Patient Details for Howrigan, Katherine A (Patient ID 1280577, MRN 1280577)

Date Of Birth	06/30/1934	Status	Pending - Discharged
Screened Allergies	No Known Drug Allergy	Unscreened Allergies	See Screened Allergies
Start Of Care	03/31/2025	Gender	Female
Patient Address	824 W McLeroy Blvd. Saginaw, TX 76179		

Hospice Certification of Terminal Illness

<u>Benefit Period</u> 11	<u>Benefit Period Start Date</u> 09/03/2025	<u>Benefit Period End Date</u> 11/01/2025	<u>Certification Start Date</u> 09/03/2025
<u>Certification End Date</u> 11/01/2025	<u>Signed Date</u> 08/29/2025	<u>Medical Director or Certifying Physician</u> Adams, John	<u>Medical Director or Certifying Physician Address</u> 906 W. Randol Mill Rd Arlington, TX 76012
<u>Certifying Attending Physician</u> Adams, John	<u>Certifying Attending Physician Address</u> 906 W. Randol Mill Rd Arlington, TX 76012	<u>Patient Hi Claim No</u> 1X83VK6VY96	<u>Disaster Acuity</u> Medical services required within 72 hours

Provider Information

<u>Provider Name</u> AKG Hospice Care LLC dba Trinity Hospice	<u>Provider Number</u> 741569	<u>Provider Address</u> 310 E Interstate 30, Ste 300 Garland, TX 75043-8000
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Admission

New Admission 03/31/2025 (ROUTINE)

Verbal Certification

Verbal Certification received from Medical Director: Adams, John by Taylor-Anyanwu, Debbie (dtayloranyanwu@trinityhospice.com) on 08/29/2025

I certify that Howrigan, Katherine A is terminally ill with a life expectancy of six months or less if the terminal illness runs its normal course.

Admin **Taylor-Anyanwu, Debbie, RN** (User) signed on **08/29/2025**. Recorded by dtayl on 09/02/2025 15:50:15.

Brief Narrative Statement and Attestation

Review the patient's clinical circumstances and synthesize the medical information to provide clinical justification for admission and continuation to hospice services. If in third (3rd) or more benefit periods, clinical findings from the face to face encounter have been used to determine continued eligibility for hospice care.

Composing Physician

Adams, John

Katherine Howrigan is a 91-year-old female admitted to hospice services with a diagnosis of senile degeneration of the brain with comorbid illnesses of dementia, osteoporosis, osteoarthritis, hypothyroidism, essential tremor, glaucoma. On evaluation the patient has decreased appetite aches consuming about 50% of her meals previously now has decreased to 25% sleeping most of the time. Family reports the patient pulls herself to the floor several times during the evening. Her previous agitation has improved. Patient is considered to be terminal in the next 6 months if disease progresses as expected.

Attestation: I confirm that I composed this narrative and that it is based on my review of the patient's medical record and/or examination of the patient.

Medical Director **Adams, DO, John** (Medical Director) signed on **08/29/2025**. Recorded by jadam on 08/29/2025 10:27:08.

Face to Face Visit Details

Physician

Whitener, Pam

Comments

A Face-to-Face assessment was performed with Katherine Howrigan (91 y/o) who resides in the Memory Care Facility @ Edison. She is receiving hospice services for Senile Degeneration of the Brain. Her co-morbidities include Osteoporosis, Osteoarthritis, Hypothyroidism, essential tremors, glaucoma, and Alopecia. She was sitting in the common area, asleep in a wingback chair. She has her hat on and wrapped in a blanket. She stays cold all the time. She was easily aroused, allowed me to assess her and take her vital signs. She was calm and cooperative. CV/Resp: HR irregular, respirations are CTA, diminished at bases, no edema, no cough or congestion. GI/GU: cachetic appearance, incontinent. Staff changes her diapers 3-4 x/daily. She is able to feed herself with much encouragement staff stated. She tires easily when feeding. Skin: Alopecia. M/S: (+) muscle wasting, no ambulation, staff transfers her to wheelchair. She makes no attempt to get out of chair w/o assistance. Neuro: sleeps 12-16 hour daily and naps throughout the day in the chair. She is frail, continues to decline, meets the criteria for hospice services. She is terminal.

BP 96/62 HR 70 R 14 O2 Sat 98% Temp 96.6

MAC 18.5 cm (L)

Karnofsky 30 Palliative 30

Face To Face Physician Attestation

I confirm that I had a face-to-face encounter with Katherine A Howrigan on 08/16/2025 and that the clinical findings of that encounter have been provided to the certifying physician for use in determining continued eligibility for hospice care.

Nurse Practitioner **Whitener, NP, Pam** (Nurse Practitioner) signed on **08/17/2025**. Recorded by pwhit on 08/17/2025 18:43:43.

Medical Director or Certifying Physician Attestation

I confirm that I used the results of this face to face encounter with Howrigan, Katherine A in determining continued eligibility for hospice care.

Medical Director **Adams, DO, John** (Medical Director) signed on **08/29/2025**. Recorded by jadam on 08/29/2025 10:27:15.

Certification Statement

I recertify that this patient is under my care, and to the best of my medical knowledge given the data available, has a life expectancy of six (6) months or less if the terminal illness runs its normal course.

**Medical Director or
Certifying Physician:** Medical Director **Adams, DO, John** (Medical Director) signed on **08/29/2025**. Recorded by jadam on 08/29/2025 10:27:15.

Patient Details for Howrigan, Katherine A (Patient ID 1280577, MRN 1280577)

Date Of Birth	06/30/1934	Status	Pending - Discharged
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IDG Comprehensive Assessment Details

Effective Date 10/10/2025	Next Idg Date 10/24/2025	Bereavement Assessment discussed (if deceased)? No	Recertification discussed (if recertification)? No
IDG Meeting Notes Continue Present Goals	Volunteer Services Offered Other	Volunteer Services Offered (Other): declined	Comprehensive Medication Review Yes
Medication Review Follow-Up ongoing with physician	Patient/Caregiver is in continued agreement with POC Yes	Primary Physician notified of updated POC Yes	Disaster Acuity CLASS3 - Medical services required within 72 hours
Disaster Priority Tree CLASS3			
Patient and/or family/caregivers have been determined to be safely administering medications/biologicals Yes			
Medication/Biologicals Administration Plan Caregiver can safely administer medication			
Patient/Caregiver able and willing to provide care Yes			
Patient/Caregiver plan for providing care Caregiver able to provide care			
Patient or Representative requested Hospice Election Statement Addendum? N/A	Outside provider requested Hospice Election Statement Addendum? N/A		

Other

Patient/Caregiver POC change requests
None

Additional Comprehensive Assessment Comments

SN: dermatologist appointment to remove skin tags. SN weekly. HHA 5x weekly

SCC: Chaplain reviewed and will continue current POC. Visits 2x mo. No new spiritual issues to report. Pt/family seem to benefit from spiritual support.

SW Plan of care reviewed, no new psychosocial needs noted this period.. No concerns reported by family. . SW continuing to follow x2 monthly and PRN.

Issues For Next IDG Meeting
continue to evolve POC

Medications

DOB	Screened Allergies	Unscreened Allergies	Current Diagnosis/Code
06/30/1934	No Known Drug Allergy	See Screened Allergies	G31.1 - Senile degeneration of brain, not elsewhere classified

Pending Medications

Current Medications

Acetaminophen Extra Strength [1000 MG/30ML] ml (ID: 16941571) - 15 ml ORAL 2 times a day for Pain Non-Acute (1000 MG/30ML ml)

Order Date	04/29/2025	Ordering MD	Adams, John
Admin By	Staff (Facility), Nurse (Hospice)	Coverage	Covered
Change	Updated	Updated Date	04/29/2025
Directions For Use	Administer 15mL PO BID for Chronic Pain		
Reasons	Pain Non-Acute		

Ammonium Lactate [12 %] Cream (ID: 16775507) - 1 Cream TOPICAL 1 times a day (12 % Cream)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Not Covered
Change	Updated	Updated Date	04/01/2025
Directions For Use	apply to skin after shower daily		

Cosopt PF [2-0.5 %] Drop(s) (ID: 16775551) - 1 Drop(s) OPHTHALMIC 1 times a day (2-0.5 % Drop(s))

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Not Covered
Change	Updated	Updated Date	04/01/2025
Directions For Use	1 gtt ea eye at bedtime		

Diclofenac Sodium [1 %] ml (ID: 17470560) - 1 ml TOPICAL (1 % ml)

Order Date	08/01/2025	Ordering MD	Adams, John
Admin By	Staff (Facility), Nurse (Hospice)	Coverage	Not Covered
Change	Updated	Updated Date	08/01/2025
Directions For Use	apply small amount to lower back and hips twice daily for pain		

Hydrocortisone [1 %] Cream (ID: 16775634) - 1 Cream TOPICAL 2 times a day As**Needed for Itching (1 % Cream)**

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Not Covered
Change	Updated	Updated Date	04/01/2025
Reasons	Itching		

Ibuprofen [800 MG] Tablet (ID: 16775637) - 1 Tablet ORAL 3 times a day As Needed**for Pain (800 MG Tablet)**

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Covered
Change	Updated	Updated Date	04/01/2025
Reasons	Pain		

Latanoprost [0.005 %] Solution (ID: 17470565) - 1 Solution OPHTHALMIC (0.005 %**Solution)**

Order Date	08/01/2025	Ordering MD	Adams, John
Admin By	Staff (Facility), Nurse (Hospice)	Coverage	Not Covered
Change	Updated	Updated Date	08/01/2025
Directions For Use	instill 1 drop to both eyes every night		

Levothyroxine Sodium [75 MCG] Tablet (ID: 17470572) - 1 Tablet ORAL (75 MCG Tablet)

Order Date	08/01/2025	Ordering MD	Adams, John
Admin By	Staff (Facility), Nurse (Hospice)	Coverage	Not Covered
Change	Updated	Updated Date	08/01/2025
Directions For Use	take one tablet by mouth daily		

Loperamide HCl [2 MG] Tablet (ID: 16775595) - 1 Tablet ORAL every 4 hours As Needed for Diarrhea (2 MG Tablet)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By		Coverage	Covered
Change	Updated	Updated Date	04/01/2025
Reasons	Diarrhea		

Loratadine [10 MG] Tablet (ID: 16775573) - 1 Tablet ORAL 1 times a day (10 MG Tablet)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Not Covered
Change	Updated	Updated Date	04/01/2025

MiraLax [17 GM] Packet (ID: 17470584) - 1 Packet ORAL (17 GM Packet)

Order Date	08/01/2025	Ordering MD	Adams, John
Admin By	Staff (Facility), Nurse (Hospice)	Coverage	Covered
Change	Updated	Updated Date	08/01/2025
Directions For Use	mix one capful in an 8oz of liquid of choice and drink by mouth every day as needed for constipation		

MiraLax [17 GM/SCOOP] Powder (ID: 16942761) - 1 Powder ORAL As Needed for Constipation (17 GM/SCOOP Powder)

Order Date	04/29/2025	Ordering MD	Adams, John
Admin By		Coverage	Not Covered
Change	Updated	Updated Date	04/29/2025
Reasons	Constipation		

Multivitamin (ID: 16775569) - 1 Tablet ORAL 1 times a day (Tablet)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Not Covered
Change	Updated	Updated Date	04/01/2025

Oyster Shell Calcium [500 MG] Tablet (ID: 16775610) - 1 Tablet ORAL 1 times a day (500 MG Tablet)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Not Covered
Change	Updated	Updated Date	04/01/2025

Senna [8.6 MG] Tablet (ID: 16775653) - 1 Tablet ORAL 1 times a day As Needed for Constipation (8.6 MG Tablet)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Covered
Change	Updated	Updated Date	04/01/2025
Directions For Use	1 tab po qd		
Reasons	Constipation		

Tums [500 MG] Tablet Chewable (ID: 16775601) - 1 - 2 Tablet Chewable ORAL every 4 hours As Needed for Indigestion (500 MG Tablet Chewable)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Covered
Change	Updated	Updated Date	04/01/2025
Reasons	Indigestion		

vitamin B12 [1000 mcg] Tablet (ID: 16775624) - 1 Tablet ORAL 1 times a day (1000 mcg Tablet)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By		Coverage	Not Covered
Change	Updated	Updated Date	04/01/2025

Vitamin D3 [25 MCG (1000 UT)] Capsule (ID: 16775565) - 1 Capsule ORAL 1 times a day (25 MCG (1000 UT) Capsule)

Order Date	03/31/2025	Ordering MD	Adams, John
Admin By	Family, Staff, Nurse	Coverage	Not Covered
Change	Updated	Updated Date	04/01/2025

Active Care Plan Problems

Assignment Details

Effective Date
08/24/2025

IDG Team

Discipline	Name	Email	Primary?	Visits	PRN Visits	Frequency Notes
Bereavement	Peoples, Kelly	kpeoples@trinityhospice.com	Yes			
Chaplain	Parish, David	dparish@trinityhospice.com	Yes	2 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	
Hospice Aide	Smith, Shenna	ssmith@trinityhospice.com	Yes	5 visit(s) Weekly		
MD Director	Adams, John		Yes			
Skilled Nurse	Ndah, Bih	bndah@trinityhospice.com	Yes	1 visit(s) Weekly	2 PRN visit(s) every 7 day(s)	
Social Worker	Mosley, Marissa	mmosley@trinityhospice.com	Yes	2 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	

Form 15-12-01-01-01-01-01

OUT-OF-HOSPITAL DO-NOT-RESUSCITATE (OOH-DNR) ORDER

TEXAS DEPARTMENT OF STATE HEALTH SERVICES



Signature: Patrick Bibb Date of birth: 04/21/2075

4. Declaration of the adult person: I am a competent and at least 18 years of age. I direct that none of the following resuscitation measures be initiated or continued for me: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Signature: _____ Date: _____ Printed name: _____

8. Declaration by legal guardian, agent or proxy on behalf of the adult person who is incompetent or otherwise incapable of communication: I am the above noted person's: ☐ legal guardian ☐ agent in a Medical Power of Attorney, OR ☐ mentally or physically incapable of communication.

9. Declaration by the known desires of the person, or a determination of the best interests of the person, I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Signature: _____ Date: _____ Printed name: _____

11. Declaration by a qualified relative of the adult person who is incompetent or otherwise incapable of communication: I am the above noted person's: ☐ spouse ☒ adult child ☐ parent, OR ☐ nearest living relative, and I am qualified to make this treatment decision under Health and Safety Code §160.061.

12. I am the known desires of the person, or a determination of the best interests of the person, I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Signature: Patrick Bibb Date: 04/21/2075 Printed name: Patrick Bibb

13. Declaration by physician based on directive to physicians by a person now incompetent or nonwritten communication to the physician by a competent person: I am the above noted person's attending physician and have:

☐ seen evidence of his/her previously issued directive to physicians by the adult, now incompetent, OR ☐ observed his/her spouse take two witnesses of an OOH-DNR in a nonwritten manner.

I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Attending physician's signature: _____ Date: _____ Printed name: _____ Title: _____

14. Declaration on behalf of the minor person: I am the minor's: ☐ parent ☐ legal guardian, OR ☐ managing conservator.

A physician has diagnosed the minor as suffering from a terminal or irreversible condition. I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Signature: _____ Date: _____ Printed name: _____

TWO WITNESSES (See qualifications on backside) We have witnessed the above-noted competent adult person or authorized declarant making his/her signature above and, if applicable, the above-noted adult person making an OOH-DNR by nonwritten communication to the attending physician.

Witness 1 signature: _____ Date: _____ Printed name: _____

Witness 2 signature: _____ Date: _____ Printed name: _____

Notary in the State of Texas and County of TARRANT The above noted person personally appeared before me and signed the above noted declaration on this date 04/21/2075.

Signature & seal: Marissa Masley Notary's printed name: MARISSA MASLEY Notary's commission expires: 04/2026

[Notar: Notary cannot acknowledge the witnessing of the person making an OOH-DNR order in a nonwritten manner.]

PHYSICIAN'S STATEMENT: I am the attending physician of the above-noted person and have noted the existence of this order in this person's medical record. I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Physician's signature: John W Adams, DO Date: 04/21/2075
Physician's name: JOHN W ADAMS License #: E2145

15. I authorize the last instructions on behalf of the adult, who is incompetent or unable to communicate and without guardian, agent, proxy or relative. The person's last wishes are a document that should be kept in the person's medical record or in a secure location. I direct that none of the following resuscitation measures be initiated or continued for the person: cardiopulmonary resuscitation (CPR), transcutaneous cardiac pacing, defibrillation, advanced airway management, artificial ventilation.

Witness 1 signature: _____ Date: _____ Printed name: _____
Witness 2 signature: _____ Date: _____ Printed name: _____

16. I am a competent adult and I am at least 18 years of age. I have read and understand the contents of this document and I have signed it.

All persons who have signed above must sign below, acknowledging that this document has been properly completed.

Signature: _____ Date: _____ Printed name: _____

Signature: John W Adams, DO Date: 04/21/2075 Printed name: JOHN W ADAMS

**MEDICAL POWER OF ATTORNEY
AND HIPAA RELEASE AUTHORIZATION
OF
KATHERINE ANN HOWRIGAN**

Prepared by:

**Steven A. Early, P.C.
5850 Colleyville Blvd., Suite 110
Colleyville, Texas 76034**

**817-605-8880
817-605-8882 (facsimile)**

MEDICAL POWER OF ATTORNEY DISCLOSURE STATEMENT

OF

KATHERINE ANN HOWRIGAN

THIS IS AN IMPORTANT LEGAL DOCUMENT REGARDING YOUR MEDICAL POWER OF ATTORNEY. BEFORE SIGNING THIS DOCUMENT, YOU SHOULD KNOW THESE IMPORTANT FACTS:

Except to the extent you state otherwise, this document gives the person you name as your agent the authority to make any and all health care decisions for you in accordance with your wishes, including your religious and moral beliefs, when you are no longer capable of making them yourself. Because "health care" means any treatment, service or procedure to maintain, diagnose, or treat your physical or mental condition, your agent has the power to make a broad range of health care decisions for you. Your agent may consent, refuse to consent or withdraw consent to medical treatment and may make decisions about withdrawing or withholding life-sustaining treatment. Your agent may not consent to voluntary inpatient mental health services, convulsive treatment, psychosurgery or abortion. A physician must comply with your agent's instructions or allow you to be transferred to another physician. Your agent's authority begins when your doctor certifies that you lack the competence to make health care decisions. Your agent is obligated to follow your instructions when making decisions on your behalf. Unless you state otherwise, your agent has the same authority to make decisions about your health care as you would have had.

It is important that you discuss this document with your physician or other health care provider before you sign it to make sure that you understand the nature and range of decisions that may be made on your behalf. If you do not have a physician, you should talk with someone else who is knowledgeable about these issues and can answer your questions. You do not need a lawyer's assistance to complete this document, but if there is anything in this document that you do not understand, you should ask a lawyer to explain it to you. The person you appoint as agent should be someone you know and trust. The person must be 18 years of age or older or a person under 18 years of age who has had the disabilities of minority removed. If you appoint your health or residential care provider (e.g., your physician or an employee of a home health agency, hospital, nursing home or residential care home, other than a relative), that person has to choose between acting as your agent or as your health or residential care provider; the law does not permit a person to do both at the same time.

You should inform the person you appoint that you want the person to be your health care agent. You should discuss this document with your agent and your physician and give each a signed copy. You should indicate on the document itself the people and institutions who have copies of the signed originals. Your agent is not liable for health care decisions made in good faith on your behalf. Even after you have signed this document, you have the right to make health care decisions for yourself as long as you are able to do so and treatment cannot be given to you or stopped over your objection. You have the right to revoke the authority granted to your agent by informing your agent or your health or residential care provider orally or in writing, or

by your execution of a subsequent medical power of attorney. This document may not be changed or modified. If you want to make changes in the document, you must make an entirely new one. You may wish to designate an alternate agent in the event that your agent is unwilling, unable or ineligible to act as your agent. Any alternate agent you designate has the same authority to make health care decisions for you.

THIS POWER OF ATTORNEY IS NOT VALID UNLESS:

- (1) YOU SIGN IT AND HAVE YOUR SIGNATURE ACKNOWLEDGED BEFORE A NOTARY PUBLIC; OR
- (2) YOU SIGN IT IN THE PRESENCE OF TWO COMPETENT ADULT WITNESSES.

THE FOLLOWING PERSONS MAY NOT ACT AS ONE OF THE WITNESSES:

- (1) the person you have designated as your agent;
- (2) a person related to you by blood or marriage;
- (3) a person entitled to any part of your estate after your death under a will or codicil executed by you or by operation of law;
- (4) your attending physician;
- (5) an employee of your attending physician;
- (6) an employee of a health care facility in which you are a patient if the employee is providing direct patient care to you or is an officer, director, partner, or business office employee of the health care facility or of any parent organization of the health care facility; or
- (7) a person who, at the time this power of attorney is executed, has a claim against any part of your estate after your death.

I have read and understand the contents of this disclosure statement, and I certify that I signed this disclosure statement prior to signing my Medical Power of Attorney.

Date: October 03, 2014.

Katherine Ann Howrigan
KATHERINE ANN HOWRIGAN

**MEDICAL POWER OF ATTORNEY
AND HIPAA RELEASE AUTHORIZATION**

I, KATHERINE ANN HOWRIGAN (also known as KATHERINE ANN SCHULER),
designate:

Name: PATRICK JOHN BIBO
Address: 8404 Cielo Vista Drive
El Paso, Texas 79925
Phone: (915) 772-6763

as my agent (hereinafter referred to as "agent") to make any and all health care decisions for me. This medical power of attorney takes effect if I become unable to make my own health care decisions and this fact is certified in writing by my physician.

DESIGNATION OF ALTERNATE AGENT

If PATRICK JOHN BIBO is unable or unwilling to make health care decisions for me, I designate the following person as my agent to make health care decisions for me as authorized by this document:

Name: THERESE JUSTINE COX
Address: 5317 Hibbs Drive
Fort Worth, Texas 76137
Phone: (817) 689-9918

LIMITATIONS

Limitations on the decision-making authority of my agent are as follows:

I hereby require my agent to direct my physician to comply with any valid Directive to Physicians (or similar document) which I may have heretofore executed or which I may hereafter execute. My agent is not authorized to direct my physician in a manner which would contradict any such valid Directive to Physicians (or similar document).

HIPAA RELEASE AUTHORITY

I intend for my agent to be treated as I would be treated with respect to my rights regarding the use and disclosure of my individually identifiable health information and other medical records. This release authority applies to any information governed by the Health Insurance Portability and Accountability Act of 1996 ("HIPAA"), 42 USC 1320d and 45 CFR 160-164. This release authority is effective immediately.

Accordingly, I hereby authorize any doctor, physician, medical specialist, psychiatrist, chiropractor, health-care professional, dentist, optometrist, health plan, hospital, hospice, clinic, laboratory, pharmacy or pharmacy benefit manager, medical facility, pathologist, or other provider of medical or mental health care, as well as any insurance company and the Medical Information Bureau Inc. or other health-care clearinghouse that has paid for or is seeking payment from me for such services (referred to herein as a "covered entity"), to give, disclose and release to my agent who is named herein, without restriction, all of my individually identifiable health information and medical records regarding any past, present or future medical or mental health condition, including all information relating to the diagnosis and treatment of HIV/AIDS, sexually transmitted diseases, mental illness, and drug or alcohol abuse. Additionally, my agent shall have the ability to ask questions and discuss my protected medical information with the person or entity who has possession of the protected medical information even if I am fully competent to ask questions and discuss this matter at the time. It is my intention to give a full authorization to any protected medical information to my agent. Such information may also be released to any person designated as a primary or successor agent or attorney-in-fact in a durable power of attorney which I have executed, whether or not such person is presently serving as such, and to any person presently serving as trustee or named as a successor trustee in any revocable or irrevocable trust created by me as grantor.

In determining whether I am incapacitated, all individually identifiable health information and medical records shall be released to the person who is nominated as my agent hereunder, including any written opinion relating to my incapacity that the person nominated as my agent may have requested. This release authority applies to any information governed by HIPAA and applies even if that person has not yet begun serving as my agent.

This authority given to my agent shall supersede any prior agreement that I may have made with my health-care providers to restrict access to or disclosure of my individually identifiable health information. The individually identifiable health information and other medical records given, disclosed, or released to my agent may be subject to redisclosure by my agent and may no longer be protected by HIPAA. Each covered entity that acts in reliance on this release shall be released from liability which may result from disclosing my individually identifiable health information and other medical records. I authorize my agent to bring a legal action against a covered entity which refuses to accept and recognize this release authority. No covered entity may condition treatment, payment, enrollment or eligibility for benefits on whether I sign this authorization when the prohibition on conditioning of authorizations in 45 CFR 164.508(b)(4) applies. Further, in order to fulfill my intent as expressed herein, I authorize my agent to sign any documentation that my agent deems necessary or appropriate in order to secure the disclosure of my individually identifiable health information and other medical records. Any information disclosed to my agent may subsequently be disclosed to another party by my agent. My agent shall not be required to indemnify a covered entity or perform any act in the event information is subsequently disclosed by my agent. The authority given to my agent herein has no expiration date and shall expire only in the event that I revoke this Medical Power of Attorney in writing and deliver it to my health-care provider. There are no exceptions to my right to revoke this Medical Power of Attorney.

ORIGINAL

The original of this document is kept at:

COPIES

The following individuals or institutions have copies of the signed originals:

Name:

Address:

Name:

Address:

DURATION

I understand that this power of attorney exists indefinitely from the date I execute this document unless I establish a shorter time or revoke the power of attorney. If I establish a shorter time and if I am unable to make health care decisions for myself when this power of attorney does expire, the authority I have granted my agent shall, nevertheless, continue to exist until the time I become able to make health care decisions for myself. I do not wish to have this power of attorney end on a specified date.

PRIOR DESIGNATIONS REVOKED

I revoke any prior durable power of attorney for health care and any prior medical power of attorney.

ACKNOWLEDGMENT OF DISCLOSURE STATEMENT

I have been provided with a disclosure statement explaining the effect of this document. I have read and understand the information contained in the disclosure statement.

I sign my name to this medical power of attorney on October 03, 2014, 2014, at El Paso County, Texas.

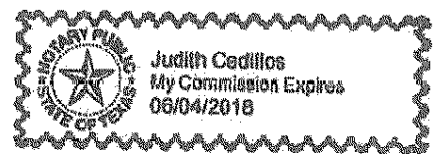
Katherine Ann Howrigan
KATHERINE ANN HOWRIGAN, Principal

THE STATE OF TEXAS

COUNTY OF EL PASO

§
§
§

This instrument was acknowledged before me on October 3rd, 2014, by KATHERINE ANN HOWRIGAN.



J. Cedillos
Notary Public, State of Texas

Patient Documents for Howrigan, Katherine A

MRN: 1280577

Console Unique Patient ID: 1280577

DOB: 6/30/34

Patient Home Address: 824 W McLeroy Blvd. Saginaw, TX 76179

Facility Address: 824 W McLeroy Blvd. Saginaw, TX 76179 **Facility Phone:** 8178651438

From: Aimee Jenkins <ajenkins@trinityhospice.com>

Local time: Wed Oct 15 2025 13:33:41 GMT+0000 (Coordinated Universal Time)

Agency: Echo Hospice, LLC

Phone: 4697264402

Fax: 4695653781

To: Homage Hospice

IMPORTANT: This facsimile transmission contains confidential information, some or all of which may be protected health information as defined by the federal Health Insurance Portability & Accountability Act (HIPAA) Privacy Rule. This transmission is intended for the exclusive use of the individual or entity to whom it is addressed and may contain information that is proprietary, privileged, confidential and/or exempt from disclosure under applicable law. If you are not the intended recipient (or an employee or agent responsible for delivering this facsimile transmission to the intended recipient), you are hereby notified that any disclosure, dissemination, distribution or copying of this information is strictly prohibited and may be subject to legal restriction or sanction. Please notify the sender by telephone (number listed above) to arrange the return or destruction of the information and all copies.

Plan of Care Effective 8/19/25

Notes

Recert POC, see IDG snapshot Q 2 weeks for updates

Patient Details

Name	Consono Patient ID	MRN	Office	DOB	SSN	Address
Katherine Howrigan	1280577		AKG Hospice Care LLC dba Trinity Hospice	6/30/34	0963	824 W McLeroy Blvd. Saginaw, TX 76179

Assignments

8/24/25

Discipline	Name	Primary?	Visits	PRN Visits	Frequency Notes
Skilled_nurse	Ndah, Bih	Yes	1 visit(s) Weekly	2 PRN visit(s) every 7 day(s)	
Bereavement	Peoples, Kelly	Yes			
Chaplain	Parish, David	Yes	2 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	
Medical_director	Adams, John	Yes			
Hospice_aide	Smith, Shenna	Yes	5 visit(s) Weekly		
Social_worker	Mosley, Marissa	Yes	2 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	

4/29/25

Discipline	Name	Primary?	Visits	PRN Visits	Frequency Notes
Skilled_nurse	Ndah, Bih	Yes	1 visit(s) Weekly	2 PRN visit(s) every 7 day(s)	
Bereavement	Peoples, Kelly	Yes			
Chaplain	Parish, David	Yes	2 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	
Medical_director	Adams, John	Yes			
Hospice_aide	Smith, Shenna	Yes	3 visit(s) Weekly		
Social_worker	Mosley, Marissa	Yes	2 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	

4/14/25

Discipline	Name	Primary?	Visits	PRN Visits	Frequency Notes
Skilled_nurse	Ndah, Bih	Yes	2 visit(s) Weekly	2 PRN visit(s) every 7 day(s)	
Bereavement	Peoples, Kelly	Yes			
Chaplain	Parish, David	Yes	1 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	
Medical_director	Adams, John	Yes			
Hospice_aide	Smith, Shenna	Yes	3 visit(s) Weekly		
Social_worker	Mosley, Marissa	Yes	1 visit(s) Monthly	2 PRN visit(s) every 30 day(s)	

3/31/25

Discipline	Name	Primary?	Visits	PRN Visits	Frequency Notes
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Skilled_nurse	Ndah, Bih	Yes	1 visit(s) Weekly	2 PRN visit(s) every 7 day(s)
Bereavement	Peoples, Kelly	Yes		
Chaplain	Parish, David	Yes	1 visit(s) Monthly	2 PRN visit(s) every 30 day(s)
Medical_director	Adams, John	Yes		
Hospice_aide	Smith, Shenna	Yes	3 visit(s) Weekly	
Social_worker	Mosley, Marissa	Yes	1 visit(s) Monthly	2 PRN visit(s) every 30 day(s)

Care Plan Problems

Problem	Goal
Spiritual	Comfort from prayer and scripture reading
Effective Date	Last Updated
4/3/25	8/20/25

Interventions

Name	Due	Updated
Provide prayer and scripture reading		8/20/25

Problem	Goal
Wound: Right forearm skin tear	Patient will remain free on infection Skin integrity will be promoted and comfort maintained
Effective Date	Last Updated
	5/26/25

Interventions

Name	Due	Updated
Assess skin and monitor for skin breakdown on every visit		5/6/25
Complete detailed wound assessment with measurements at least once weekly		5/6/25
Provide wound care as prescribed by physician: < Cleanse with wound cleanser, pat dry put hydrogel, cover with silicone, change 2x weekly weekly by hospice nurse or when soiled by facility >		5/6/25
Teach patient/CG on wound care, when to notify nurse		5/6/25
Teach patient/CG regarding: skin care; positioning; signs of infection		5/6/25

Problem	Goal
Patient demonstrates loss of memory, dementia diagnosis, limited ability to communicate wishes and needs	Ensure comfort, support, and reassurance through interactions that accommodate patient's ability to communicate
Effective Date	Last Updated
	8/8/25

Interventions

Name	Due	Updated
Promote positive psychosocial environment	8/8/25	
Inform patient/CG of community resources available to them, coordinating with these resources	8/8/25	
Provide reassurance and conflict resolution	8/8/25	

Problem

Problem	Goal
Pre-Bereavement Risk Assessment	Increased bereavement risks will be identified, support and resources offered as-needed

Effective Date

Last Updated

8/8/25

Interventions

Name	Due	Updated
Assess level of bereavement risk during the initial comprehensive assessment. Notify Bereavement Coordinator and IDG members if moderate or high risk identified.	6/5/25	
Assess ongoing bereavement risk. Notify IDG of any potential changes in bereavement risk.	7/30/25	
LOW RISK: CH and SW frequency minimum once monthly or per patient/family preference to monitor for changes to bereavement risk. RNCM monitor bereavement risk and coordinate with IDG if changes to bereavement risk occur.	7/30/25	

Problem

Goal

Problem	Goal
Pre-existing DNR filled out incorrectly by previous hospice provider. Patient currently full code	Complete new OOHDNR with daughter

Effective Date

Last Updated

5/14/25

Interventions

Name	Due	Updated
Educate daughter on proper filing of DNR and complete new document	5/14/25	

Problem

Goal

Problem	Goal
Integumentary Status	Skin integrity will be promoted and comfort maintained Patient remains free from skin breakdown Patient remains free of infection

Effective Date

Last Updated

3/31/25

8/27/25

Interventions

Name	Due	Updated
Assess skin and monitor for skin breakdown on every visit		7/25/25
Monitor for signs of discomfort every visit		7/25/25
Teach patient/CG on measures to prevent skin breakdown <SPECIFY>		8/27/25
Teach patient/CG regarding: skin care; positioning; signs of infection		8/27/25

Problem	Goal
Musculoskeletal Function	Patient/CG will understand changes in strength and endurance as it relates to disease process Patient will remain free from injury and function as optimally as disease allows Patient/CG will verbalize/demonstrate safe use of DME

Effective Date	Last Updated
3/31/25	8/25/25

Interventions

Name	Due	Updated
Assess safety in transfers		8/4/25
Assess safety in home for DME use		8/4/25
Assess strength, level of activity tolerance, ability to perform ADLs		7/2/25
Evaluate need for assistive devices and make arrangements		7/2/25
Hospice Aide to assist with ADLs		8/25/25
Instruct patient/CG on prevention of injury with activity (specify)		8/4/25
Teach patient/CG on comfort measures (specify)		8/25/25
Teach patient/CG on proper use of DME for safety		8/25/25

Problem	Goal
Genitourinary Function	Skin integrity will be promoted Patient remains free from infection (specify: skin/UTI) Effective control of bladder discomfort Symptoms managed

Effective Date	Last Updated
3/31/25	8/25/25

Interventions

Name	Due	Updated
Assess/monitor for signs of infection		7/2/25
Assess/monitor for skin breakdown related to incontinence		7/2/25
Assess/monitor for urinary incontinence		4/29/25
Assess/monitor for: urinary retention; urinary frequency; anuria; polyuria		7/2/25
Assess/monitor urinary status on every visit		7/2/25
Teach patient/CG on medication administration		8/25/25

Teach patient/CG regarding: comfort and symptom control ; skin care; disease process

8/25/25

Problem	Goal
Neurological Function	Patient will function as optimally as disease allows Patient able to respond to reality orientation Patient/CG will develop personally acceptable coping mechanisms Symptoms of anxiety are decreased Symptoms managed with medications for reducing discomfort

Effective Date	Last Updated
3/31/25	8/25/25

Interventions

Name	Due	Updated
Assess/monitor ability to communicate every visit		7/19/25
Assess/monitor for mental status changes every visit (i.e. confusion; agitation; restlessness)		7/19/25
Provide alternative methods of communication: writing; pictures		8/25/25
Provide reality orientation on every visit		7/19/25
Refer to SW for support and resources		7/19/25
Teach patient/CG on safety measures (i.e. remove area rugs; special call bell for assistance)		7/19/25

Problem	Goal
Initial Plan of Care prior to service provision	Facilitate compliance with CMS regulation regarding new admissions to Hospice Services. Assess/implement immediate global needs

Effective Date	Last Updated
3/31/25	4/30/25

Interventions

Name	Due	Updated
Assess for immediate Global needs		4/1/25
Assess for Knowledge deficit and provide needed education to Patient/family/caregiver		4/1/25
Assess pain/symptoms and intervene as necessary to bring patient to level of comfort		4/1/25
Assign chaplain services as necessary for ongoing spiritual needs		4/1/25
Assign Hospice Aide services to provide for personal care needs		4/1/25
Assign RN case manager to assess scope of service and oversee ongoing patient care needs		4/1/25
Assign social worker as necessary to assess for ongoing psychosocial needs		4/1/25
Collaborate with interdisciplinary team for team agreement		4/1/25
Collaborate with Medical Director/Physician for Admission orders and obtain authorization		4/1/25
If patient in facility collaborate with facility staff		4/1/25
Obtain DME/supplies as needed for patient support		4/1/25

Obtain medication and treatment orders-institute immediately 4/1/25

RN to initiate plan of care and make necessary revisions based upon patient current assessment 4/1/25

Indicators

3/31/25

Primary Diagnosis/Code: G31.1 - Senile degeneration of brain, not elsewhere classified

LCD Worksheet(s)

- CGS: Non-Cancer Diagnosis - Dementia due to Alzheimer's Disease and Related Disorders - The patient is a 90 y/o female admitted to Hospice services for Senile degeneration of brain. PMH: Dementia, Osteoporosis, Osteoarthritis, hypothyroidism, essential tremors, glaucoma, alopecia. Her speech is garbled and although she remembers her 1st name she does not remember last name is Howrigan -she mumbles a name that dtr said sounded like 2nd husband that she has not talked about in over 20 years. She is very sensitive, even to point that touching her is painful. She has arthritis of most joints that cause her pain. Noted today when RN attempted to move her legs to examine her feet. She cried out and jerked back - she did allow examination after a short time. Pain is controlled with Tylenol at this time and daughter does not want opioids given at this time.
- Symptoms:

Diagnoses/Codes

- G31.1 - Senile degeneration of brain, not elsewhere classified
- F02.80 - Dementia in other diseases classified elsewhere, unspecified severity, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety
- K59.01 - Slow transit constipation
- R13.11 - Dysphagia, oral phase
- M62.81 - Muscle weakness (generalized)
- R32 - Unspecified urinary incontinence
- R15.9 - Full incontinence of feces
- Z99.3 - Dependence on wheelchair
- Z91.81 - History of falling
- Z66 - Do not resuscitate
- Z51.5 - Encounter for palliative care
- I10 - Essential (primary) hypertension
- M15.0 - Primary generalized (osteo)arthritis
- M81.0 - Age-related osteoporosis without current pathological fracture
- G25.0 - Essential tremor
- D64.9 - Anemia, unspecified
- E03.9 - Hypothyroidism, unspecified
- E55.9 - Vitamin D deficiency, unspecified
- H40.9 - Unspecified glaucoma
- Z85.828 - Personal history of other malignant neoplasm of skin
- Z87.81 - Personal history of (healed) traumatic fracture

Treatments to Date:

Other Health Problems:

Other Comorbid: Dementia, Osteoporosis, Osteoarthritis, hypothyroidism, essential tremors, glaucoma, alopecia.

3/31/25

Primary Diagnosis/Code: G31.1 - Senile degeneration of brain, not elsewhere classified

LCD Worksheet(s)

- CGS: Non-Cancer Diagnosis - Dementia due to Alzheimer's Disease and Related Disorders - The patient is a 90 y/o female admitted to Hospice services for Senile degeneration of brain. PMH: Dementia, Osteoporosis, Osteoarthritis, hypothyroidism, essential tremors, glaucoma, alopecia. Her speech is garbled and although she remembers her 1st name she does not remember last name is Howrigan -she mumbles a name that dtr said sounded like 2nd husband that she has not talked about in over 20 years. She is very sensitive, even to point that touching her is painful. She has arthritis of most joints that cause her pain. Noted today when RN attempted to move her legs to examine her feet. She cried out and jerked back - she did allow examination after a short time. Pain is controlled with Tylenol at this time and daughter does not want opioids given at this time.
- Symptoms:

Diagnoses/Codes

- G31.1 - Senile degeneration of brain, not elsewhere classified

Treatments to Date:

Other Health Problems:

Other Comorbid: Dementia, Osteoporosis, Osteoarthritis, hypothyroidism, essential tremors, glaucoma, alopecia.

Medications

Order Date	Start Date	DC Date	Medication	Dosage	Instructions	Covered	Reasons
2025-03-31			Ammonium Lactate	1 Cream TOPICAL 1 times a day (12 % Cream) apply to skin after shower daily		NO	
2025-03-31			Cosopt PF	1 Drop(s) OPHTHALMIC 1 times a day (2-0.5 % Drop(s)) 1 gtt ea eye at bedtime		NO	
2025-03-31			Vitamin D3	1 Capsule ORAL 1 times a day (25 MCG (1000 UT) Capsule)		NO	
2025-03-31			Multivitamin	1 Tablet ORAL 1 times a day (Tablet)		NO	
2025-03-31			Loratadine	1 Tablet ORAL 1 times a day (10 MG Tablet)		NO	
2025-03-31			Loperamide HCl	1 Tablet ORAL every 4 hours As Needed for Diarrhea (2 MG Tablet)		YES	Diarrhea
2025-03-31			Tums	1 - 2 Tablet Chewable ORAL every 4 hours As Needed for Indigestion (500 MG Tablet Chewable)		YES	Indigestion
2025-03-31			Oyster Shell Calcium	1 Tablet ORAL 1 times a day (500 MG Tablet)		NO	
2025-03-31			vitamin B12	1 Tablet ORAL 1 times a day (1000 mcg Tablet)		NO	
2025-03-31			Hydrocortisone	1 Cream TOPICAL 2 times a day As Needed for Itching (1 % Cream)		NO	Itching
2025-03-31			Ibuprofen	1 Tablet ORAL 3 times a day As Needed for Pain (800 MG Tablet)		YES	Pain
2025-03-31			Senna	1 Tablet ORAL 1 times a day As Needed for Constipation (8.6 MG Tablet) 1 tab po qd		YES	Constipation
2025-04-29			Acetaminophen Extra Strength	15 ml ORAL 2 times a day for Pain Non-Acute (1000 MG/30ML ml) Administer 15mL PO BID for Chronic Pain		YES	Pain Non-Acute
2025-04-29			MiraLax	1 Powder ORAL As Needed for Constipation (17 GM/SCOOP Powder)		NO	Constipation
2025-08-01			Diclofenac Sodium	1 ml TOPICAL (1 % ml) apply small amount to lower back and hips twice daily for pain		NO	
2025-08-01			Latanoprost	1 Solution OPHTHALMIC (0.005 % Solution) instill 1 drop to both eyes every night		NO	

Order Date	Start Date	DC Date	Medication	Dosage	Instructions	Covered	Reasons
2025-08-01			Levothyroxine Sodium	1 Tablet ORAL (75 MCG Tablet) take one tablet by mouth daily		NO	
2025-08-01			MiraLax	1 Packet ORAL (17 GM Packet) mix one capful in an 8oz of liquid of choice and drink by mouth every day as needed for constipation		YES	
2028-04-29			ATIVAN / BENADRYL / HALDOL	1 Cream TOPICAL As Needed for Agitation/Restlessness (1 dose Cream) APPLY 1ML TO TO INNER WRIST EVERY 4 HOURS AS NEEDED FOR AGITATION/RESTLESSNESS		YES	Agitation/Restlessness

Medication Notes

Effective Date - 4/25/25
Physician - patient uses tylenol extra strength 500/30ml. unable to find in consolo
Priority -
Reminder -
Days -
Patient in Pain? - false
If yes, CM notified of pain/symptom? - false
Pain Intensity -

Physician Orders

Order Date	Time of Event	Orders	Physician	Clinician	Oversight	Read Back	Non Verbal Order	E Prescription	Medications
8/27/25	1747	May change patient to pleasure feed, regular diet mechanical soft	Adams, John	Berglund, Christi	No	Yes	No	No	
8/25/25	12:19	HHA schedule	Adams, John	Ndah, Bih	No	No	Yes	No	
8/1/25	11:13	Discontinue Medication: Xalatan 1 Drop(s) OPHTHALMIC 1 times a day (0.005 % Drop(s)) Discontinue Medication: Synthroid 1 Tablet ORAL 1 times a day (75 MCG Tablet) Discontinue Medication: Voltaren 1 Dose TOPICAL every 12 hours As Needed for Pain (1 % Dose)	Adams, John	Ndah, Bih	No	No	Yes	No	Xalatan, Synthroid, Voltaren, Diclofenac Sodium, Latanoprost, Levothyroxine Sodium, MiraLax
4/29/25	16:54	Schedule	Adams, John	Ndah, Bih	No	No	Yes	No	

4/29/25	16:41		Adams, John	Ndah, Bih	No	No	Yes	No	MiraLax
4/29/25	1450	Discontinue Medication Tylenol PM Extra Strength 1 ml ORAL 2 times a day (1000-50 MG/30ML ml) on 04/29/2025	Adams, John	Roberts, Dylan	No	No	Yes	No	
4/29/25	14:49		Adams, John	Roberts, Dylan	Yes	No	Yes	No	Acetaminophen Extra Strength
4/29/25	10:00		Adams, John	Ndah, Bih	No	No	Yes	No	ATIVAN / BENADRYL / HALDOL
4/25/25	14:10	Discontinue wound care ordersDiscontinue Medication: MiraLax 1 Dose ORAL 1 times a day As Needed for Constipation (17 GM/SCOOP Dose) Discontinue Medication: Wound Cleanser 1 Liquid TOPICAL (Liquid) Discontinue Medication: Hydrogel 1 Ointment TOPICAL (Ointment)	Adams, John	Ndah, Bih	No	No	Yes	No	MiraLax, Wound Cleanser, Hydrogel
4/21/25	1232	Cleanse right forearm wound with wound cleanser. Apply hydrogel and keep open to air. Wound care 2x weekly by hospice nurse or when soiled by facility	Adams, John	Ndah, Bih	No	No	Yes	No	
4/10/25	02:47	Right forearm skin tear Cleanse with wound cleanser, pat dry put hydrogel, cover with silicone, change 2x weekly weekly by hospice nurse or when soiled by facility	Adams, John	Ndah, Bih	No	No	Yes	No	Hydrogel, Wound Cleanser
3/31/25	22:51	Admit to Trinity Hospice on 3/31/2025 for: Dr John Adams DX: Senile Degeneration of brain Code Status: DNR DME: W/C with cushion, hospital bed with fall pad Incontinent: med PU, wipes, L	Adams, John	Tomlin, Nan	No	Yes	No	No	Synthroid, Ammonium Lactate, Cosopt PF, Xalatan, Vitamin D3, Multivitamin, Loratadine, MiraLax, Loperamide HCl, Tums, Oyster Shell Calcium, Vitamin B12,

gloves, Barrier
Cream, chux Diet:
Regular diet as
tolerated
Medications: see
below May crush
meds if needed
Allergies: NKDA
Ht: 65 in Wt 96lbs
MAC 20 cm RUE
PPS-30 KPS 30
FAST 7c
Treatments: total
dependence of
5/6 ADLs able to
feed self May
check pulse ox
per visit and PRN
for SOB/anxiety
Mobility: Up as
tolerated SN: 1 x
week with 2 PRN
visits every seven
days for
medication or
symptom
management
CNA: 3 times
weekly for
personal care,
hygiene and
safety Social
worker: eval and
1 x month with 1
PRN visits every
30 days for
psycho-social
needs Chaplain:
eval and 1 x
month with 1 PRN
visits Volunteer:
TBD FH: TBD MD
or RN to
pronounce at time
of death Call
Trinity Hospice to
notify of death
469-726-4402
Hospice Nurse will
follow up with
Medical Examiner
and Funeral Home
Hospice MD to
sign death
certificate
Contact Trinity
Hospice at 469-
726-4402 for any
change of
condition,
transfer, fall,
infection or death.
Also contact
Hospice before
any labs or xrays
are completed,
along with any
possible transfer
to Hospital/ER.

Hydrocortisone,
Ibuprofen,
Voltaren,
Tylenol PM
Extra Strength,
Senna

Visit Task Lists

Role Name: Hospice Aide
Effective Date: 2025-03-31

Tasks

Ambulatory with Assistance W/C every visit 3 x week

Chair as Tolerated every visit 3 x week

Durable Medical Equipment present hosp bed with floor pad check every visit 3xwk

Fall Precautions 2nd to weakness- remind pt to ask for help getting up every visit 3 x week

Visit Task List Description: Communication

Role Name: Hospice Aide
Effective Date: 2025-03-31

Tasks

Communicate with FACILITY staff on arrival and departure every visit 3 x wk

Notify RNCM for any assigned task not completed, change in condition, or new skin issues every visit 3 x week

Notify RNCM if no BM for 3 days every visit 3 x week

Visit Task List Description: Elimination

Role Name: Hospice Aide
Effective Date: 2025-03-31

Tasks

Barrier Products in Use every visit 3xwk

Bathroom Assistance every visit 3 x week

Incontinence Pads/Adult briefs (M PU} every visit 3 x week

Incontinent with pericare done past each incidence every visit 3 x week

Visit Task List Description: Nutrition

Role Name: Hospice Aide
Effective Date: 2025-03-31

Tasks

Diet reg check amount consumed every visit 3 x wk

Encourage Fluids every visit 3 x week

Visit Task List Description: Personal Care

Role Name: Hospice Aide
Effective Date: 2025-03-31

Tasks

Clothing Assist every visit 3 x week

Foot Care every visit 3 x weeks

- Nail Care every visit 3 x week use emory board every visit as needed notify CM when fingernails need clipping
- Oral Care every visit 3 x week
- Perineal Care everyvisit 3 x week
- Shampoo every visit 3 x week
- Shower every visit 3 x wk
- Skin Care every visit 3 x week use rx lotion after shower

Clinical Charts

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
08/25/2025 02:52 PM (CDT)	Ndah, Bih	Skilled Nurse

Encounter Type
RN Weekly Comprehensive (v1)

Signatures

Skilled Nurse **Ndah, Bih, RN** (User) signed on **08/27/2025**. Recorded by bndah on 08/27/2025 09:40:48.

Not Resolved

Care Plan Problem Details

Goal

Start Date	End Date	Last Updated	Participants
Source	Category	Outcome Completed	Engagement Engaged

Care Plan Problem Interventions

Assess skin and monitor for skin breakdown on every visit

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Monitor for signs of discomfort every visit

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Teach patient/CG on measures to prevent skin breakdown <SPECIFY>

Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Teach patient/CG regarding: skin care; positioning; signs of infection

Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
08/25/2025 02:52 PM (CDT)	08/25/2025 03:30 PM (CDT)	RN Comprehensive Assess

Vital Sign Measurement #1

Blood Pressure

Blood Pressure - Systolic / Diastolic
1022/61

Oxygen

Oxygen Saturation On
RA

Respiration
16

Pulse

Radial Pulse	Pulse Quality
82	Normal

Temperature and Blood Sugar

Temperature
97.2

Skin Vitals

Skin Color	Skin Moisture	Skin Temperature
Pallor	Dry	Cold

Performance Scales

Palliative Performance Scale Current
30

Patient Measurements

Mid-Arm Circumference (cm)	Mid-Arm Circumference Arm
21	Right

Pain Observation #1

General

Scale Type	Pain Type	Other Indications of Pain
PAINAD	C-Chronic	

Pain 3-6 Months Ago - 0 - No Pain



PAINAD Details

Reason unable to answer	PAINAD Breathing	PAINAD Negative Vocalization
Cognitive defect		
PAINAD Facial Expression	PAINAD Body Language	PAINAD Consolability

Comprehensive

Was Comprehensive Pain Assessment Done?	Location	Severity	
Yes	generalized	no pain	
Character	Duration	Frequency	Pain is relieved by
no pain	chronic	O-Occasional	tylenol

Respiratory

Shortness of Breath

Shortness of Breath



No

Respiratory Findings

Character

☒ CTA

Other

Cardiovascular

Cardiac / Circulatory

Regular

☒ yes

Comments

WNL

Gastrointestinal

Elimination

Incontinent

Yes

Incontinence Type

Both

Bowel

Last Bowel Movement Date

08/25/2025

Present Bowel Pattern

☒ regular

Bowel Sounds Notes

audible x4

Abdominal Assessment

nontender

Additional Findings & Comments

Adult Briefs / Pull-Ups

Yes

Skin

General

Color

☒ pale

Skin Turgor

☒ fair

Bruises

☒ generalized

Moisture

☒ dry

Nutrition \ Hydration

Nutrition / Hydration

Nausea Intensity -
None



Vomiting Intensity -
None



Dysphagia Intensity -
None



Diet

Diet
Mechanical Soft

Appetite / Amount Consumed
FAIR

Additional Findings & Comments

Musculoskeletal

Activities in Daily Living

Feeding	Dressing	Bathing	Toileting
2-Moderate Assist / Limited Assistance	4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence
Ambulation	Transfers		
4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence		

Mobility

Mobility Current
4-Complete Assist / Total Dependence

Summary

ADL Support/Care Provided By
facility staff/HHA

Neurological

Level of Consciousness / Presentation

- ☒ Confused
- ☒ Disoriented

Neurology

- Coordination

☒ bedbound
- Balance

☒ bedbound

Psychiatric

General

Hours Asleep in 24 Current
18

Comments and Intensities

Comments
no issues or concerns

Communication
Communication

Ability to Communicate Current
Word Salad

Nursing Summary

Nursing Summary Notes

Visit Notes/Ongoing Comprehensive Assessment

Weekly comprehensive visit for 91 y/o female admitted to Hospice services for Senile degeneration of the brain. PMH: Dementia, Osteoporosis, Osteoarthritis, hypothyroidism, essential tremors, glaucoma, alopecia. patient was in the common area sleeping on the chair upon arrival. No issues or concerns reported by staff. Disease progression is appropriate. update given to daughter Teresa. she was advised to take ABH gel to patient appointment the next day to help with agitation during doctors visit.

Plan

continue poc

Visit Frequency Discussion

Discussed Visit Frequency

weekly

Discharge Planning

Discussion

when goals are met

Supervisory Visit

Supervisory Visit General Details

Form Completed By

Ndah, Bih

Visit Type

Assisted Living *

Hospice Aide Care

Hospice Aide
Name

jessica

Type of
Supervision

Indirect

Follow-Through
on the Plan of
Care Has Been

Good

Goals Have
Been Met on a
Consistent Basis

Yes

Care Quality
According To

Facility Staff

Creates
Successful
Interpersonal
Relationships
with
Patient/Family

Yes

Complies with
Infection Control
Policies and
Procedures

Yes

Reports
Changes in the
Patient
Condition

Yes

Comments

notify SN with changes to condition

Nursing Related Observation

Emergency Preparedness

Disaster Acuity

CLASS3

Safety / Security

Universal Precautions Followed

Handwashing

Yes

Gloves Worn

Yes

Environment
Assessed for
New Safety
Needs

Yes

Other

☒ none

Coordination of Care

Coordination of Care

RN/CM

Caregiver

☒ bih Ndah

☒ Teresa

Discussion

Discussion

ongoing with family and facility

Medication Record Reviewed

Yes

Teaching & Learning

General Teaching and Learning Assessment Details

Taught by

Ndah, Bih

Taught to

Topics

☒ Changing of Diaper/Brief

☒ Pain management techniques

☒ Symptom Management

Fields Taught

Family/Caregiver

Yes

Response

understood

General Comments

none

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
08/21/2025 10:28 AM (CDT)	Ndah, Bih	Skilled Nurse

Encounter Type

Phone Call (non-telehealth) (ADV) (v1)

Signatures

Skilled Nurse **Ndah, Bih, RN** (User) signed on **08/25/2025**. Recorded by bndah on 08/25/2025 10:39:37.

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
08/21/2025 10:28 AM (CDT)	08/21/2025 10:40 AM (CDT)	PhoneCall

Visit Note

General Visit Note Information

Visit Type

Phone Call

Note

updated daughter on most recent visit. She reports she is working on moving patient out of facility and is looking at new places. Reasons being patient was left unchanged for over 13 hours some nights. Advised to update trinity hospice for assistance to ensure smooth transition. IDG team noted. HHA shenna and Jessica notified. Alternative to provide resources for facilities also offered.

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
08/20/2025 02:05 PM (CDT)	Ndah, Bih	Skilled Nurse

Encounter Type

RN Recertification Visit (v1)

Signatures

Skilled Nurse **Ndah, Bih, RN** (User) signed on **08/25/2025**. Recorded by bndah on 08/25/2025 09:50:40.

Not Resolved

Care Plan Problem Details

Goal

Start Date	End Date	Last Updated	Participants
Source	Category	Outcome Completed	Engagement Engaged

Care Plan Problem Interventions

Assess skin and monitor for skin breakdown on every visit

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Monitor for signs of discomfort every visit

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Teach patient/CG on measures to prevent skin breakdown <SPECIFY>

Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Teach patient/CG regarding: skin care; positioning; signs of infection

Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Not Resolved

Care Plan Problem Details

Goal			
Start Date	End Date	Last Updated	Participants
Source	Category	Outcome	Engagement
		No Response	No Response

Care Plan Problem Interventions

<i>Assess safety in home for DME use</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Assess safety in transfers</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Assess strength, level of activity tolerance, ability to perform ADLs</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Evaluate need for assistive devices and make arrangements</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Hospice Aide to assist with ADLs</i>		
Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged
<i>Instruct patient/CG on prevention of injury with activity (specify)</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Teach patient/CG on comfort measures (specify)</i>		
Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Teach patient/CG on proper use of DME for safety

Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Not Resolved

Care Plan Problem Details

Goal			
Start Date	End Date	Last Updated	Participants
Source	Category	Outcome	Engagement
		No Response	No Response

Care Plan Problem Interventions

<i>Assess/monitor for signs of infection</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Assess/monitor for skin breakdown related to incontinence</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Assess/monitor for: urinary retention; urinary frequency; anuria; polyuria</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Assess/monitor urinary status on every visit</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Teach patient/CG on medication administration</i>		
Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged
<i>Teach patient/CG regarding: comfort and symptom control ; skin care; disease process</i>		
Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Not Resolved

Care Plan Problem Details

Goal

Start Date	End Date	Last Updated	Participants
Source	Category	Outcome Completed	Engagement Engaged

Care Plan Problem Interventions

Assess/monitor for mental status changes every visit (i.e. confusion; agitation; restlessness)

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Provide alternative methods of communication: writing; pictures

Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Provide reality orientation on every visit

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
08/20/2025 02:05 PM (CDT)	08/20/2025 02:50 PM (CDT)	Re-Certification

Vital Sign Measurement #1

Blood Pressure

Blood Pressure - Systolic / Diastolic	Blood Pressure Position
117/69	Sitting

Oxygen

Oxygen Saturation

96

Oxygen Saturation On

RA

Respiration

16

Pulse

Radial Pulse	Pulse Quality
89	Weak

Temperature and Blood Sugar

Temperature	Method
97	Temporal

Skin Vitals

Skin Color	Skin Moisture	Skin Temperature
Pallor	Dry	Warm

Performance Scales

FAST	Palliative Performance Scale Current	Palliative Performance Scale 3-6 Months Ago
7a	30	30

Performance Scale Notes

Unchanged

Patient Measurements

Mid-Arm Circumference (cm)

22

Mid-Arm Circumference Arm

Right

Constitutional

General Appearance

Orientation

☒ to self

Alertness

☒ awake

Respiratory

Shortness of Breath

Shortness of Breath



No

Respiratory Findings

Character

☒ CTA

Skin Color

☒ pale

Other

Comments

WNL

Cardiovascular

Cardiac / Circulatory

Pedal Pulses

☒ weak

Comments

WNL

Gastrointestinal

Elimination

Incontinent

Yes

Incontinence Type

Both

Hx of UTI

☒ yes

Bowel

Abdomen/Bowel
Notes

☒ nontender

Last Bowel
Movement Date

08/20/2025

Present Bowel
Pattern

☒ regular

Usual Bowel
Pattern

☒ regular

Bowel Sounds Notes

audible

Additional Findings & Comments

Adult Briefs / Pull-Ups

Yes

Skin

General

Color

☒ pale

Skin Turgor

☒ poor

Comments

WNL

Musculoskeletal

General

Level of Overall Function on Admission

4-Complete Assist / Total Dependence

Activities in Daily Living

Feeding

3-Maximum Assist / Extensive Assistance

Dressing

4-Complete Assist / Total Dependence

Dressing 3-6 Months Ago

4-Complete Assist / Total Dependence

Bathing

4-Complete Assist / Total Dependence

Bathing 3-6 Months Ago

4-Complete Assist / Total Dependence

Toileting

4-Complete Assist / Total Dependence

Toileting 3-6 Months Ago

4-Complete Assist / Total Dependence

Ambulation

4-Complete Assist / Total Dependence

Transfers

4-Complete Assist / Total Dependence

Transfers 3-6 Months Ago

4-Complete Assist / Total Dependence

Mobility

Fall Risk

Yes

See Morse Fall Assessment

Yes

Mobility Current

4-Complete Assist / Total Dependence

Mobility 3-6 Months Ago

4-Complete Assist / Total Dependence

Summary

ADL Support/Care Provided By

facility staff/HHA

Neurological

Level of Consciousness / Presentation

☒ Confused

☒ Disoriented

☒ Interactive

Communication

Communication

Patient Responds To

to name and nonverbal stimuli

Comments

patient is unable to make needs known

Ability to Communicate Current	Ability to Communicate 3-6 Months Ago
Word Salad	Word Salad

Psychiatric

General

Agitation/Restlessness 3-6 Months Ago	Agitation/Restlessness Current	Alertness/Attention 3-6 Months Ago
Calm	Intermittent Agitation	Disoriented, Confused
Alertness/Attention Current	Hours Asleep in 24 3-6 Months Ago	Hours Asleep in 24 Current
Disoriented, Confused	14	10
Interactivity 3-6 Months Ago	Interactivity Current	
Tires easily, Wants to be left alone	Tires easily, Wants to be left alone	

Comments and Intensities

Agitated Intensity - None	Depressed Intensity - None
★	★

Pain Observation #1

General

Scale Type

PAINAD

Pain Type

C-Chronic

Is Pain an Active Problem for the Patient?

Yes

Other Indications of Pain

Pain 3-6 Months Ago - 0 - No Pain



PAINAD Details

Reason unable to answer

Cognitive defect

PAINAD Breathing



PAINAD Negative Vocalization



PAINAD Facial Expression



PAINAD Body Language



PAINAD Consolability



Comprehensive

Was Comprehensive Pain Assessment Done?

Yes

Location

generalized

Severity

no pain

Character

no pain

Duration

chronic

Frequency

O-Occasional

Pain is relieved by
tylenol

Nutrition \ Hydration

Nutrition / Hydration

Nausea Intensity -
None



Vomiting Intensity -
None



Dysphagia Intensity -
None



Diet

Diet
As Tolerated

Appetite / Amount Consumed
GOOD

Additional Findings & Comments

Morse Fall

Morse Fall Assessment General Details

Ambulatory Aid - Bedrest/Nurse
Assist



Gait/Transferring -
Normal/Bedrest/Immobile



Edmonton Assessment (ESAS)

Edmonton Assessment

Pain Level

1

Tiredness Level

1

Drowsiness Level

2

Nausea Level

Lack of Appetite Level

1

Shortness of Breath Level

Depression Level

1

Anxiety Level

Wellbeing Level

6

Constipation Level

Completed By

☒ Patient

☒ Healthcare Professional Caregiver

Nursing Summary

Nursing Summary Notes

Visit Notes/Ongoing Comprehensive Assessment

Recertification visit for 91 y/o female admitted to Hospice services for Senile degeneration of the brain. PMH: Dementia, Osteoporosis, Osteoarthritis, hypothyroidism, essential tremors, glaucoma, alopecia. Patient was having activity upon visit. Assessment was completed in private. Patient appetite has decreased, she eating about 50% of her meals and now she eats about 25%. Patient has a decline in sleep patterns, she used to sleep during the night and now she has trouble staying asleep. Family has reported patient pulling herself to the floor several times during the night time. Patient's agitation has improved, staff denies agitation during incontinent care. No PRN medication for agitation has been administered since last BP. Patient has had skin irritations and flare up. The skin growth in chest area has increased and color change to dark. Dermatologist visit scheduled by daughter for week of 8/25/25. Staff denies having concerns. Advised to call with concerns

Changes In Condition and Progress Towards Goals

decrease staying asleep during the night

improved agitation without needing medication

decrease appetite

Patient/Family/Representative/CG Goals

patient will remain comfortable as disease progresses

Plan

continue poc

Coordination of Care

Coordination of Care

RN/CM	SW	Chaplain	Medical Director
☒ Bih Ndah	☒ Marissa Mosley	☒ David Parish	☒ Dr Adams

Caregiver

☒ Teresa

Discussion

Discussion

ongoing with family and facility

Medication Record Reviewed	POC Updated
Yes	☒ current

Nursing Related Observation

Emergency Preparedness

Disaster Acuity

CLASS3

Safety / Security

Universal Precautions Followed

Handwashing	Gloves Worn	Environment Assessed for New Safety Needs
Yes	Yes	Yes

Supervisory Visit

Supervisory Visit General Details

Form Completed By

Ndah, Bih

Visit Type

Assisted Living *

Hospice Aide Care

Hospice Aide Name	Type of Supervision	Follow-Through on the Plan of Care Has Been	Goals Have Been Met on a Consistent Basis
Shenna	Indirect	Excellent	Yes
Care Quality According To Facility Staff	Creates Successful Interpersonal Relationships with Patient/Family	Complies with Infection Control Policies and Procedures	Reports Changes in the Patient Condition
	Yes with Feedback	Yes with Feedback	Yes with Feedback

Teaching & Learning

General Teaching and Learning Assessment Details

Taught by

Ndah, Bih

Taught to

Topics

- ☒ Changing of Diaper/Brief
- ☒ Diet/Nutrition
- ☒ Symptom Management

Fields Taught

General Comments

none

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
08/19/2025 10:05 AM (CDT)	Parish, David	Chaplain

Encounter Type

CH Spiritual Visit Note (ADV) (v1)

Signatures

Chaplain **Parish, David** (User) signed on **08/20/2025**. Recorded by dpari on 08/20/2025 09:36:27.

Not Resolved

Care Plan Problem Details

Goal			
Start Date	End Date	Last Updated	Participants
Source	Category	Outcome Effective	Engagement Engaged

Care Plan Problem Interventions

Provide prayer and scripture reading

Intervention Performed	Outcome	Engagement
Yes	Effective	Engaged

Patient Time

Patient Time

In date/time	Out date/time	Travel start time
08/19/2025 10:05 AM (CDT)	08/19/2025 10:35 AM (CDT)	10:01 AM (CDT)

Travel stop time	Clinical Care Type
10:04 AM (CDT)	Regular Visit

Visit Note

General Visit Note Information

Visit Type

Nursing Home

Note

Chaplain visited pt and was greeted warmly by facility staff. Chaplain encountered patient in the common area watching TV with other residents. Pt is well groomed and appears to be receiving excellent care. Patient is nonverbal; speaking only Word salad. Patient was in a pleasant mood and presenting no indications of spiritual distress or excessive anxiety. Chaplain prayed with patient and will attempt to contact primary caregiver / family member. Chaplain will continue to visit this patient two times per month providing Spiritual Care & Support to Patient and Family.

Spiritual Concerns

Spiritual Concerns (Progress, New Concerns)

There are no new spiritual concerns at this time.

Assessment / Insight

Pt is well groomed and appears to be receiving excellent care.

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit

08/16/2025 11:40 AM (CDT)

Owner

Whitener, Pam

Discipline

Physician

Encounter Type

Face to Face Encounter (v1)

Signatures

Nurse Practitioner **Whitener, NP, Pam** (Nurse Practitioner) signed on **08/17/2025**. Recorded by pwhit on 08/17/2025 18:43:43.

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
08/16/2025 11:40 AM (CDT)	08/16/2025 12:10 PM (CDT)	Face to Face

Face to Face Visit

General Face to Face Visit Details

Physician

Whitener, Pam

Comments

A Face-to-Face assessment was performed with Katherine Howrigan (91 y/o) who resides in the Memory Care Facility @ Edison. She is receiving hospice services for Senile Degeneration of the Brain. Her co-morbidities include Osteoporosis, Osteoarthritis, Hypothyroidism, essential tremors, glaucoma, and Alopecia. She was sitting in the common area, asleep in a wingback chair. She has her hat on and wrapped in a blanket. She stays cold all the time. She was easily aroused, allowed me to assess her and take her vital signs. She was calm and cooperative. CV/Resp: HR irregular, respirations are CTA, diminished at bases, no edema, no cough or congestion. GI/GU: cachetic appearance, incontinent. Staff changes her diapers 3-4 x/daily. She is able to feed herself with much encouragement staff stated. She tires easily when feeding. Skin: Alopecia. M/S: (+) muscle wasting, no ambulation, staff transfers her to wheelchair. She makes no attempt to get out of chair w/o assistance. Neuro: sleeps 12-16 hour daily and naps throughout the day in the chair. She is frail, continues to decline, meets the criteria for hospice services. She is terminal.

BP 96/62 HR 70 R 14 02 Sat 98% Temp 96.6
MAC 18.5 cm (L)
Karnofsky 30 Palliative 30

Patient Document

Documents

Face to Face Physician Attestation

I confirm that I had a face-to-face encounter with Howrigan, Katherine A on 08/16/2025 and that the clinical findings of that encounter have been provided to the certifying physician for use in determining continued eligibility for hospice care.

Signature: _____ Date: _____

Nurse Practitioner **Whitener, NP, Pam** (Nurse Practitioner) signed on **8/17/25** . Recorded by Whitener, Pam on 08/17/2025 06:43 PM (CDT)

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
08/14/2025 10:00 AM (CDT)	Ndah, Bih	Skilled Nurse

Encounter Type

RN Weekly Comprehensive (v1)

Signatures

Skilled Nurse **Ndah, Bih, RN** (User) signed on **08/15/2025**. Recorded by bndah on 08/15/2025 13:45:03.

Not Resolved

Care Plan Problem Details

Goal

Start Date	End Date	Last Updated	Participants
Source	Category	Outcome Effective	Engagement Engaged

Care Plan Problem Interventions

Assess skin and monitor for skin breakdown on every visit

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Monitor for signs of discomfort every visit

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Teach patient/CG on measures to prevent skin breakdown <SPECIFY>

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Teach patient/CG regarding: skin care; positioning; signs of infection

Intervention Performed	Outcome	Engagement
Yes	Completed	Engaged

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
08/14/2025 10:00 AM (CDT)	08/14/2025 10:40 AM (CDT)	RN Comprehensive Assess

Vital Sign Measurement #1

Blood Pressure

Blood Pressure - Systolic / Diastolic
107/67

Oxygen

Oxygen Saturation On
RA

Respiration
16

Pulse

Radial Pulse	Pulse Quality
72	Weak

Temperature and Blood Sugar

Temperature
96.7

Skin Vitals

Skin Color	Skin Moisture	Skin Temperature
Pallor	Dry	Warm

Performance Scales

Palliative Performance Scale Current
30

Patient Measurements

Mid-Arm Circumference (cm)	Mid-Arm Circumference Arm
22.5	Right

Pain Observation #1

General

Scale Type

PAINAD

Pain Type

C-Chronic

Other Indications of Pain

Pain 3-6 Months Ago - 0 - No Pain



PAINAD Details

Reason unable to answer

Cognitive defect

PAINAD Breathing

★

★

PAINAD Negative Vocalization

★

★

PAINAD Facial Expression

★

PAINAD Body Language

★

PAINAD Consolability

★

Comprehensive

Was Comprehensive Pain Assessment Done?

Yes

★

Location

gen

★

Severity

no pain

★

Character

no pain

★

Duration

chronic

★

Frequency

O-Occasional

★

Pain is relieved by

tylenol

★

Respiratory

Shortness of Breath

Shortness of Breath



No

Respiratory Findings

Character

☒ CTA

Other

Cardiovascular

Cardiac / Circulatory

Comments

WNL

Gastrointestinal

Bowel

Last Bowel Movement Date

08/13/2025

Bowel Sounds Notes

audible x4

Abdominal Assessment

nontender

Skin

General

Color

☒ pale

Skin Turgor

☒ poor

Moisture

☒ dry

Nutrition \ Hydration

Nutrition / Hydration

Nausea Intensity -
None



Vomiting Intensity -
None



Dysphagia Intensity -
None



Diet

Diet
As Tolerated

Appetite / Amount Consumed
GOOD

Additional Findings & Comments

Musculoskeletal

Activities in Daily Living

Feeding	Dressing	Bathing	Toileting
2-Moderate Assist / Limited Assistance	4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence
Ambulation	Transfers		
4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence		

Mobility

Mobility Current
4-Complete Assist / Total Dependence

Summary

ADL Support/Care Provided By
facility staff/HHA

Neurological

Level of Consciousness / Presentation

☒ Awake/Alert

☒ Confused

☒ Disoriented

Neurology

Coordination

☒ chairbound

Balance

☒ chairbound

Psychiatric

General

Hours Asleep in 24 Current

16

Comments and Intensities

Comments

no issues or concerns reported

Communication

Communication

Ability to Communicate Current

Word Salad

Nursing Summary

Nursing Summary Notes

Visit Notes/Ongoing Comprehensive Assessment

Weekly comprehensive visit for 90 y/o female admitted to Hospice services for Senile degeneration of the brain. PMH: Dementia, Osteoporosis, Osteoarthritis, hypothyroidism, essential tremors, glaucoma, alopecia. patient was outside participating in activity upon arrival. Patient was taken to a private area to completed assessment after one activity was over and was brought back to complete the next activity. no distress or s/s of pain noted. in grown skin tag on chest, protruding, will watch and wait. no other issues or concerns reported

Plan

continue poc

Visit Frequency Discussion

Discussed Visit Frequency

weekly

Discharge Planning

Discussion

when goals are met

Supervisory Visit

Supervisory Visit General Details

Form Completed By

Ndah, Bih

Visit Type

Assisted Living *

Hospice Aide Care

Hospice Aide
Name

jessica

Type of
Supervision

Indirect

Follow-Through
on the Plan of
Care Has Been

Good

Goals Have
Been Met on a
Consistent Basis

Yes

Care Quality
According To

Facility Staff

Creates
Successful
Interpersonal
Relationships
with
Patient/Family

Yes with Feedback

Complies with
Infection Control
Policies and
Procedures

Yes with Feedback

Reports
Changes in the
Patient
Condition

Yes with Feedback

Comments

notify SN with changes to condition

Nursing Related Observation

Emergency Preparedness

Disaster Acuity

CLASS3

Safety / Security

Universal Precautions Followed

Handwashing

Yes

Gloves Worn

Yes

Environment
Assessed for
New Safety
Needs

Yes

Other

☒ none

Coordination of Care

Coordination of Care

RN/CM

Caregiver

☒ Bih Ndah

☒ Teresa

Discussion

Discussion

ongoing with daughter and facility

Medication Record Reviewed

Yes

Teaching & Learning

General Teaching and Learning Assessment Details

Taught by

Ndah, Bih

Taught to

Topics

☒ Diet/Nutrition

☒ Incontinence Care

☒ Skin Care

Fields Taught

Family/Caregiver

Yes

Response

understood

General Comments

none

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
08/12/2025 10:35 AM (CDT)	Parish, David	Chaplain

Encounter Type

CH Spiritual Visit Note (ADV) (v1)

Signatures

Chaplain **Parish, David** (User) signed on **08/17/2025**. Recorded by dpari on 08/17/2025 19:11:37.

Not Resolved

Care Plan Problem Details

Goal			
Start Date	End Date	Last Updated	Participants
Source	Category	Outcome	Engagement

Care Plan Problem Interventions

<i>Provide prayer and scripture reading</i>		
Intervention Performed	Outcome	Engagement
Yes		

Patient Time

Patient Time

In date/time	Out date/time	Travel start time
08/12/2025 10:35 AM (CDT)	08/12/2025 11:05 AM (CDT)	10:31 AM (CDT)

Travel stop time	Clinical Care Type
10:34 AM (CDT)	

Visit Note

General Visit Note Information

Visit Type

Nursing Home

Note

Chaplain visited pt and was greeted warmly by facility staff. Chaplain encountered patient in the common area watching TV with other residents. Pt is well groomed and appears to be receiving excellent care. Patient is nonverbal; speaking only Word salad. Patient was in a pleasant mood and presenting no indications of spiritual distress or excessive anxiety. Chaplain prayed with patient and will attempt to contact primary caregiver / family member. Chaplain will continue to visit this patient two times per month providing Spiritual Care & Support to Patient and Family.

Spiritual Concerns

Spiritual Concerns (Progress, New Concerns)

There are no new spiritual concerns at this time.

Assessment / Insight

Pt is well groomed and appears to be receiving excellent care.

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit

08/11/2025 11:39 AM (CDT)

Owner

Ndah, Bih

Discipline

Skilled Nurse

Encounter Type

Phone Call (non-telehealth) (ADV) (v1)

Signatures

Skilled Nurse **Ndah, Bih, RN** (User) signed on **08/11/2025**. Recorded by bndah on 08/11/2025 14:34:30.

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
08/11/2025 11:39 AM (CDT)	08/11/2025 11:40 AM (CDT)	

Visit Note

General Visit Note Information

Visit Type

Phone Call

Note

No answer phone call from daughter Teresa. updated daughter on ingrown skin tag via text message(verbal approval to send text message). It is more protruding than before and has increased in size. Will continue to watch and wait.

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
08/06/2025 10:31 AM (CDT)	Tyus, Sheila	LPN/LVN

Encounter Type

NURSE VISIT (v1)

Signatures

Skilled Nurse **Tyus, Sheila, LVN** (User) signed on **08/23/2025**. Recorded by styus on 08/23/2025 15:25:12.

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
08/06/2025 10:31 AM (CDT)	08/06/2025 11:01 AM (CDT)	

Vital Sign Measurement #1

Blood Pressure

Blood Pressure - Systolic / Diastolic
146/84

Oxygen

Oxygen Saturation
 96

Oxygen Saturation On
RA

Respiration
16

Pulse

Radial Pulse
68

Temperature and Blood Sugar

Temperature
97.1

Performance Scales

Palliative Performance Scale Current
30

Pain Observation #1

General

Scale Type

Pain Type

Verbal

Comments

No c/o pain or discomfort except when touched

Other Indications of Pain

Verbal Details

Present Intensity - 0  - No Pain	Best Intensity - 0 - No Pain	Worst Intensity - 0 - No Pain
One Hour Intensity Post-Medicated - 0 - No Pain	Three Hour Intensity Post-Medicated - 0 - No Pain	Verbal Patient Acceptable Level of Pain - 0 - No Pain

Uncomfortable because of pain?

Comprehensive

Respiratory

Shortness of Breath

Respiratory Findings

Respiratory Assessed, No Respiratory Symptoms Identified - NA	Character
Yes	☒ CTA

Other Respiratory Notes

Resp even and unlabored

Other

Cardiovascular

Cardiac / Circulatory

Cardiovascular Assessed, No Cardiac
Symptoms Identified - NA

Yes

Edema Notes

No edema observed or noted

Pedal Pulses

☒ present

Comments

PPPx4 normal

Gastrointestinal

Elimination

Incontinent

Yes

Incontinence Type

Both

Bowel

Last Bowel Movement Date

08/06/2025

Abdominal Assessment

ABD soft, non distended

BSx4

Skin

General

Color

☒ Pallor

Skin Turgor

☒ poor

Edema Notes

No edema observed or noted

Moisture

☒ Dry

Neurological

Level of Consciousness / Presentation

☒ Awake/Alert

☒ Interactive

Neurology

Grasps Equal

☒

Comments

Easily agitated by tactile stimuli

Nursing Summary

Nursing Summary Notes

Visit Notes/Ongoing Comprehensive Assessment

Pt resides in memory care ALF. Pt sitting up in chair upon arrival. Assisted pt to w/c and took to private area for assessment. SN assessment and VS done. No c/o pain or discomfort unless touched then pt yells out. No SOB observed or noted. Pt A&Ox1 with some confusion. No refills needed per staff. No distress noted upon departure. Will continue with current POC. Daughter of patient updated via phone of visit by Bih RNCM.

Visit Frequency Discussion

Discussed Visit Frequency

Current POC

Coordination of Care

Coordination of Care

Discussion

General Details

Patient Information

Howrigan, Katherine A

824 W McLeroy Blvd.

Saginaw TX 76179

Time of Visit

08/05/2025 11:20 AM (CDT)

Owner

Mosley, Marissa

Discipline

Social Worker

Encounter Type

SW Regular Visit (ADV) (v1)

Signatures

Social Worker **Mosley, Marissa, MSW** (User) signed on **08/07/2025**. Recorded by mmosl on 08/07/2025 22:52:09.

Not Resolved

Care Plan Problem Details

Goal			
Start Date	End Date	Last Updated	Participants
Source	Category	Outcome	Engagement

Care Plan Problem Interventions

Inform patient/CG of community resources available to them, coordinating with these resources

Intervention Performed	Outcome	Engagement
Yes		

Promote positive psychosocial environment

Intervention Performed	Outcome	Engagement
Yes		

Provide reassurance and conflict resolution

Intervention Performed	Outcome	Engagement
Yes		

Not Resolved

Care Plan Problem Details

Goal

Start Date	End Date	Last Updated	Participants
Source	Category	Outcome	Engagement
		No Response	No Response

Care Plan Problem Interventions

Assess ongoing bereavement risk. Notify IDG of any potential changes in bereavement risk.

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

LOW RISK: CH and SW frequency minimum once monthly or per patient/family preference to monitor for changes to bereavement risk. RNCM monitor bereavement risk and coordinate with IDG if changes to bereavement risk occur.

Intervention Performed	Outcome	Engagement
No Response	No Response	No Response

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
08/05/2025 11:20 AM (CDT)	08/05/2025 11:40 AM (CDT)	

Visit Note

General Visit Note Information**Visit Type**

Assisted Living *

Note

SW made visit to patient at assisted living where she resides in memory care. Patient seated in common area, watching television. SW greeted patient, who appeared to be in pleasant mood. Patient able to answer with 1-2 words, mostly yes or no questions. Patient did not exhibit signs of acute distress. SW sat with patient to promote socialization and provide supportive presence

Supplementary Visit Note Information/Ongoing Comprehensive Assessment**Goals**

Ensure comfort, support, reassurance. Engage with patient at level of ability to communicate and interact

Assessment

Patient needs appear met by family, facility, and hospice plan of care at this time. Patient appears near and clean, in a pleasant mood. No emergent psychosocial concerns noted.

Plans

SW will continue to follow x2 monthly and PRN

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
07/30/2025 02:36 PM (CDT)	Ndah, Bih	Skilled Nurse

Encounter Type

Care Conference/Care Plan Meeting (v1)

Signatures

Skilled Nurse **Ndah, Bih, RN** (User) signed on **07/31/2025**. Recorded by bndah on 07/31/2025 22:22:10.

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
07/30/2025 02:36 PM (CDT)	07/30/2025 02:39 PM (CDT)	

Coordination of Care

Coordination of Care

RN/CM

☒ Bih Ndah

Family

☒ Patrick

Facility Representative

☒ Ashley Solomon

Discussion

Discussion

Son patrick participated via phone call, His concerns were addressed and he was satisfied with responses

1. Concerns about medication for agitation: Patient has not displayed aggressive behavior for a while, so no interventions to agitation has been used. But antianxiety medication ABH gel present as needed.
2. Concerns for patient appetite and diet: patient is satisfied with food sometimes and lacks appetite other times. Ashley states a weekly menu is given to daughter teres and substitution is made as requested.
3. Concerns for BM: Patient has large BMs at night time and was wondering if something could be done to prevent it. Unable to find a solution at this time, nothing is given to agitate bowels that could be discontinued. possibly a chronic condition or patient's natural system. No abdominal distress or pain is noted during the time so will continue to monitor situation.

Visit Note

General Visit Note Information

Visit Type

Coordination of Care

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
07/30/2025 01:05 PM (CDT)	Ndah, Bih	Skilled Nurse

Encounter Type

RN Weekly Comprehensive (v1)

Signatures

Skilled Nurse **Ndah, Bih, RN** (User) signed on **08/04/2025**. Recorded by bndah on 08/04/2025 09:42:12.

Not Resolved

Care Plan Problem Details

Goal			
Start Date	End Date	Last Updated	Participants
Source	Category	Outcome	Engagement

Care Plan Problem Interventions

<i>Assess safety in home for DME use</i>		
Intervention Performed	Outcome	Engagement
Yes		
<i>Assess safety in transfers</i>		
Intervention Performed	Outcome	Engagement
Yes		
<i>Assess strength, level of activity tolerance, ability to perform ADLs</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Evaluate need for assistive devices and make arrangements</i>		
Intervention Performed	Outcome	Engagement
No Response	No Response	No Response
<i>Hospice Aide to assist with ADLs</i>		
Intervention Performed	Outcome	Engagement
Yes		
<i>Instruct patient/CG on prevention of injury with activity (specify)</i>		
Intervention Performed	Outcome	Engagement
Yes		
<i>Teach patient/CG on comfort measures (specify)</i>		
Intervention Performed	Outcome	Engagement
Yes		

Teach patient/CG on proper use of DME for safety

Intervention Performed	Outcome	Engagement
Yes		

Patient Time

Patient Time

In date/time	Out date/time	Clinical Care Type
07/30/2025 01:05 PM (CDT)	07/30/2025 01:50 PM (CDT)	

Vital Sign Measurement #1

Blood Pressure

Blood Pressure - Systolic / Diastolic
77/51

Oxygen

Oxygen Saturation
 95

Oxygen Saturation On
RA

Respiration
12

Pulse

Radial Pulse	Pulse Quality
76	Weak

Temperature and Blood Sugar

Temperature
97.1

Skin Vitals

Skin Color	Skin Moisture	Skin Temperature
Pallor	Dry	Cool

Performance Scales

Palliative Performance Scale Current
30

Patient Measurements

Weight (pounds)	Mid-Arm Circumference (cm)	Mid-Arm Circumference Arm
98		

21

Right

Pain Observation #1

General

Scale Type	Pain Type	Other Indications of Pain
PAINAD		

Pain 3-6 Months Ago - 0 - No Pain



PAINAD Details

PAINAD Breathing		PAINAD Negative Vocalization		PAINAD Facial Expression	
PAINAD Body Language		PAINAD Consolability			

Comprehensive

Was Comprehensive Pain Assessment Done?		Location		Severity			
Yes		generalized		no pain			
Character		Duration		Frequency		Pain is relieved by	
no pain		no pain				tylenol	

Respiratory

Shortness of Breath

Respiratory Findings

Character

☒ CTA

Other

Cardiovascular

Cardiac / Circulatory

Comments

WNL

Gastrointestinal

Bowel

Last Bowel Movement Date

07/30/2025

Bowel Sounds Notes

audible and active x4

Abdominal Assessment

flat and nontender

Skin

General

Color	Skin Turgor	Moisture
☒ pale	☒ poor	☒ dry and flaky

Nutrition \ Hydration

Nutrition / Hydration

Nausea Intensity - None	Vomiting Intensity - None	Dysphagia Intensity - None
★	★	★

Diet

Diet	Appetite / Amount Consumed
As Tolerated	50%

Additional Findings & Comments

Musculoskeletal

Activities in Daily Living

Feeding	Dressing	Bathing	Toileting
2-Moderate Assist / Limited Assistance	4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence
Ambulation	Transfers		
4-Complete Assist / Total Dependence	4-Complete Assist / Total Dependence		

Mobility

Mobility Current
4-Complete Assist / Total Dependence

Summary

ADL Support/Care Provided By
facility staff/HHA

Neurological

Level of Consciousness / Presentation

- ☒ Awake/Alert
- ☒ Disoriented

Neurology

- Coordination

☒ chairbound
- Balance

☒ chairbound

Psychiatric

General

Hours Asleep in 24 Current

14

Comments and Intensities

Comments

no issues or concerns reported

Communication

Communication

Ability to Communicate Current

Word Salad

Nursing Summary

Nursing Summary Notes

Visit Notes/Ongoing Comprehensive Assessment

Weekly comprehensive visit for 90 y/o female admitted to Hospice services for Senile degeneration of the brain. PMH: Dementia, Osteoporosis, Osteoarthritis, hypothyroidism, essential tremors, glaucoma, alopecia. Patient refused to be moved to a private area for assessment. Assessment completed in common area. Fluid build up noted in right toe area. No other issues or concerns reported. Facility staff advised to call with any questions or concerns. patient has adequate supplies

Plan

continue poc

Visit Frequency Discussion

Discussed Visit Frequency

weekly

Discharge Planning

Discussion

when goals are met

Supervisory Visit

Supervisory Visit General Details

Form Completed By

Ndah, Bih

Visit Type

Assisted Living *

Hospice Aide Care

Hospice Aide
Name

jessica

Type of
Supervision

Indirect

Follow-Through
on the Plan of
Care Has Been

Excellent

Goals Have
Been Met on a
Consistent Basis

Yes

Care Quality
According To

Facility Staff

Creates
Successful
Interpersonal
Relationships
with
Patient/Family

Yes

Complies with
Infection Control
Policies and
Procedures

Yes

Reports
Changes in the
Patient
Condition

Yes

Comments

notify Sn with any changes

Nursing Related Observation

Emergency Preparedness

Disaster Acuity

CLASS3

Safety / Security

Universal Precautions Followed

Handwashing

Yes

Gloves Worn

Yes

Environment
Assessed for
New Safety
Needs

Yes

Other

☒ none

Coordination of Care

Coordination of Care

RN/CM

Caregiver

☒ Bih Ndah

☒ Patrick

Discussion

Discussion

ongoing with family and facility

Medication Record Reviewed

Yes

Teaching & Learning

General Teaching and Learning Assessment Details

Taught by

Ndah, Bih

Taught to

- Topics
- ☒ Call Hospice for Condition Change

☒ Diet/Nutrition

☒ Multiple Topics

☒ Pain management techniques

☒ Symptom Management

Fields Taught

Family/Caregiver

Yes

Response

understood

General Comments

none

General Details

Patient Information

Howrigan, Katherine A
824 W McLeroy Blvd.
Saginaw TX 76179

Time of Visit	Owner	Discipline
07/29/2025 02:45 PM (CDT)	Parish, David	Chaplain

Encounter Type

CH Spiritual Visit Note (ADV) (v1)

Signatures

Chaplain **Parish, David** (User) signed on **08/04/2025**. Recorded by dpari on 08/04/2025 06:05:35.

Not Resolved

Care Plan Problem Details

Goal			
Start Date	End Date	Last Updated	Participants
Source	Category	Outcome	Engagement

Care Plan Problem Interventions

Provide prayer and scripture reading

Intervention Performed	Outcome	Engagement
Yes		

Patient Time

Patient Time

In date/time	Out date/time	Travel start time
07/29/2025 02:45 PM (CDT)	07/29/2025 03:15 PM (CDT)	02:41 PM (CDT)

Travel stop time	Clinical Care Type
02:44 PM (CDT)	

Visit Note

General Visit Note Information

Visit Type

Nursing Home

Note

Chaplain visited pt and was greeted warmly by facility staff. Chaplain encountered patient in the common area watching TV with other residents. Pt is well groomed and appears to be receiving excellent care. Patient is nonverbal; speaking only Word salad. Patient was in a pleasant mood and presenting no indications of spiritual distress or excessive anxiety. Chaplain prayed with patient and will attempt to contact primary caregiver / family member. Chaplain will continue to visit this patient two times per month providing Spiritual Care & Support to Patient and Family.

Spiritual Concerns

Spiritual Concerns (Progress, New Concerns)

There are no new spiritual concerns at this time.

Assessment / Insight

Well groomed

Signatures

Medical Director **Adams, DO, John** (Medical Director) signed on **08/29/2025**. Recorded by jadam on 08/29/2025 10:27:15.
Admin **Taylor-Anyanwu, Debbie, RN** (User) signed on **08/28/2025**. Recorded by dtayl on 08/28/2025 16:41:36.

PHYSICIAN'S TELEPHONE/VERBAL ORDER

Homage Hospice

DATE	12/17/2025
PATIENT NAME	Katherine Howrigan
DOB	06/30/1934
PHYSICIAN NAME	Dr. Diego Restrepo
ADDRESS	
PHONE	
	Macrobid 100 mg. Take 1 capsule twice daily
	Haloperidol 0.5 mg. Take 1 tablet every 6 hours as needed
	for Anxiety
	DC Lorazepam due to hallucinations.

ORDERS RECEIVED BY	
PHYSICIAN SIGNATURE	V.O.R.B Dr. Diego Restrepo Virginia Allan MSN, RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOPE - ADMISSION

Section A: Administrative Information

Patient / Admission Information

A0215. Site of service at admission 01. Patient's Home/Residence
A0220. Admission date 10/15/2025
A0500A. Patient first name KATHERINE
A0500B. Patient middle initial A
A0500C. Patient last name HOWRIGAN
A0550. Patient zip code 76248-2525
A0600A. Social Security Number 048-26-0963
A0600B. Patient Medicare/railroad insurance number 1X83VK6VY96
☒ N. Patient is not a Medicaid recipient
A0810. Sex 2. Female
A0900. Birthdate 6/30/1934

A1005. Ethnicity

☒ A1005A. No, not of Hispanic, Latino/a, or Spanish origin

A1010. Race

☒ A1010A. White

Language

A1110A. Preferred Language English
A1110B. Interpreter requested 0. No

Payor Information

☒ A1400A. Medicare (FFS)

Admitted From

A1805. Admitted From 01. Home/Community (pvt home/apt, board/care, alf, etc)

Living Arrangements

A1905 Living Arrangements 3. Congregate home (alf, residential home care)

Availability of Assistance

A1910 Availability of Assistance 1. Around-the-clock (24 hours a day with few exceptions)

Section F: Preferences

F2000A. Was patient asked about CPR 1. Yes
F2000B. Date asked about CPR 10/15/2025
F2100A. Was patient asked about treatments other than CPR 1. Yes
F2100B. Date asked about treatment other than CPR 10/15/2025
F2200A. Was patient asked about hospitalization 1. Yes
F2200B. Date asked about hospitalization 10/15/2025
F3000A. Was patient asked spiritual/existential concerns 1. Yes
F3000B. Date asked about spiritual/existential concerns 10/15/2025

Section I: Active Diagnoses

I0010. Principal diagnosis 02. Dementia (including Alzheimer's disease)

Comorbidities and Co-existing Conditions

Section J: Health Conditions

Pain

J0900A. Was patient screened for pain 1. Yes
J0900B. Date of first screening for pain 10/15/2025
J0900C. Patient's pain severity was 1. Mild
J0900D. Type of standardized pain tool used 3. Patient visual
J0905. Is pain an active problem for the patient? 0. No

Respiratory Status

J2030A. Was patient screened for shortness of breath 1. Yes
J2030B. Date of first screening for shortness of breath 10/17/2025
J2030C. Did screening indicate the patient had shortness of breath 0. No

Symptom Impact

J2050A. Was a symptom impact screening completed? 1. Yes
J2050B. Date of symptom impact screening 10/15/2025
J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	1. Slight	
B. Shortness of breath	0. Not at all	
C. Anxiety	1. Slight	
D. Nausea	0. Not at all	
E. Vomiting	0. Not at all	
F. Diarrhea	0. Not at all	
G. Constipation	0. Not at all	
H. Agitation	1. Slight	

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions? 0. No

Section N: Medications

N0500A. Was scheduled opioid initiated or continued 0. No
N0510A. Was PRN opioid initiated or continued 0. No

Section Z: Record Administration

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion 10/15/2025

Electronically signed by: VIRGINIA ALLAN RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOPE - UPDATE VISIT

Section A: Administrative Information

A0250. Reason For Record2. HOPE Update Visit 1 (HUV1)

Patient / Admission Information

A0220. Admission date10/15/2025
A0500A. Patient first nameKATHERINE
A0500B. Patient middle initialA
A0500C. Patient last nameHOWRIGAN
A0600A. Social Security Number048-26-0963
A0600B. Patient Medicare/railroad insurance number1X83VK6VY96
☒ N. Patient is not a Medicaid recipient

A0810. Sex2. Female
A0900. Birthdate6/30/1934

Payor Information

☒ A1400A. Medicare (FFS)

Section J: Health Conditions

Symptom Impact

J2050A. Was a symptom impact screening completed?1. Yes
J2050B. Date of symptom impact screening10/21/2025
J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	1. Slight	
B. Shortness of breath	1. Slight	
C. Anxiety	1. Slight	
D. Nausea	0. Not at all	
E. Vomiting	0. Not at all	
F. Diarrhea	0. Not at all	
G. Constipation	0. Not at all	
H. Agitation	0. Not at all	

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions?0. No

Section N: Medications

N0500A. Was scheduled opioid initiated or continued0. No
N0510A. Was PRN opioid initiated or continued1. Yes
N0510B. Date PRN opioid initiated or continued10/21/2025
N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes")2. Yes
N0520B. Date bowel regimen initiated or continued10/21/2025

Section Z: Record Administration

Z0350. Date Assessment was Completed10/21/2025

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for

payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion

11/4/2025

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOPE - UPDATE VISIT

Section A: Administrative Information

A0250. Reason For Record3. HOPE Update Visit 2 (HUV2)

Patient / Admission Information

A0220. Admission date10/15/2025
A0500A. Patient first nameKATHERINE
A0500B. Patient middle initialA
A0500C. Patient last nameHOWRIGAN
A0600A. Social Security Number048-26-0963
A0600B. Patient Medicare/railroad insurance number1X83VK6VY96
☒ N. Patient is not a Medicaid recipient

A0810. Sex2. Female
A0900. Birthdate6/30/1934

Payor Information

☒ A1400A. Medicare (FFS)

Section J: Health Conditions

Symptom Impact

J2050A. Was a symptom impact screening completed?1. Yes
J2050B. Date of symptom impact screening10/31/2025
J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	1. Slight	
B. Shortness of breath	1. Slight	
C. Anxiety	1. Slight	
D. Nausea	0. Not at all	
E. Vomiting	0. Not at all	
F. Diarrhea	0. Not at all	
G. Constipation	0. Not at all	
H. Agitation	0. Not at all	

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions?0. No

Section N: Medications

N0500A. Was scheduled opioid initiated or continued0. No
N0510A. Was PRN opioid initiated or continued1. Yes
N0510B. Date PRN opioid initiated or continued10/31/2025
N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes")2. Yes
N0520B. Date bowel regimen initiated or continued10/31/2025

Section Z: Record Administration

Z0350. Date Assessment was Completed10/31/2025

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for

payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion

11/9/2025

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE CARE PLAN

Goals:

- ☒ Safe and effective personal care and hygiene
- ☒ Patient clean and comfortable
- ☒ Socialization/support
- ☒ Environment safe and clean

Update #1

Date: 10/15/2025 4:30 PM

Describe patient condition requiring HA interventions: Patient needs assistance with ADL

Patient Info

General Condition

- ☒ Symptoms managed well

Nutrition

- ☒ Diet
- Describe: As tolerated

Functional Limitations

- ☒ OOB with assist

Assistive Devices/Equipment Used During Patient Care

- ☒ Walker
- ☒ Wheelchair

Elimination

- ☒ Incontinent
 - ☒ Bladder
 - ☒ Bowel

Senses

- ☒ Hard of hearing
- ☒ Poor vision
- ☒ Speech problems

Living Arrangements

- ☒ Lives in residential facility
- Describe: Hillside Assisted Living

Interventions

Pain Management

☒ Notify nurse if patient exhibits any verbal or non-verbal signs of pain or symptoms

Bathing

☒ Shower
Frequency: every visit

Hair Care

☒ Shampoo/style
Frequency: Every Visit

Skin Care

☒ Inspect skin; check pressure areas
Frequency: Every Visit

☒ Call nurse if new skin problems are noted
Frequency: Every Visit

☒ Trim nails
☒ Fingers
Frequency: Monthly

☒ Clean nails
☒ Fingers
☒ Toes
Frequency: every visit

☒ Perineal care
Frequency: Every Visit

Foot Care

☒ Bathe/inspect
Frequency: Every Visit

Oral Care

☒ Inspect mouth. Call nurse if problems
Frequency: Every Visit

☒ Assist with brushing teeth
Frequency: Every Visit

Home Management

☒ Clean patient's room/area

Frequency: every visit

Date: 10/15/2025 4:30 PM

Electronic Signature: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/16/2025 6:30:00 AM
Date/time out: 10/16/2025 7:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/17/2025 6:30:00 AM
Date/time out: 10/17/2025 7:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☐ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/20/2025 6:00:00 AM
Date/time out: 10/20/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☐ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/21/2025 6:00:00 AM
Date/time out: 10/21/2025 6:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/22/2025 6:00:00 AM
Date/time out: 10/22/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/23/2025 6:00:00 AM
Date/time out: 10/23/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/24/2025 6:30:00 AM
Date/time out: 10/24/2025 7:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/27/2025 6:00:00 AM
Date/time out: 10/27/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/28/2025 6:00:00 AM
Date/time out: 10/28/2025 6:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/29/2025 6:00:00 AM
Date/time out: 10/29/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/30/2025 6:30:00 AM
Date/time out: 10/30/2025 7:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☐ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported):

Rash on bottom of right shin

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date: 11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 10/31/2025 6:30:00 AM
Date/time out: 10/31/2025 7:15:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☐ No new skin problems
- ☒ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/3/2025 6:00:00 AM
Date/time out: 11/3/2025 6:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Nq

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/3/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/4/2025 6:45:00 AM
Date/time out: 11/4/2025 7:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/5/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/5/2025 6:00:00 AM
Date/time out: 11/5/2025 7:10:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported):

Patient did void and had a bm as well. Daughter was concerned about the BM

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date: 11/5/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/6/2025 6:00:00 AM
Date/time out: 11/6/2025 6:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☐ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/9/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/7/2025 6:00:00 AM
Date/time out: 11/7/2025 6:48:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/9/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/10/2025 6:00:00 AM
Date/time out: 11/10/2025 7:05:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/12/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/11/2025 6:05:00 AM
Date/time out: 11/11/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/12/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/12/2025 6:00:00 AM
Date/time out: 11/12/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☐ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/12/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/13/2025 6:30:00 AM
Date/time out: 11/13/2025 7:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/17/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/14/2025 6:00:00 AM
Date/time out: 11/14/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/24/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/15/2025 6:00:00 AM
Date/time out: 11/15/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/17/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/17/2025 6:30:00 AM
Date/time out: 11/17/2025 7:40:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/17/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/18/2025 6:00:00 AM
Date/time out: 11/18/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/24/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/19/2025 6:00:00 AM
Date/time out: 11/19/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/24/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/20/2025 6:30:00 AM
Date/time out: 11/20/2025 7:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/24/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/21/2025 6:00:00 AM
Date/time out: 11/21/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/24/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/25/2025 6:00:00 AM
Date/time out: 11/25/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked:

Attempted to file nails but patient got upset and began screaming so will attempt tomorrow

Date: 11/30/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/26/2025 6:00:00 AM
Date/time out: 11/26/2025 6:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/30/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/28/2025 6:00:00 AM
Date/time out: 11/28/2025 6:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

11/30/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/1/2025 6:00:00 AM
Date/time out: 12/1/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☐ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

12/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/2/2025 6:00:00 AM
Date/time out: 12/2/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

12/2/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/3/2025 6:30:00 AM
Date/time out: 12/3/2025 7:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

12/3/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/4/2025 6:00:00 AM
Date/time out: 12/4/2025 6:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

12/17/2025

Electronically signed by: CRYSTAL KEEBLE CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/5/2025 11:00:00 AM
Date/time out: 12/5/2025 12:00:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☐ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☐ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☐ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☐ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): N/a

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: N/a

Date:

12/5/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/8/2025 6:45:00 AM
Date/time out: 12/8/2025 7:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☐ No new skin problems
- ☒ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☐ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

12/8/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/9/2025 6:00:00 AM
Date/time out: 12/9/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☐ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

12/9/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/10/2025 6:00:00 AM
Date/time out: 12/10/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☐ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

12/10/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/11/2025 6:00:00 AM
Date/time out: 12/11/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☐ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☐ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Na

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: Na

Date:

12/11/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/12/2025 6:30:00 AM
Date/time out: 12/12/2025 7:15:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☐ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): NA

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: NA

Date:

12/12/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/15/2025 6:00:00 AM
Date/time out: 12/15/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): Pt had a small BM

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: NA

Date:

12/15/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/16/2025 6:25:00 AM
Date/time out: 12/16/2025 7:25:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☐ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☐ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☐ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported):

No Shower/ pt had a Dr. appointment

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: NA

Date:

12/16/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/17/2025 6:00:00 AM
Date/time out: 12/17/2025 7:00:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☐ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☐ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): NA

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: NA

Date:

12/17/2025

Electronically signed by: GEORGIANNA GWEN STEWARD CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/18/2025 12:52:00 PM
Date/time out: 12/18/2025 1:52:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☐ Shower

Hair Care

- ☐ Shampoo/style

Skin Care

- ☐ Inspected skin; checked pressure areas
- ☐ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails
- ☐ Perineal care

Foot Care

- ☐ Bathed and inspected feet

Oral Care

- ☐ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☐ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/18/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/19/2025 1:01:00 PM
Date/time out: 12/19/2025 2:01:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/19/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/22/2025 1:01:00 PM
Date/time out: 12/22/2025 2:01:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/22/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/23/2025 11:32:00 AM
Date/time out: 12/23/2025 12:32:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/23/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/24/2025 1:03:00 PM
Date/time out: 12/24/2025 2:03:00 PM

Pain Management

- ☐ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/24/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/26/2025 12:46:00 PM
Date/time out: 12/26/2025 1:46:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/26/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/29/2025 12:01:00 PM
Date/time out: 12/29/2025 1:01:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/29/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/30/2025 11:31:00 AM
Date/time out: 12/30/2025 12:31:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/30/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/31/2025 1:01:00 PM
Date/time out: 12/31/2025 2:01:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Shampoo/style

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☒ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails
- ☒ Perineal care

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☒ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Home Management

- ☒ Cleaned patient’s room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): No report

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: No report

Date:

12/31/2025

Electronically signed by: DARKEISHA BRITTANY JONES CNA

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

COMFORT KIT ORDERS

All of the patient’s medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring

- ☒ Morphine Sulfate, 100mg/5ml = 20 mg/ml, 0.25-1ml, Sublingual, Q1-2 hours prn, Pain/SOB - #120mL, 00054040441
- ☒ Lorazepam Liquid, 1.75mg/ml, 0.5-1 mg, PO/SL, q4 hrs prn, Anxiety/SOB - #30mL / RF: 5, 00054353244
- ☒ Atropine sulfate oral Soluntion, 0.75%, 1-2 drops, Take Sublingually, every 2 hours as needed, for excess secretions - #15mL / RF: 11
- ☒ APAP 650MG SUPPS, 650 mg, 1 suppository, unwrap and insert Rectally, Q 4 HRS PRN, Pain/Fever - #4 / RF:11 , 00927067812
- ☒ Bisacodyl suppository, 10 mg, 1 suppository, unwrap and insert Rectally, Q DAY PRN, Constipation - #3 / RF:11 , 00363010908
- ☒ Zofran ODT, 1 tablet, 4mg, dissolve PO/SL, Q 8 HRS PRN, nausea/vomiting - #6 / RF:11
- ☒ Senna, 8.6mg, 1 Tab, PO, Daily, Constipation
- ☒ Oxygen, na, 2-4 liters, intranasally, PRN, Shortness of breath

Ordering physician: DIEGO F RESTREPO MD
Date: 10/15/2025

Electronically signed by: TRACY KING RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

MULTIPLE MEDICATIONS FORM

Multiple Medications Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Medications:

Start Date	Discontinue Date	Ordering Physician	Reviewing Physician	Medication	Unit strength	Dose	Form	NDC	Route	Frequency	Indication	Associated diagnosis	Days Supply	Is medication covered by hospice?	If not covered, reason medication is not covered
10/15/2025		Restrepo	DIEGO F RESTREPO MD	acetaminophen 32 MG/ML Oral Suspension	160 mg/5ml	Take 30ml	Oral Suspension	00113002026	Oral Liquid	Every 4 hours as needed for pain	Moderate pain		15	Yes	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	ammonium lactate 120 MG/ML Topical Cream	12%	Apply to skin	Topical Cream	00904598348	Topical	After shower daily	Dry skin		15	Yes	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	dorzolamide 20 MG/ML / timolol 5 MG/ML Ophthalmic Solution [Cosopt]	2-0.5%	1 drop each eye	Ophthalmic Solution	17478060510	Ophthalmic	Twice daily	Dry eye		15	Yes	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	diclofenac sodium 0.01 MG/MG Topical Gel [Voltaren]	1%	Apply to affected area	Topical Gel	00067815201	topical	Every 12 hours	mild pain		15	Yes	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	calcium ascorbate 500 MG Oral Tablet	500 mg	Take 1 tablet	Oral Tablet	10432010001	Oral Pill	Daily	Hypocalcemia		15	No	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	ascorbic acid 100 MG / biotin 0.15 MG / calcium pantothenate 5 MG / folic acid 1 MG / niacin 20 MG / pyridoxine 10 MG / riboflavin 1.7 MG / thiamine mononitrate 1.5 MG / vitamin B12 0.006 MG Oral Capsule	100-0.15-5-1-20-10-1.7-1.5-0.006 mg	Take 1 capsule	Oral Capsule	63044062201	po	Daily	vitamin		15	No	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	cholecalciferol 0.025 MG Oral Capsule	0.025 mg	Take 1 tablet	Oral Capsule	05388000924	Oral Pill	Daily	Hypocalcemia		15	No	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	cobamamide 0.1 MG / vitamin B12 5 MG Sublingual Tablet	0.1-5 mg	Take 1 tablet	Sublingual Tablet	11917015663	Sublingual	Daily	Vitamin b12 deficiency		15	No	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	loratadine 10 MG Oral Tablet [Claritin]	10 mg	Take 1 tablet	Oral Tablet	11523000701	po	Daily	antihistamine		15	Yes	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	levothyroxine sodium 0.075 MG Oral Tablet	0.075 mg	Take 1 tablet	Oral Tablet	00378180504	Oral Pill	Before breakfast	Hypothyroidism		15	No	
10/15/2025		Restrepo	DIEGO F RESTREPO MD	ABH GEL	1/25/1 mg/ml	Apply to inner wrist	Syringe		topical	Every 4 hours as needed for anxiety	anxiety		15	Yes	
10/17/2025	10/24/2025	Restrepo	DIEGO F RESTREPO MD	sulfamethoxazole 800 MG / trimethoprim 160 MG Oral Tablet [Bactrim]	800-160 mg	Take 1 tablet	Oral Tablet	13310014601	po	Twice daily	infection		7	Yes	

Date: 10/15/2025

Electronically signed by: VIRGINIA ALLAN RN
Electronically signed by: DIEGO F RESTREPO MD

No Place Like Home LLC
179 Inwood Trl
Azle, TX 76020 USA
8172307078
nplh_timesheets@yahoo.com



INVOICE

BILL TO

Homage
8204 Elmbrook Drive Suite 276
Dallas, TX 75247

INVOICE # 3639

DATE 12/02/2025

DUE DATE 01/01/2026

TERMS Net 30

ACTIVITY	QTY	RATE	AMOUNT
Franklin, Weldon Ubani, Helga LVN 11/26/2025 12:10p-12a	11.75	38.00	446.50
Franklin, Weldon Ubani, Helga LVN 11/27/2025 12a-8a (HOLIDAY)	8	57.00	456.00
Franklin, Weldon Uzuegbunam, Veronica LVN 11/27/2025 8a-10a (HOLIDAY)	2	57.00	114.00

BALANCE DUE

\$1,016.50

Pay invoice

No Place Like Home LLC
179 Inwood Trl
Azle, TX 76020 USA
8172307078
nplh_timesheets@yahoo.com



INVOICE

BILL TO
Homage
8204 Elmbrook Drive Suite 276
Dallas, TX 75247

INVOICE # 3753
DATE 12/04/2025
DUE DATE 01/03/2026
TERMS Net 30

ACTIVITY	QTY	RATE	AMOUNT
Wagoner, David 1 Woodard, Robert LVN 11/28/2025 12:30p-8p	7.50	38.00	285.00
Wagoner, David 1 Ezigbo, Amaka LVN 11/28/2025 8p-12a	4	38.00	152.00
Wagoner, David 1 Ezigbo, Amaka LVN 11/29/2025 12a-8a	8	38.00	304.00
Wagoner, David 1 Woodard, Robert LVN 11/29/2025 8a-8p	12	38.00	456.00
Wagoner, David 1 Ezigbo, Amaka LVN 11/29/2025 8p-12a	4	38.00	152.00
Wagoner, David 1 Ezigbo, Amaka LVN 11/30/2025 12a-8a	8	38.00	304.00
Wagoner, David 1 Woodard, Robert LVN 11/30/2025 8a-8p	12	38.00	456.00
Wagoner, David 1 Ezigbo, Amaka LVN 11/30/2025 8p-12a	4	38.00	152.00
Wagoner, David 1 Ezigbo, Amaka LVN 12/01/2025 12a-1:30a	1.50	38.00	57.00

BALANCE DUE **\$2,318.00**

Pay invoice

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Azle, TX 76020 USA
8172307078
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INVOICE

BILL TO

Homage
8204 Elmbrook Drive Suite 276
Dallas, TX 75247

INVOICE # 3813

DATE 12/08/2025

DUE DATE 01/07/2026

TERMS Net 30

ACTIVITY	QTY	RATE	AMOUNT
RN oncall Elizabeth Voss RN oncall 12/5/25 5p-12a	7	7.00	49.00
RN oncall Elizabeth Voss RN oncall 12/6/25 12a-12a	24	7.00	168.00
RN oncall Elizabeth Voss RN oncall 12/7/25 12a-8a	8	7.00	56.00
Pals, James RN visit Elizabeth Voss RN visit 12/6/25	1	100.00	100.00
Pals, James RN visit mileage	30	0.50	15.00
Pals, James RN visit Elizabeth Voss RN visit 12/7/25	1	100.00	100.00
Pals, James RN visit mileage	30	0.50	15.00

BALANCE DUE

\$503.00

Pay invoice

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Azle, TX 76020 USA
8172307078
nplh_timesheets@yahoo.com



INVOICE

BILL TO

Homage
8204 Elmbrook Drive Suite 276
Dallas, TX 75247

INVOICE # 3795

DATE 12/08/2025

DUE DATE 01/07/2026

TERMS Net 30

ACTIVITY	QTY	RATE	AMOUNT
RN oncall Brenda Ellis RN oncall 12/5/25 5p-12a	7	7.00	49.00
RN oncall Brenda Ellis RN oncall 12/6/25 12a-7a	7	7.00	49.00

BALANCE DUE

\$98.00

Pay invoice

Revocation of Election to Hospice Care

Homage Hospice

Patient's Name: Katherine Howrigan ID# 101996

Medicare Number 183VK6VY96 Effective Date of Revocation _____

Revocation Statement

I, Katherine Howrigan revoke my election for Medicare coverage of hospice care for the remainder of this benefit period _____.

I understand that I will lose the remaining days of this benefit period.

I understand I now resume Medicare coverage of the benefits waived when I elected the Hospice Benefit.

I understand that I may, at any time, elect to receive hospice coverage for other hospice benefit periods that I am eligible to receive. I will be electing benefit period _____.

(Number)

The reason for revocation is _____

x Shirley Cox (P.O.N.)

Patient's Signature

Date

Hospice's Staff Signature

Date

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

COVID-19 ASSESSMENT

COVID-19 ASSESSMENT

This assessment tool supports you in identifying patients at risk for COVID-19 and helps guide clinicians in clinical decision making.

Symptoms

Note: Symptoms may appear 2-14 days after exposure.

☒ None

Travel History

***Updated information on affected countries can be found at: <https://www.cdc.gov/coronavirus/2019-ncov/travelers/index.html>*

Action Taken

☒ None indicated

***Please refer to the CDC COVID-19 website for ongoing updates and guidance: <https://www.cdc.gov/coronavirus/2019-ncov>*

COVID-19 TESTING

Patient testing completed? No

Date: 10/15/2025

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

LCD - DEMENTIA

LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

Worsening of Clinical Status

- ☒ Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss

Worsening of Signs

- ☒ Edema
- ☒ Weakness
- ☒ Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease
- ☒ Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)
- ☒ Progressing dependence for performance of ADLs

Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ CHF
- ☒ Diabetes Mellitus
- ☒ Dementia

This section is specific for Alzheimer's disease and related disorders, and is not appropriate for other types, such as multi-infarct dementia.)

Patients with dementia should show all of the following characteristics:

- ☒ Stage 7 or beyond according to the FAST
- ☒ Unable to dress without assistance
- ☒ Unable to bathe without assistance
- ☒ Urinary and fecal incontinence, intermittent or constant
- ☒ No consistently meaningful verbal communication; stereotypical phrases only or the ability to speak is limited to six or fewer intelligible words

Additional supporting factors (Patients should have had one of the following within the past 12 months)

- ☒ Pyelonephritis or upper urinary tract infection

Date: 10/15/2025 12:15:00 PM

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

LCD - GENERAL DECLINE IN CLINICAL STATUS

LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

Worsening of Clinical Status

- ☒ Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss

Worsening of Signs

- ☒ Edema
- ☒ Weakness

- ☒ Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease
- ☒ Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)
- ☒ Progressing dependence for performance of ADLs

Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ CHF
- ☒ Diabetes Mellitus
- ☒ Dementia

Date: 10/15/2025 12:17:00 PM

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

LVN NURSE VISIT

LVN NURSE COMPREHENSIVE VISIT

Date/Time in: 10/18/2025 10:30:00 AM
Date/Time out: 10/18/2025 11:01:00 AM
COVID-19 screening performed? Yes
Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• *Temperature*

Type of temperature taken: Temporal:
Temporal temperature: 98

• *Respiratory Rate*

Respiratory rate (respirations per minute): 16
☒ Room air

• *Heart Rate*

Type of heart rate: Regular
Apical pulse: 74
Radial pulse: 74

• *Blood Pressure*

Hypertension or hypotension? Hypertension controlled with medication
☒ Sitting
Systolic blood pressure: 128
Diastolic blood pressure: 66

• *Pedal Pulses*

☒ Present
☒ Right
☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented
- ☒ Confused
- ☒ Forgetful
- ☒ Impaired judgement

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ 6 or fewer words
- ☒ Nonsensical

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 0
New, controlled, or uncontrolled? Controlled

Neurosensory

- ☒ Restless sleep
- ☒ Sundowner's

Cardiovascular

- ☒ Capillary refill > 3 seconds

Nutrition

- ☒ Appetite is a problem
- Percent of meals eaten: 50
Arm circumference (left): 8.5

GI Elimination

Date of last bowel movement: 10/17/2025
Bowel Pattern Regular
Bowel sounds, # of quads: 4
☒ WNL
☒ Abdomen: Soft
☒ Incontinence

GU Elimination

Urine output: Fair
Urine color: Yellow
☒ UTI
Treatment for urinary tract infection: Currently taking antibiotics sulfamethoxazole started 10/14/25
☒ Incontinent

Respiratory

- ☒ Clear
- Describe location, inspiratory/expiratory: All lobes
Oxygen saturation percentage: 96

Mobility

- ☒ Up to chair/wheelchair

☒ Transfer with assist

Transfer with assist x: 1

☒ Poor endurance

☒ Weakness/fatigue

Skin

Skin temperature: Warm

Skin turgor: Poor

Psychosocial Status

Psychosocial/emotional status: Calm

Anticipatory grief/bereavement status: Moderate

Medication Utilization

Have any PRN medications been used in the last 24 hours? No

Does patient require refill of any medication? No

Communication

• Instructed on policies and procedures related of use and disposal of controlled drugs in the home

☒ Patient

☒ Family

☒ Caregiver

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): Educated on Late-onset Alzheimer's (LOA) is the most common form of dementia, typically beginning after age 65, and is characterized by progressive memory and cognitive decline. Key aspects include a gradual decline in memory, challenges with planning and problem-solving, and difficulty with familiar tasks.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

• Instructed on infection control procedures

☒ Patient

☒ Family

☒ Caregiver

☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Family reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Plan for next visit: SNV

Additional Information/Comments: 91 y/o female admitted with primary diagnosis G30.1 - Alzheimer's disease with late onset admitted to hospice services currently living in ALF, when arrive daughter was present in facility assisting

patient with ADL care, patient sitting up in chair alert but confused, daughter stated patient can get agitated at times, she also states patient currently is being treated for a UTI started taking antibiotics on 10/14/2025 BID for 10 days, patient is not drinking enough fluids and she also is not eating a full meal, daughter stated she have tried ensure several times patient refuses, she will try something else to introduce for supplement, patient is not able to transfer herself or bare her own weight she is assist x1, assessment breath sounds clear all lobes no pain at this time, daughter understands plan of care.

Date: 10/18/2025

Electronically signed by: LATANDRA JONES LVN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOPE UPDATE VISIT

Date 10/21/2025
Reason for Visit 2. HOPE Update Visit 1 (HUV1)

At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less? 0. No

Was a symptom impact screening completed? 1. Yes

Date of symptom impact screening 10/21/2025

- Pain 1. Slight
- Shortness of breath 1. Slight
- Anxiety 1. Slight
- Nausea 0. Not at all
- Vomiting 0. Not at all
- Diarrhea 0. Not at all
- Constipation 0. Not at all
- Agitation 0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

- Was scheduled opioid initiated or continued? 0. No
- Was PRN opioid initiated or continued? 1. Yes
- Date PRN opioid initiated or continued 10/21/2025
- Was bowel regimen initiated or continued? 2. Yes
- Date bowel regimen initiated or continued 10/21/2025

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HUV NURSING VISIT NOTE

NURSE COMPREHENSIVE VISIT

Date/Time in:10/21/2025 12:00:00 PM

Date/Time out:10/21/2025 1:00:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Others present during visit:none

Patient/Family/Caregiver's primary concern during this visit:none

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 98.1
- Respiratory Rate

Respiratory rate (respirations per minute): 18

☒ Room air
- Heart Rate

Type of heart rate: Regular

Apical pulse: 63
- Blood Pressure

Hypertension or hypotension? Hypertension controlled with medication

☒ Sitting

Systolic blood pressure: 96

Diastolic blood pressure: 65
- Pedal Pulses

☒ Present

☒ Right

☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented
- ☒ Confused
- ☒ Forgetful

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Appropriate
- ☒ Garbled

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 1
New, controlled, or uncontrolled? Controlled
Location/description of pain: generalized

Neurosensory

- ☒ Dizziness

Cardiovascular

- ☒ Syncope
- ☒ Capillary refill > 3 seconds
- ☒ Edema

Location: bilateral lower extremities
Rating: +2

Nutrition

- ☒ Patient satisfied with intake
- Percent of meals eaten: 50
- ☒ Patent
 - ☒ Intact

GI Elimination

Date of last bowel movement: 10/21/2025
Bowel Pattern Daily
Bowel sounds, # of quads: 4

- ☒ WNL

Describe: times 4

- ☒ Abdomen: Soft
- ☒ Incontinence

GU Elimination

Urine output: Fair

- ☒ Frequency

Describe: clear

- ☒ Clear
- ☒ Incontinent

Respiratory

- ☒ Diminished
- Describe location, inspiratory/expiratory: lower extremities
- ☒ Dyspnea at rest
- ☒ Dyspnea with exertion

Mobility

- ☒ Ambulatory
- Steady/unsteady gait: Steady gait
- ☒ Poor endurance
- ☒ Weakness/fatigue

Skin

Skin temperature: Warm

Skin turgor: Fair

Psychosocial Status

Psychosocial/emotional status: Stable

Anticipatory grief/bereavement status: Normal

HOPE Update Visit

Reason for Visit	2. HOPE Update Visit 1 (HUV1)
At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?	0. No
Was a symptom impact screening completed?	1. Yes
Date of symptom impact screening	10/21/2025
Pain	1. Slight
Shortness of breath	1. Slight
Anxiety	1. Slight
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	1. Slight
Agitation	0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 0. No

Was PRN opioid initiated or continued? 1. Yes

Date PRN opioid initiated or continued 10/21/2025

Was bowel regimen initiated or continued? 2. Yes

Date bowel regimen initiated or continued 10/21/2025

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes

Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction):

- Hospice philosophy and goals of care
- Roles of the hospice team (RN, MD, CNA, social worker, chaplain)
- Plan of care, visit frequency, and 24/7 on-call availability
- Symptom management (pain, shortness of breath, nausea, anxiety, constipation)
- Use, purpose, and possible side effects of current medications and comfort kit
- Safety and fall precautions in the home/facility
- When to call hospice vs. 911
- Patient and family rights, responsibilities, and informed consent
- Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Communication with family:

Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

- ☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition
- ☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Date: 10/21/2025

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE PROGRESS NOTE

NURSE PROGRESS NOTE

Date/Time in:10/22/2025 2:52:00 AM

Date/Time out:10/22/2025 2:53:00 AM

Purpose of progress note:IDG Narrative

IDG Hospice Admission Skilled Nursing Note
Narrative

Patient Name: Katherine Howrigan
Age: 91
Sex/Race: Caucasian Female
Admission Date: 10/15/2025
Primary Diagnosis: Senile Degeneration of the Brain
Comorbidities: Dementia, Osteoporosis, Osteoarthritis, Hypothyroidism, Essential Tremors, Glaucoma, Alopecia
Level of Care: Routine Hospice Care
Advance Directives: OOHDR
POA/Decision Maker: Daughter
Allergies: NKDA

FAST: 6e

Scales:

PPS: 30%

KPS: 30%

LMAC: 18.5 cm

Subjective

SN admitted the patient to hospice services at her group home. The patient's daughter provided the patient's medical history and current health status. She expressed the family's wish to focus on comfort care and to avoid further hospitalizations.

Reports indicate that over the past three months, Ms. Howrigan has exhibited progressive cognitive, functional, and physical decline, including increased weakness, poor endurance, and decreased nutritional intake.

The daughter reports the patient sleeps approximately 16–18 hours per day with frequent daytime napping. Appetite remains poor—she consumes about 25% of her meals, consisting of small portions that she rarely finishes—no reports of nausea or vomiting.

Objective

General: Elderly female, frail and cachectic in appearance, seated in her wheelchair on the porch at the time of assessment. Appears calm, no acute distress noted.

Neurological: Alert and oriented ×1. Follows simple commands inconsistently. Speech is clear but limited. Displays mild essential tremors.

Functional Status:

ADLs: Requires maximum assistance with 5/6 ADLs (previously 4/6).

Feeding: Self-feeds with utensils but requires prompting to do so.

Continence: Incontinent of bowel and bladder.

Nutrition/Weight: Appetite poor; intake 25% of three small meals per day. Evidence of weight loss and generalized muscle wasting was observed.

Skin/Wounds: Skin clean, dry, and intact (CDI). No open areas or pressure injuries noted.

Falls: One fall reported in the past six months; no injury sustained.

Cardiopulmonary: LSCTA; respirations even and unlabored on room air; diminished breath sounds in the lower lobes—occasional shortness of breath with exertion.

GI/GU: Abdomen soft and non-tender; bowel sounds active ×4 quadrants—incontinent of both bowel and bladder.

Musculoskeletal: Limited mobility; primarily wheelchair-bound; unable to ambulate independently.

Pain: Denies pain at the time of the visit. Tylenol is available and used PRN for mild discomfort.

Mood/Behavior: Cooperative and calm; no anxiety or agitation observed. The patient refused blood pressure measurement, which the daughter states is typical behavior. SN will notify the primary RN to obtain a wrist BP cuff for future visits.

Assessment

91-year-old Caucasian female with end-stage senile degeneration of the brain, compounded by multiple comorbidities. The patient exhibits severe cognitive and functional decline, poor nutritional intake, weight loss, and increased dependence on ADLs. Prognosis poor.

Plan

Hospice Admission: The patient was admitted at the routine level of care for comfort management and symptom control.

Pain Management: Continue Tylenol PRN for mild discomfort; reassess as needed.

Wound Care: None required at present; monitor for pressure areas due to immobility.

Nutrition: Encourage small, frequent meals and nutritional supplements as tolerated.

Fall Precautions: Hospital bed, briefs, chux, and gloves provided. Reinforce safety during transfers and repositioning.

Respiratory Care: Monitor for dyspnea; provide comfort measures and oxygen PRN if indicated.

Incontinence Care: Continue using briefs to maintain skin integrity and hygiene.

Psychosocial Support: Chaplain and Social Work services to be initiated for patient and family support.

Education: Family educated on comfort and safety measures, disease progression, symptom management, DME, hospice philosophy, skin care, comfort kit use, medication regimen, and fall precautions.

Follow-up: SN reviewed the findings with the IDT and Dr. Restrepo. Patient deemed hospice appropriate. Initial POC reviewed with the patient's daughter and MPOA—all in agreement.

? Scales upon Admission: PPS/KPS 30 | LMAC 18.5 cm | DNR | Allergies: NKDA

Date: 10/22/2025

Electronically signed by: VIRGINIA ALLAN RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:10/28/2025 2:00:00 PM

Date/Time out:10/28/2025 2:30:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 97.8

• Respiratory Rate

Respiratory rate (respirations per minute): 18
☒ Room air

• Heart Rate

Type of heart rate: Regular
Apical pulse: 63
Radial pulse: 86

• Blood Pressure

☒ Sitting
Systolic blood pressure: 96
Diastolic blood pressure: 65

Current Mental Status

- ☒ Alert
- ☒ Disoriented
- ☒ Confused

Current Behavioral Status

- ☒ Withdrawn

Speech Status

- ☒ Garbled

Pain Assessment

Pain rating type: Non-Verbal Scale
Pain Rating: 0

Neurosensory

- ☒ Sundowner's

Cardiovascular

☒ Edema

Location: BLE

Rating: +1

Nutrition

☒ Patient satisfied with intake

GI Elimination

Date of last bowel movement: 10/28/2025

Bowel Pattern normal

Bowel sounds, # of quads: 4

☒ Abdomen: Soft

☒ Incontinence

GU Elimination

Urine output: Good

☒ Incontinent

Respiratory

☒ Clear

Describe location, inspiratory/expiratory: upper lobes

☒ Diminished

Describe location, inspiratory/expiratory: lower lobes

Mobility

☒ Up to chair/wheelchair

☒ Weakness/fatigue

Skin

Skin temperature: warm

Skin turgor: poor

☒ Pale

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide

Present/not present Not present

• Interview With

☒ Family

☒ Facility staff

• Follows assignment/plan of care

☒ Yes

- *Patient's needs are being met*

☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

- *Patient's linens are fresh and clean*

☒ Yes

• Assignment sheet/POC reviewed

☒ Yes

• Assignment sheet/POC updated

☒ Yes

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

• Teaching provided during visit

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

• Instructed on infection control procedures

☒ Patient

☒ Family reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

Additional Information/Comments: 91-year-old female patient on hospice with terminal diagnosis of Alzheimer's disease. The patient was sitting up on wheelchair during SNV. No pain or distress currently, vital WNL. Daughter was present during the visit. The patient resides in a residential care home in Keller TX. Daughter stated that the patient had completed antibiotics for UTI and her urine looked much better. Upon assessment, patient had redness to right lower extremity. She has +1 edema to BLE. Right lower extremity was warm to touch. MD was notified and received orders for antibiotic for cellulitis. Orders were faxed to the facility and patient's daughter notified of the new medication. Discussed medications, hydration, nutrition and safety. Reviewed the POC and when to call hospice with the patient's daughter and facility director.

Date: 10/28/2025

Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HOPE UPDATE VISIT

Date

Reason for Visit

At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?

Was a symptom impact screening completed?

Date of symptom impact screening

Pain

Shortness of breath

Anxiety

Nausea

Vomiting

Diarrhea

Constipation

Agitation

10/31/2025

3. HOPE Update Visit 2 (HUV2)

0. No

1. Yes

10/31/2025

1. Slight

1. Slight

1. Slight

0. Not at all

0. Not at all

0. Not at all

0. Not at all

0. Not at all

Skin Conditions

Does the patient have one or more skin conditions?

0. No

Medications

Was scheduled opioid initiated or continued?

Was PRN opioid initiated or continued?

Date PRN opioid initiated or continued

Was bowel regimen initiated or continued?

Date bowel regimen initiated or continued

0. No

1. Yes

10/31/2025

2. Yes

10/31/2025

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HUV NURSING VISIT NOTE

NURSE COMPREHENSIVE VISIT

Date/Time in: 10/31/2025 1:15:00 PM
Date/Time out: 10/31/2025 2:30:00 PM
COVID-19 screening performed? Yes
Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No
Others present during visit: none
Patient/Family/Caregiver's primary concern during this visit: none

Vital Signs

- *Temperature*
Type of temperature taken: Temporal:
Temporal temperature: 97.3
- *Respiratory Rate*
Respiratory rate (respirations per minute): 18
☒ Room air
- *Heart Rate*
Type of heart rate: Regular
Radial pulse: 72
- *Blood Pressure*
Hypertension or hypotension? Hypertension controlled with medication
☒ Sitting
Systolic blood pressure: 110
Diastolic blood pressure: 65
- *Pedal Pulses*
☒ Present
☒ Right
☒ Left

Current Mental Status

- ☒ Alert
- ☒ Confused
- ☒ Forgetful

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative
- ☒ Withdrawn

Speech Status

- ☒ Clear
- ☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 2
New, controlled, or uncontrolled? Controlled
Location/description of pain: generalized

Neurosensory

- ☒ Dizziness

Cardiovascular

- ☒ Syncope
- ☒ Capillary refill > 3 seconds
- ☒ Edema

Location: bilateral lower extremities
Rating: +2

Nutrition

- ☒ Patient satisfied with intake
- Percent of meals eaten: 50
- ☒ Patent
 - ☒ Intact

GI Elimination

Date of last bowel movement: 10/31/2025
Bowel Pattern daily
Bowel sounds, # of quads: 4

- ☒ WNL

Describe: times 4

- ☒ Abdomen: Soft
- ☒ Incontinence

GU Elimination

Urine output: Fair

- ☒ Frequency

Describe: clear

- ☒ Clear
- ☒ Incontinent

Respiratory

☒ Diminished

Describe location, inspiratory/expiratory: lower extremities

☒ Dyspnea at rest

☒ Dyspnea with exertion

Mobility

☒ Ambulatory

Steady/unsteady gait: Steady gait

☒ Poor endurance

☒ Weakness/fatigue

Skin

Skin temperature: warm

Skin turgor: poor

Psychosocial Status

Psychosocial/emotional status: stable

Anticipatory grief/bereavement status: normal

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide

Hospice caregiver name: DAVIS, DEMETRICE

Present/not present Not present

• Interview With

☒ PCG

• Follows assignment/plan of care

☒ Yes

• Patient's needs are being met

☒ Yes

• Patient/family/CG satisfied with care provided

☒ Yes

• Treats patient with dignity and respect

☒ Yes

• Interacts well with patient/family/CG

☒ Yes

• Provides care in a safe manner

☒ Yes

• Arrives on time or calls if late

☒ Yes

- *Competent with assigned tasks*
☒ Yes
- *Complies with infection control procedures*
☒ Yes
- *Reports changes in patient's condition to RN*
☒ Yes
- *Patient appears well groomed*
☒ Yes
- *Patient's hair is cleaned*
☒ Yes
- *Patient's mouth is clean and odor-free*
☒ Yes
- *Patient is clean-shaven*
☒ N/A
- *Patient's skin is moisturized*
☒ Yes
- *Patient's room is clean and tidy*
☒ Yes
- *Patient's linens are fresh and clean*
☒ Yes
- *Assignment sheet/POC reviewed*
☒ Yes
- *Assignment sheet/POC updated*
☒ Yes

RN Supervisory Visit (2)

Hospice Caregiver Discipline LPN
Hospice caregiver name: JOHNSON, ALISIA
Present/not present: Not present

- *Interview With*
☒ PCG
- *Follows assignment/plan of care*
☒ Yes
- *Patient's needs are being met*
☒ Yes

• Patient/family/CG satisfied with care provided

☒ Yes

• Treats patient with dignity and respect

☒ Yes

• Complies with infection control procedures

☒ Yes

• Reports changes in patient's condition to RN

☒ Yes

• Punctual: notifies office if late

☒ Yes

• Dresses properly

☒ Yes

• Interacts well with patient/family/CG

☒ Yes

• Provides care in a safe manner

☒ Yes

• Arrives on time or calls if late

☒ Yes

• Competent with assigned tasks

☒ Yes

HOPE Update Visit

Reason for Visit	3. HOPE Update Visit 2 (HUV2)
At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?	0. No
Was a symptom impact screening completed?	1. Yes
Date of symptom impact screening	10/31/2025
Pain	1. Slight
Shortness of breath	1. Slight
Anxiety	1. Slight
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	1. Slight
Agitation	0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 0. No
Was PRN opioid initiated or continued? 1. Yes
Date PRN opioid initiated or continued 10/31/2025
Was bowel regimen initiated or continued? 2. Yes
Date bowel regimen initiated or continued 10/31/2025

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes
Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction):
Hospice philosophy and goals of care
Roles of the hospice team (RN, MD, CNA, social worker, chaplain)
Plan of care, visit frequency, and 24/7 on-call availability
Symptom management (pain, shortness of breath, nausea, anxiety, constipation)
Use, purpose, and possible side effects of current medications and comfort kit
Safety and fall precautions in the home/facility
When to call hospice vs. 911
Patient and family rights, responsibilities, and informed consent
Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Communication with family: Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

- ☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition
- ☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Date: 10/31/2025

Electronically signed by: TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:11/4/2025 1:00:00 PM

Date/Time out:11/4/2025 1:30:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 97.3
- Respiratory Rate

Respiratory rate (respirations per minute): 20

☒ Room air
- Heart Rate

Type of heart rate: Regular

Radial pulse: 65
- Blood Pressure

☒ Sitting

Systolic blood pressure: 97

Diastolic blood pressure: 65

Current Mental Status

- ☒ Disoriented
- ☒ Forgetful
- ☒ Lethargic

Current Behavioral Status

- ☒ Withdrawn

Speech Status

- ☒ Garbled
- ☒ Nonsensical

Pain Assessment

Pain rating type: Non-Verbal Scale

Pain Rating: 0

Neurosensory

- ☒ Sundowner's

Cardiovascular

☒ Edema

Location: BLE

Rating: +1

Nutrition

☒ Patient satisfied with intake

GI Elimination

Date of last bowel movement: 11/3/2025

Bowel Pattern occasional constipation

Bowel sounds, # of quads: 4

☒ Abdomen: Soft

☒ Constipation

GU Elimination

Urine output: Good

☒ Clear

Respiratory

☒ Clear

Describe location, inspiratory/expiratory: upper lobes

☒ Diminished

Describe location, inspiratory/expiratory: lower lobes

Mobility

☒ Up to chair/wheelchair

☒ Poor endurance

☒ Weakness/fatigue

Skin

Skin temperature: warm

Skin turgor: poor

☒ Pale

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide

Present/not present Present

• Interview With

☒ Family

☒ Facility staff

- *Follows assignment/plan of care*

☒ Yes

- *Patient's needs are being met*

☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

• Patient's linens are fresh and clean

☒ Yes

• Assignment sheet/POC reviewed

☒ Yes

• Assignment sheet/POC updated

☒ Yes

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

• Instructed on policies and procedures related of use and disposal of controlled drugs in the home

- ☒ Family
- ☒ Caregiver

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): medication compliance

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

• Instructed on infection control procedures

- ☒ Family
- ☒ Caregiver

- ☒ Family reminded to call hospice and not 911 for any concerns, problems or changes in condition
- ☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

Additional Information/Comments: 91-year-old female on service with terminal diagnosis of Alzheimer's disease. The patient was sitting on chair in the living room. No pain or distress currently, vitals WNL. She was pleasantly confused. Speech is garbled, speaks with word salad. The patient is currently on antibiotics for cellulitis. No redness noted to BLE. She is responding well to treatment. Facility caregiver reported that the patient was constipated. She has senna on her profile, but they have been giving it PRN. Scheduled the medication to be administered daily. Called the patient's daughter and updated her on the visit. Reviewed the POC and when to call hospice with the facility director.

Date: 11/4/2025

Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:11/12/2025 11:00:00 AM

Date/Time out:11/12/2025 11:30:00 AM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 97

• Respiratory Rate

Respiratory rate (respirations per minute): 18
☒ Room air

• Heart Rate

Type of heart rate: Regular
Radial pulse: 76

• Blood Pressure

☒ Sitting
Systolic blood pressure: 121
Diastolic blood pressure: 78

• Pedal Pulses

☒ Present
☒ Right
☒ Left

Current Mental Status

☒ Disoriented
☒ Confused
☒ Forgetful

Current Behavioral Status

☒ Appropriate

Speech Status

☒ Garbled
☒ Nonsensical

Pain Assessment

Pain rating type: Non-Verbal Scale
Pain Rating: 0

Nutrition

☒ Patient satisfied with intake

GI Elimination

Date of last bowel movement: 11/12/2025
Bowel Pattern: ocassional constipation
Bowel sounds, # of quads: 4
☒ Abdomen: Soft
☒ Constipation

GU Elimination

Urine output: Good
☒ Incontinent

Respiratory

☒ Clear
Describe location, inspiratory/expiratory: all lobes
Oxygen saturation percentage: 97

Mobility

☒ Up to chair/wheelchair
☒ Poor endurance
☒ Weakness/fatigue

Skin

Skin temperature: warm
Skin turgor: poor
☒ Pale

RN Supervisory Visit (1)

Hospice Caregiver Discipline: Hospice Aide
Present/not present: Not present

• Interview With

☒ Family
☒ Facility staff

• Follows assignment/plan of care

☒ Yes

• Patient's needs are being met

☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

- *Patient's linens are fresh and clean*

☒ Yes

• *Assignment sheet/POC reviewed*

☒ Yes

• *Assignment sheet/POC updated*

☒ Yes

RN Supervisory Visit (2)

Hospice Caregiver Discipline Hospice Aide
Present/not present: Not present

• *Interview With*

☒ Patient

☒ Family

☒ Facility staff

• *Follows assignment/plan of care*

☒ Yes

• *Patient's needs are being met*

☒ Yes

• *Patient/family/CG satisfied with care provided*

☒ Yes

• *Treats patient with dignity and respect*

☒ Yes

• *Complies with infection control procedures*

☒ Yes

• *Reports changes in patient's condition to RN*

☒ Yes

• *Punctual: notifies office if late*

☒ Yes

• *Dresses properly*

☒ Yes

• *Interacts well with patient/family/CG*

☒ Yes

• *Provides care in a safe manner*

☒ Yes

• *Arrives on time or calls if late*

☒ Yes

• *Competent with assigned tasks*

☒ Yes

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

• *Instructed on policies and procedures related of use and disposal of controlled drugs in the home*

☒ Patient

• *Teaching provided during visit*

Topic(s) discussed (include topic(s) and person(s) receiving instruction): nutrition, safety precautions.

• *Method(s)*

☒ Verbal instruction

• *Instructed on infection control procedures*

☒ Family

☒ Caregiver

☒ Family reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

Additional Information/Comments: 91-year-old female on service with terminal diagnosis of Alzheimer's disease. The patient was sitting outside with daughter during SNV. Caregiver reported that she now having normal bowel movements. When she does have a bowel movement at night, she will eat her BM. They have caught her in two occasions. Discussed the patient's nutrition with the caregiver and advised them to give her snack prior to bedtime so that she is not so hungry at bedtime. No pain or distress currently vitals WNL. Refilled the patient's medications this visit. Reviewed the POC and when to call hospice.

Date: 11/12/2025

Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:11/18/2025 3:30:00 PM

Date/Time out:11/18/2025 4:00:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 98.1

• Respiratory Rate

Respiratory rate (respirations per minute): 20
☒ Room air
Oxygen liters per minute: 97

• Heart Rate

Type of heart rate: Regular
Radial pulse: 72

• Blood Pressure

☒ Sitting
Systolic blood pressure: 95
Diastolic blood pressure: 62

• Pedal Pulses

☒ Present
☒ Right
☒ Left

Current Mental Status

☒ Disoriented
☒ Confused
☒ Forgetful

Current Behavioral Status

☒ Appropriate

Speech Status

☒ Garbled
☒ Nonsensical

Pain Assessment

Pain rating type: Non-Verbal Scale
Pain Rating: 0
New, controlled, or uncontrolled? Controlled

Nutrition

☒ Patient satisfied with intake

GI Elimination

Date of last bowel movement: 11/18/2025
Bowel Pattern Constipation
Bowel sounds, # of quads: 4
☒ WNL
☒ Abdomen: Soft
☒ Constipation

GU Elimination

Urine output: Good
☒ Incontinent

Respiratory

☒ Clear
Describe location, inspiratory/expiratory: All lobes

Mobility

☒ Up to chair/wheelchair
☒ Poor endurance
☒ Weakness/fatigue

Skin

Skin temperature: Warm
Skin turgor: Poor
☒ Pale

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide
Hospice caregiver name: Crystal Keeble, CNA
Present/not present Not present

• Interview With

☒ Family
☒ Facility staff

• Follows assignment/plan of care

☒ Yes

- *Patient's needs are being met*

☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

- *Patient's linens are fresh and clean*

☒ Yes

• *Assignment sheet/POC reviewed*

☒ Yes

• *Assignment sheet/POC updated*

☒ Yes

Comments: Continue POC

Medication Utilization

Have any PRN medications been used in the last 24 hours? No

Does patient require refill of any medication? No

Communication

• *Teaching provided during visit*

Topic(s) discussed (include topic(s) and person(s) receiving instruction): Education was provided on strategies to help manage constipation, along with a review of the current plan of care and guidance on when to contact hospice for changes or concerns. Information was also shared regarding expected disease progression to help the family feel prepared and supported.

• *Method(s)*

☒ Verbal instruction

• *Outcome(s)*

☒ Verbalized understanding

☒ Family reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

Additional Information/Comments: The patient, a 91-year-old female on hospice service with a terminal diagnosis of Alzheimer's disease, was assessed. The patient was sitting up on wheelchair in the living room during SNV. No pain or distress currently, vitals WNL. Patient 's daughter was present. Facility director and caregiver stated that the patient was constipated and had no BM in 4 days. They had administered laxatives including magnesium citrate with no luck. SN spoke with the patient's daughter and agreed to administer enema. Patient was wheeled to her room, transferred to bed, repositioned to left side. Prior to transfer, caregiver administered ABH gel due to patient's increased agitation. SN administered enema, patient tolerated well. There was a little BM and leakage after enema administration. Advised the patient's daughter to leave the patient in bed lying on her left side. Patient was resting quietly when SN left. Advised the caregiver to continue administering daily laxatives as ordered. Reviewed the POC and when to call hospice with the patient's daughter and caregiver.

Date: 11/18/2025

Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:11/26/2025 10:00:00 AM

Date/Time out:11/26/2025 10:30:00 AM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 97

• Respiratory Rate

Respiratory rate (respirations per minute): 18
☒ Room air
Oxygen liters per minute: 94

• Heart Rate

Type of heart rate: Regular
Radial pulse: 53

• Blood Pressure

☒ Sitting
Systolic blood pressure: 102
Diastolic blood pressure: 50

• Pedal Pulses

☒ Present
☒ Right
☒ Left

Current Mental Status

- ☒ Oriented
- ☒ Disoriented
- ☒ Confused
- ☒ Forgetful

Other: Oriented to self.

Current Behavioral Status

- ☒ Appropriate

Speech Status

- ☒ Garbled

☒ Nonsensical

Pain Assessment

Pain rating type: Non-Verbal Scale
Pain Rating: 0
New, controlled, or uncontrolled? Controlled

Nutrition

☒ Patient satisfied with intake

GI Elimination

Date of last bowel movement: 11/26/2025
Bowel Pattern Regular
Bowel sounds, # of quads: 4
☒ WNL
☒ Abdomen: Soft
☒ Incontinence

GU Elimination

Urine output: Good
Urine color: Yellow
☒ Clear
☒ Incontinent

Respiratory

☒ Clear
Describe location, inspiratory/expiratory: All lobes

Mobility

☒ Up to chair/wheelchair
☒ Poor endurance
☒ Weakness/fatigue

Skin

Skin temperature: Warm
Skin turgor: Poor

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide
Hospice caregiver name: Crystal Keeble, CNA
Present/not present Not present

• Interview With

☒ Family
☒ Facility staff

- *Follows assignment/plan of care*

☒ Yes

- *Patient's needs are being met*

☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

• Patient's linens are fresh and clean

☒ Yes

• Assignment sheet/POC reviewed

☒ Yes

• Assignment sheet/POC updated

☒ Yes

Comments: Continue current POC

Medication Utilization

Have any PRN medications been used in the last 24 hours? No

Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): The caregiver was reminded to continue monitoring bowel patterns and to report any signs of constipation or discomfort. Education provided regarding changes in cognition, emphasizing the importance of maintaining a calm, familiar environment and using simple communication. Encouraged family to report any sudden changes in confusion, behavior, or level of alertness.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

☒ Family reminded to call hospice and not 911 for any concerns, problems or changes in condition

☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

Additional Information/Comments: 91-year-old female on service with terminal diagnosis of Alzheimer's disease. No pain or distress noted currently, vitals WNL. The patient is now having normal bowel movements according to staff. She presents with increased confusion but remains oriented to self. Staff anticipates her needs. No medication refills needed this visit, reviewed the POC and when to call hospice with the caregiver and daughter.

Date: 11/26/2025

Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:12/12/2025 11:00:00 AM

Date/Time out:12/12/2025 11:30:00 AM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 97.7

• Respiratory Rate

Respiratory rate (respirations per minute): 18
☒ Room air

• Heart Rate

Type of heart rate: Regular
Radial pulse: 65

• Blood Pressure

☒ Sitting
Systolic blood pressure: 94
Diastolic blood pressure: 55

• Pedal Pulses

☒ Present
☒ Right
☒ Left

Current Mental Status

- ☒ Oriented
- ☒ Confused
- ☒ Forgetful

Current Behavioral Status

- ☒ Appropriate

Speech Status

- ☒ Garbled
- ☒ Nonsensical

Pain Assessment

Pain rating type: Non-Verbal Scale
Pain Rating: 0
New, controlled, or uncontrolled? Controlled

Nutrition

☒ Patient satisfied with intake

GI Elimination

Date of last bowel movement: 12/12/2025
Bowel Pattern Regular
Bowel sounds, # of quads: 4
☒ WNL
☒ Abdomen: Soft
☒ Incontinence

GU Elimination

Urine output: Good
☒ Incontinent

Respiratory

☒ Clear
Describe location, inspiratory/expiratory: All lobes
Oxygen saturation percentage: 92

Mobility

☒ Up to chair/wheelchair
☒ Poor endurance
☒ Weakness/fatigue

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide
Hospice caregiver name: Georgianna Gwen Steward, CNA
Present/not present Not present

• Interview With

☒ PCG

• Follows assignment/plan of care

☒ Yes

• Patient's needs are being met

☒ Yes

• Patient/family/CG satisfied with care provided

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

- *Patient's linens are fresh and clean*

☒ Yes

- *Assignment sheet/POC reviewed*

☒ Yes

- *Assignment sheet/POC updated*

☒ Yes

Comments: Continue current POC.

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): Education was reinforced regarding the importance of ongoing skin care and incontinence management, including frequent brief changes, gentle cleansing, and routine skin inspections to help prevent breakdown. Caregivers were encouraged to continue monitoring for any changes in comfort, skin integrity, elimination patterns, or overall condition. Reinforcement was given for notifying hospice promptly if any new concerns arise. Ongoing hospice support and availability were reiterated to ensure continued comfort-focused care.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Additional Information/Comments: 91-year-old female on hospice with terminal diagnosis of Alzheimer's disease. During today's visit, the patient appeared comfortable and calm, with no signs of pain or distress noted. Vital signs were within normal limits. Skin assessment revealed skin that was intact, warm, and dry, with no wounds or areas of breakdown observed. The patient remains incontinent of both bowel and bladder and continues to require assistance with personal care. There were no significant changes in the patient's condition compared to the previous visit. Supportive presence was provided throughout the assessment. No medication refills needed currently, reviewed the POC and when to call hospice with the memory care director and patient's daughter.

Date: 12/12/2025

Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

HUV NURSING VISIT NOTE

NURSE COMPREHENSIVE VISIT

Date/Time in:12/26/2025 11:31:00 AM

Date/Time out:12/26/2025 11:48:00 AM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Others present during visit:Facility staff

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 97.2
- Respiratory Rate

Respiratory rate (respirations per minute): 20

☒ Room air
- Heart Rate

Type of heart rate: Regular

Radial pulse: 73
- Blood Pressure

☒ Sitting

Systolic blood pressure: 118

Diastolic blood pressure: 74
- Pedal Pulses

☒ Diminished

☒ Right

☒ Left

Current Mental Status

- ☒ Disoriented
- ☒ Confused
- ☒ Forgetful
- ☒ Impaired judgement

Current Behavioral Status

- ☒ Cooperative
- ☒ Agitated

Speech Status

☒ Garbled

Pain Assessment

Pain rating type: Non-Verbal Scale
Pain Rating: 0

Cardiovascular

Other: Denies chest pain

Nutrition

☒ Appetite is a problem

Arm circumference (left): 22

☒ Patent

☒ Intact

GI Elimination

Date of last bowel movement: 12/25/2025
Bowel sounds, # of quads: 4

☒ Hypoactive

☒ Abdomen: Soft

☒ Constipation

GU Elimination

☒ Incontinent

Respiratory

☒ Diminished

Describe location, inspiratory/expiratory: Shallow respirations, diminished throughout all lung fields.
Oxygen saturation percentage: 95

Mobility

☒ Up to chair/wheelchair

☒ Transfer with assist

Transfer with assist x: 1

☒ Poor endurance

☒ Weakness/fatigue

Skin

Skin temperature: Warm
Skin turgor: Poor
☒ Pale

RN Supervisory Visit (1)

Hospice Caregiver Discipline

LPN

Present/not present

Not present

• Interview With☒ Facility staff**• Follows assignment/plan of care**☒ Yes**• Patient's needs are being met**☒ Yes**• Patient/family/CG satisfied with care provided**☒ Yes**• Treats patient with dignity and respect**☒ Yes**• Interacts well with patient/family/CG**☒ Yes**• Provides care in a safe manner**☒ Yes**• Arrives on time or calls if late**☒ Yes**• Competent with assigned tasks**☒ Yes**• Complies with infection control procedures**☒ Yes**• Reports changes in patient's condition to RN**☒ Yes**• Patient appears well groomed**☒ Yes**• Patient's nails are clean and trimmed**☒ Yes**• Patient's hair is cleaned**☒ Yes**• Patient's mouth is clean and odor-free**☒ Yes**• Patient is clean-shaven**☒ Yes

• *Patient's skin is moisturized*

☒ Yes

• *Patient's room is clean and tidy*

☒ Yes

• *Patient's linens are fresh and clean*

☒ Yes

• *Assignment sheet/POC reviewed*

☒ Yes

• *Assignment sheet/POC updated*

☒ Yes

RN Supervisory Visit (2)

Hospice Caregiver Discipline	Hospice Aide
Present/not present:	Not present

• *Interview With*

☒ Facility staff

• *Follows assignment/plan of care*

☒ Yes

• *Patient's needs are being met*

☒ Yes

• *Patient/family/CG satisfied with care provided*

☒ Yes

• *Treats patient with dignity and respect*

☒ Yes

• *Complies with infection control procedures*

☒ Yes

• *Reports changes in patient's condition to RN*

☒ Yes

• *Punctual: notifies office if late*

☒ Yes

• *Dresses properly*

☒ Yes

• *Interacts well with patient/family/CG*

☒ Yes

• *Provides care in a safe manner*

☒ Yes

• *Arrives on time or calls if late*

☒ Yes

• *Competent with assigned tasks*

☒ Yes

HOPE Update Visit

Reason for Visit	3. HOPE Update Visit 2 (HUV2)
At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?	0. No
Was a symptom impact screening completed?	1. Yes

Date of symptom impact screening	12/26/2025
Pain	1. Slight
Shortness of breath	0. Not at all
Anxiety	2. Moderate
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	2. Moderate
Agitation	3. Severe

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued?	0. No
Was PRN opioid initiated or continued?	0. No
Was bowel regimen initiated or continued?	2. Yes
Date bowel regimen initiated or continued	12/26/2025

Medication Utilization

Have any PRN medications been used in the last 24 hours?	No
Does patient require refill of any medication?	No

Communication

• *Communication With Physician*

Physician name: Restrepo

• *Instructed on policies and procedures related of use and disposal of controlled drugs in the home*

☒ Caregiver

• *Instructed on infection control procedures*

☒ Caregiver

☒ Visit reviewed with facility staff

Name(s) & title(s): Jennifer

- ☒ **Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition**
- ☒ **Family reminded to call hospice and not 911 for any concerns, problems or changes in condition**
- ☒ **Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition**
- ☒ **Caregiver/Family educated about HCAHPS survey process as appropriate**

Plan for next visit: X1 week

Additional Hospice Skilled Nursing Visit Note

Information/Comments: Patient Name: Howrigan, Katherine

Age: 91

Sex/Race: Caucasian female

Date of Visit: 12/26 /2026

Level of Care: Routine

Primary Diagnosis: Alzheimer's disease

Comorbidities: HTN, acute respiratory distress, anxiety, depression

Residence: Assisted living

Subjective

SN arrived to find the patient sitting in her wheelchair with a blanket on, in front of the television. Pt appears to be sleeping.

Mental status: A&O x 1.

Patient/caregiver report(s): No issues at this time. Facility staff reports she is sensitive to touch and becomes easily agitated. Pt received bath today and was agitated about that. RN expressed need for assessment to Ms. Howrigan and minimized touch with minimal agitation noted.

Pain reported: No

Objective

- Respiratory: Respirations are even and unlabored on room air; breath sounds are diminished in all lung fields, pt slept though majority of assessment with shallow respirations.

- Cardiovascular: No CP noted. HR regular rhythm, VS WNL.

- GI/GU: Abdomen soft on palpation; bowel sounds hypoactive in all four quadrants.. The patient is incontinent of bowel and bladder, however, wears briefs.

- Musculoskeletal/Functional: The patient noted with generalized weakness, wheelchair bound.

- Skin: No breakdown noted.

- Nutrition: Feeds independently. The caregiver reports the patient eats approximately 50% of 3 meals per day.

- Sleep: Sleeps approximately 12-14 hours per day, with frequent daytime napping.

Medications

- Medication list reviewed: Yes

- Refills needed: No

- Administered by: Caregiver

Assessment

Patient remains appropriate for hospice services. Decline noted as evidenced by: Ms. Hoowrigan demonstrates progressive functional decline and symptom burden related to Alzheimers and comorbid conditions. Hospice care at the routine level is appropriate for symptom management, safety, and comfort-focused interventions.

Overall status: Stable

Plan

- Continue routine SN visits.
- Monitor respiratory status
- Reinforce education on medications.
- Provide supportive measures as appropriate.
- Update IDT and physician as needed.

Caregiver Communication

Caregiver updated on patient status: Yes

Concerns voiced: None.

Caregiver agrees with POC: Yes.

Date: 12/26/2025

Electronically signed by: SHANNON COSGROVE RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

NURSE PROGRESS NOTE

NURSE PROGRESS NOTE

Date/Time in: 12/29/2025 10:07:00 PM
Date/Time out: 12/29/2025 10:08:00 PM
Purpose of progress note: IDG Narrative

IDG Hospice Recertification Skilled Nursing Note
Narrative

Patient Name: Katherine Howrigan
Age: 91
Sex/Race: Caucasian Female
Date of Visit: 12/17/2025
Admission Date: 10/15/2025
Primary Hospice Diagnosis: Senile Degeneration of the Brain
Related Comorbidities: Dementia, Osteoporosis, Osteoarthritis, Hypothyroidism, Essential Tremors, Glaucoma, Alopecia
Level of Care: Routine Hospice Care
Advance Directives: OOH DNR
POA/Decision Maker: Daughter
Allergies: NKDA

Scales This Visit

FAST: 7c from a 6e

PPS: 30%

KPS: 30%

LMAC: 18.5 cm

Subjective

SN completed a hospice recertification visit at the patient's group home. The patient's daughter provided updates regarding the patient's current status and ongoing decline. Family continues to desire comfort-focused care only, with no hospitalizations or aggressive interventions.

Over the past several months, the patient has demonstrated continued cognitive, functional, and physical decline. The daughter reports worsening weakness, poor endurance, and significantly decreased oral intake. The patient now sleeps approximately 16–18 hours per day, including frequent daytime naps.

Appetite remains poor, with intake averaging approximately 25% of meals. Meals consist of small portions that are rarely finished. No nausea or vomiting reported. Family reports increased dependence on activities of daily living and reduced engagement with surroundings.

Objective

General: Elderly female, frail and cachectic in appearance, seated in a recliner in the living room at the time of assessment. Calm, no acute distress noted.

Neurological: Alert and oriented ×1. Inconsistently follows simple commands. Speech is clear but limited. Mild essential tremors noted. The patient was changed from Lorazepam to Haloperidol due to hallucinations.

Functional Status:

ADLs: Requires maximum assistance with 5/6 ADLs (previously 4/6).

Mobility: Primarily wheelchair-bound; unable to ambulate independently.

Feeding: Able to self-feed with utensils but requires frequent prompting.

Continence: Incontinent of bowel and bladder.

Nutrition/Weight: Poor appetite with intake of approximately 25% of three small meals daily. Visible muscle wasting and ongoing weight loss were observed. LMAC remains at 18.5 cm.

Skin/Wounds: Skin clean, dry, and intact. No open areas or pressure injuries noted at this time.

Falls: Two falls were reported within the past 6 months; no injuries were sustained.

Cardiopulmonary: Lungs clear to auscultation; respirations even and unlabored on room air. Diminished breath sounds at the bilateral bases. Occasional shortness of breath with exertion.

GI/GU: Abdomen soft and non-tender; bowel sounds present in all four quadrants. Incontinent of bowel and bladder. The patient has been experiencing s/s of a UTI. She has been started on Macrobid 100 mg twice daily x 10days.

Pain: Patient denies pain at the time of the visit. Acetaminophen is available PRN for mild discomfort.

Mood/Behavior: Cooperative and calm. No agitation or anxiety observed. Patient refused blood pressure measurement; daughter reports this is baseline behavior. SN to notify primary RN to obtain wrist BP cuff for future assessments.

Assessment

91-year-old female with end-stage senile degeneration of the brain demonstrating continued and progressive decline, as evidenced by worsening functional dependence, decreased oral intake, ongoing weight loss with muscle wasting, increased sleep time, and further decline in ADLs. FAST score remains at 6e with significant nutritional compromise and poor endurance. Overall prognosis remains poor, with a life expectancy of six months or less if the disease follows its expected course.

Patient continues to meet hospice eligibility criteria.

Plan

Continued Hospice Care: Patient remains appropriate for routine hospice level of care.

Pain Management: Continue acetaminophen PRN; monitor for changes in pain or discomfort.

Nutrition: Encourage small, frequent meals and nutritional supplements as tolerated.

Skin Integrity: Continue routine skin monitoring due to immobility and incontinence.

Fall Precautions: Maintain safety measures; DME in place, including hospital bed, briefs, chux, and gloves.

Respiratory Care: Monitor for dyspnea; provide comfort measures and oxygen PRN if indicated.

Incontinence Care: Continue use of briefs; reinforce pericare to maintain skin integrity.

Psychosocial Support: Continue Social Work and Chaplain involvement to support the patient and family.

Education: Family educated on disease progression, symptom management, comfort measures, DME use, medication regimen, fall precautions, and hospice philosophy.

IDT Coordination: Findings reviewed with IDT and Dr. Restrepo. Recertification and plan of care reviewed with daughter/MPOA, who remains in agreement with hospice goals.

Scales: PPS 30% | KPS 30% | LMAC 18.5 cm

Code Status: OOHDNR

Allergies: NKDA

Date: 12/29/2025

Electronically signed by: VIRGINIA ALLAN RN ADON

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

LVN NURSE VISIT

LVN NURSE COMPREHENSIVE VISIT

Date/Time in:12/31/2025 10:03:00 AM

Date/Time out:12/31/2025 10:35:00 AM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 97

• Respiratory Rate

Respiratory rate (respirations per minute): 18
☒ Room air

• Heart Rate

Type of heart rate: Regular
Apical pulse: 81

• Blood Pressure

☒ Sitting
Systolic blood pressure: 110
Diastolic blood pressure: 67

• Pedal Pulses

☒ Present
☒ Right
☒ Left
☒ Diminished
☒ Right
☒ Left

Current Mental Status

☒ Alert
☒ Disoriented
☒ Confused

Current Behavioral Status

☒ Combative
☒ Agitated

Speech Status

- ☒ Non-verbal
- ☒ 6 or fewer words
- ☒ Garbled

Pain Assessment

Pain rating type: Non-Verbal Scale
Pain Rating: 0

Neurosensory

- ☒ Sundowner's

Nutrition

- ☒ Appetite is a problem
- Arm Circumference (right): 22
- ☒ Patent
 - ☒ Intact

GI Elimination

Date of last bowel movement: 12/30/2025
Bowel sounds, # of quads: 4

- ☒ Hypoactive
- ☒ Abdomen: Soft
- ☒ Constipation
- ☒ Incontinence

GU Elimination

- ☒ Incontinent

Respiratory

Other: Pt refused

Mobility

- ☒ Ambulatory
- Steady/unsteady gait: Steady gait

Skin

Skin temperature: Warm

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? Yes
Medication called in Docusate sodium,loratadine,levothyroxine

Pharmacy

Ascript

Communication

Additional Information/Comments: Pt was alert sitting in chair pt refused pulse ox was able to obtain bp on right lower leg, listen to lung/bowel sounds before pt became agitated.no concerns at this time per staff. Will continue care.

Date: 12/31/2025

Electronically signed by: DENISE YARBROUGH LVN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

TREATMENT ORDERS

TREATMENT ORDERS

Ordering physician: DIEGO F RESTREPO MD
Order: Admit to Hospice: Dx: Alzheimer's Disease with late onset
Patient is an OOH-DNR
Diet & Activity as tolerated
DME: Hospital Bed, Bedside Table, BSC, Wheelchair
Date ordered: 10/15/2025

Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DIEGO F RESTREPO MD

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

ADMISSION ORDERS / INITIAL PLAN OF CARE

Skilled Nursing Orders

- ☒ Admit to hospice.
- Level of care: Routine Home Care
- ☒ Assess vital signs and physiological systems as needed.
- ☒ Assess pain and symptoms and need for further intervention.
- ☒ Teach need for giving pain medications around-the-clock and potential side effects of medications.
- ☒ May give medications related to palliative care sublingually or rectally if patient is unable to swallow.
- ☒ May anchor Foley catheter and change/irrigate per hospice protocol.
- ☒ Assess bowel status with each visit.
- ☒ May check for impaction PRN.
- ☒ Oxygen at 2-4 L per nasal cannula PRN for dyspnea and/or respiratory distress.
- ☒ May check pulse oximetry for dyspnea, altered respirations, altered mental status and prior to ordering oxygen. Notify physician if reading is under 90%.
- ☒ Initiate measures to prevent/manage skin breakdown: hydrocolloid dressing, pressure-relieving mattress PRN.
- ☒ Teach and support caregiver in care of patient.
- ☒ Teach safety measures, infection control, and emergency preparedness.
- ☒ Medication orders attached.
- ☒ May place Hospice Emergency Medication Kit in patient's residence and use PRN as directed.

Nutrition Orders

- ☒ Oral diet orders
- Describe: Diet as tolerated

Activity Orders

- ☒ As tolerated

Durable Medical Equipment Orders

- ☒ Hospital bed
- ☒ Overbed table
- ☒ Oxygen

INITIAL PLAN OF CARE

Skilled Nursing Visits

- ☒ Supervisory Visits every 2 weeks if patient receives Hospice Aide services
- Weekly Visit Frequency (to be reassessed at next IDG): 1
- +PRN Weekly Visits 1
- Reason for PRN Visits Change in conditions

Social Worker Visits

- ☒ Psychosocial/Anticipatory Grief/Bereavement assessment
- ☒ Assist patient/family in planning future care
- ☒ Assist with access to community resources
- ☒ Assist with Advance Directives
- ☒ Assist with funeral planning
- ☒ Assist patient/family in obtaining financial assistance
- ☒ Educate patient/family in coping with loss/disease, grief symptoms, anticipatory grief, and available hospice services
- ☒ Evaluation for volunteer services
- Visit Frequency for next month (to be reassessed at next IDG): 1
- +PRN Visits for next month: 1

Spiritual Visits

- ☒ Spiritual needs assessment
- ☒ Phone call to patient/family's own pastor
- Visit Frequency for next month (to be reassessed at next IDG): 1
- +PRN Visits for next month: 1

Hospice Aide Visits

- ☒ Assist with personal care/hygiene and ADLs
- ☒ Light housekeeping
- ☒ Aide Assignment Sheet/Plan of Care completed
- Weekly Visit Frequency (to be reassessed at next IDG): 5

Therapy Visits

- ☒ No therapy needs identified at this time

Volunteer Visits

- ☒ Volunteer services were explained to patient/POA.

Medication Review

- ☒ Medical Director and/or Attending Physician has reviewed all the medications including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.
- ☒ All medications have been reviewed and reconciled with Attending Physician and/or Medical Director.

As the Interdisciplinary Team members, we have participated in the planning of this patient's care prior to providing care (as applicable)

Medical Director/Physician Designee: DIEGO F RESTREPO MD

Clinical Director: TRACY KING RN

Registered Nurse: VIRGINIA ALLAN RN

Social Worker: ASHLEY DAVIS LMSW

Spiritual Counselor: VERNON MORRISON

Volunteer Coordinator: VERNON MORRISON

Bereavement Coordinator: VERNON MORRISON

Date: 10/15/2025

Electronically signed by: VIRGINIA ALLAN RN

Electronically signed by: VERNON MORRISON
Electronically signed by: TRACY KING RN
Electronically signed by: DIEGO F RESTREPO MD
Electronically signed by: ASHLEY DAVIS LMSW

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Anxiety/Agitation

Measurable Goals: Patient and family/caregiver will report understanding of the causes and treatment of anxiety.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Anxiety/Agitation	1	Assess for factors contributing to family members' feelings of anxiety: Feeling unqualified in their ability to care for a very sick person, Feeling uneasy discussing death, dying, or other difficult issues, Multiple demands of work, family, patient, etc. Provide maximum symptom management to reduce or relieve anxiety.	10/15/2025 12:23 PM	TRACY KING RN
2	Anxiety/Agitation	1	Assess for factors contributing to family members' feelings of anxiety: Feeling unqualified in their ability to care for a very sick person, Feeling uneasy discussing death, dying, or other difficult issues, Multiple demands of work, family, patient, etc. Provide maximum symptom management to reduce or relieve anxiety.	11/5/2025 2:11 AM	TRACY KING RN
3	Anxiety/Agitation	1	Assess for factors contributing to family members' feelings of anxiety: Feeling unqualified in their ability to care for a very sick person, Feeling uneasy discussing death, dying, or other difficult issues, Multiple demands of work, family, patient, etc. Provide maximum symptom management to reduce or relieve anxiety.	11/18/2025 2:20 AM	TRACY KING RN
4	Anxiety/Agitation	1	Assess for factors contributing to family members' feelings of anxiety: Feeling unqualified in their ability to care for a very sick person, Feeling uneasy discussing death, dying, or other difficult issues, Multiple demands of work, family, patient, etc. Provide maximum symptom management to reduce or relieve anxiety.	12/1/2025 11:25 AM	TRACY KING RN
5	Anxiety/Agitation	1	Assess for factors contributing to family members' feelings of anxiety: Feeling unqualified in their ability to care for a very sick person, Feeling uneasy discussing death, dying, or other difficult issues, Multiple demands of work, family, patient, etc. Provide maximum symptom management to reduce or relieve anxiety.	12/13/2025 6:53 PM	TRACY KING RN
6	Anxiety/Agitation	1	Assess for factors contributing to family members' feelings of anxiety: Feeling unqualified in their ability to care for a very sick person, Feeling uneasy discussing death, dying, or other difficult issues, Multiple demands of work, family, patient, etc. Provide maximum symptom management to reduce or relieve anxiety.	12/30/2025 7:17 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN

Medical Record: #101996

CARE PLAN

Problem being care planned: Cognitive Loss/Dementia

Measurable Goals: Provide positive/enjoyable experiences for resident that do not involve overly demanding tasks and stress.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Cognitive Loss/Dementia	2	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Instruct family on Alzheimer's medications that are only effective in early to middle stages of the disease process; and discontinue these medications for end-stage (terminal phase) Alzheimer's patients. Donepezil (Aricept), Memantine (Namenda)	10/15/2025 12:25 PM	TRACY KING RN
2	Cognitive Loss/Dementia	2	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Instruct family on Alzheimer's medications that are only effective in early to middle stages of the disease process; and discontinue these medications for end-stage (terminal phase) Alzheimer's patients. Donepezil (Aricept), Memantine (Namenda)	11/5/2025 2:11 AM	TRACY KING RN
3	Cognitive Loss/Dementia	2	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Instruct family on Alzheimer's medications that are only effective in early to middle stages of the disease process; and discontinue these medications for end-stage (terminal phase) Alzheimer's patients. Donepezil (Aricept), Memantine (Namenda)	11/18/2025 2:19 AM	TRACY KING RN
4	Cognitive Loss/Dementia	2	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Instruct family on Alzheimer's medications that are only effective in early to middle stages of the disease process; and discontinue these medications for end-stage (terminal phase) Alzheimer's patients. Donepezil (Aricept), Memantine (Namenda)	12/1/2025 11:24 AM	TRACY KING RN
5	Cognitive Loss/Dementia	2	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Instruct family on Alzheimer's medications that are only effective in early to middle stages of the disease process; and discontinue these medications for end-stage (terminal phase) Alzheimer's patients. Donepezil (Aricept), Memantine (Namenda)	12/13/2025 6:53 PM	TRACY KING RN
6	Cognitive Loss/Dementia	2	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Instruct family on Alzheimer's medications that are only effective in early to middle stages of the disease process; and discontinue these medications for end-stage (terminal phase) Alzheimer's patients. Donepezil (Aricept), Memantine (Namenda)	12/30/2025 7:17 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN

Medical Record: #101996

CARE PLAN

Problem being care planned: Constipation

Measurable Goals: Patient will report soft, formed stools every one to three days.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Constipation	1	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	10/15/2025 12:26 PM	TRACY KING RN
2	Potential for Constipation	1	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	11/5/2025 2:10 AM	TRACY KING RN
3	Potential for Constipation	1	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	11/18/2025 2:18 AM	TRACY KING RN
4	Potential for Constipation	1	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	12/1/2025 11:23 AM	TRACY KING RN
5	Potential for Constipation	1	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	12/13/2025 6:52 PM	TRACY KING RN

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
6	Potential for Constipation	1	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	12/30/2025 7:16 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Depression
Measurable Goals: Patient's family will verbalize understanding of the patient's depression and interventions for its management.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents Provide for psychotherapy for clinical depression, as appropriate.	10/15/2025 12:27 PM	TRACY KING RN
2	Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents Provide for psychotherapy for clinical depression, as appropriate.	11/5/2025 2:09 AM	TRACY KING RN
3	Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents Provide for psychotherapy for clinical depression, as appropriate.	11/18/2025 2:18 AM	TRACY KING RN
4	Potential for Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents Provide for psychotherapy for clinical depression, as appropriate.	12/1/2025 11:22 AM	TRACY KING RN
5	Potential for Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents Provide for psychotherapy for clinical depression, as appropriate.	12/13/2025 6:51 PM	TRACY KING RN
6	Potential for Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents Provide for psychotherapy for clinical depression, as appropriate.	12/30/2025 7:15 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Diarrhea

Measurable Goals: Patient and family will report understanding of disease process and management of loose, frequent stools.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Instruct patient to replenish fluids to maintain hydration	10/15/2025 12:29 PM	TRACY KING RN
2	Potential for diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Instruct patient to replenish fluids to maintain	11/5/2025 2:08 AM	TRACY KING RN
3	Potential for diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Instruct patient to replenish fluids to maintain	11/18/2025 2:17 AM	TRACY KING RN
4	Potential for diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Instruct patient to replenish fluids to maintain	12/1/2025 11:20 AM	TRACY KING RN
5	Potential for diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Instruct patient to replenish fluids to maintain	12/13/2025 6:50 PM	TRACY KING RN
6	Potential for diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Instruct patient to replenish fluids to maintain	12/30/2025 7:14 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Dyspnea
Measurable Goals: Patient will report decreased feelings of breathlessness and increased comfort.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Dyspnea	1	Request/administer Morphine (drug of choice) to decrease inappropriate respiratory drive and make breathing more efficient. Request/administer a steroid for an underlying inflammatory process such as in COPD, asthma, lymphangitis, or tracheal or superior vena cava obstruction:, Dexamethasone (Decadron), Prednisone	10/15/2025 12:30 PM	TRACY KING RN
2	Dyspnea	1	Request/administer Morphine (drug of choice) to decrease inappropriate respiratory drive and make breathing more efficient. Request/administer a steroid for an underlying inflammatory process such as in COPD, asthma, lymphangitis, or tracheal or superior vena cava obstruction:, Dexamethasone (Decadron), Prednisone	11/5/2025 2:07 AM	TRACY KING RN
3	Dyspnea	1	Request/administer Morphine (drug of choice) to decrease inappropriate respiratory drive and make breathing more efficient. Request/administer a steroid for an underlying inflammatory process such as in COPD, asthma, lymphangitis, or tracheal or superior vena cava obstruction:, Dexamethasone (Decadron), Prednisone	11/18/2025 2:17 AM	TRACY KING RN
4	Potential for Dyspnea	1	Request/administer Morphine (drug of choice) to decrease inappropriate respiratory drive and make breathing more efficient. Request/administer a steroid for an underlying inflammatory process such as in COPD, asthma, lymphangitis, or tracheal or superior vena cava obstruction:, Dexamethasone (Decadron), Prednisone	12/1/2025 11:19 AM	TRACY KING RN
5	Potential for Dyspnea	1	Request/administer Morphine (drug of choice) to decrease inappropriate respiratory drive and make breathing more efficient. Request/administer a steroid for an underlying inflammatory process such as in COPD, asthma, lymphangitis, or tracheal or superior vena cava obstruction:, Dexamethasone (Decadron), Prednisone	12/13/2025 6:50 PM	TRACY KING RN
6	Potential for Dyspnea	1	Request/administer Morphine (drug of choice) to decrease inappropriate respiratory drive and make breathing more efficient. Request/administer a steroid for an underlying inflammatory process such as in COPD, asthma, lymphangitis, or tracheal or superior vena cava obstruction:, Dexamethasone (Decadron), Prednisone	12/30/2025 7:13 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Fatigue/Weakness
Measurable Goals: Patient will participate in personally identified desired activities.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Fatigue/Weakness	2	Assess possible causes of fatigue and weakness:, History of past chemotherapy or radiation therapy, Poorly managed pain, Decreased nutritional intake, Metabolic disorders, Dehydration , Rapid tumor growth, Decreased cardiac output, Reduced oxygen transport, Fear, grief, emotional exhaustion, Medications side effects, Depression, Anemia, Sleep disturbances, Infectious process Instruct patient and family/caregiver on energy conservation techniques:, Frequent rests/naps, Pacing of activities, Planning ahead for special events or visits, Arranging room for maximum ease in reaching objects	10/15/2025 12:31 PM	TRACY KING RN
2	Fatigue/Weakness	2	Assess possible causes of fatigue and weakness:, History of past chemotherapy or radiation therapy, Poorly managed pain, Decreased nutritional intake, Metabolic disorders, Dehydration , Rapid tumor growth, Decreased cardiac output, Reduced oxygen transport, Fear, grief, emotional exhaustion, Medications side effects, Depression, Anemia, Sleep disturbances, Infectious process Instruct patient and family/caregiver on energy conservation techniques:, Frequent rests/naps, Pacing of activities, Planning ahead for special events or visits, Arranging room for maximum ease in reaching objects	11/5/2025 2:06 AM	TRACY KING RN
3	Fatigue/Weakness	2	Assess possible causes of fatigue and weakness:, History of past chemotherapy or radiation therapy, Poorly managed pain, Decreased nutritional intake, Metabolic disorders, Dehydration , Rapid tumor growth, Decreased cardiac output, Reduced oxygen transport, Fear, grief, emotional exhaustion, Medications side effects, Depression, Anemia, Sleep disturbances, Infectious process Instruct patient and family/caregiver on energy conservation techniques:, Frequent rests/naps, Pacing of activities, Planning ahead for special events or visits, Arranging room for maximum ease in reaching objects	11/18/2025 2:16 AM	TRACY KING RN

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
4	Fatigue/Weakness	2	Assess possible causes of fatigue and weakness:, History of past chemotherapy or radiation therapy, Poorly managed pain, Decreased nutritional intake, Metabolic disorders, Dehydration , Rapid tumor growth, Decreased cardiac output, Reduced oxygen transport, Fear, grief, emotional exhaustion, Medications side effects, Depression, Anemia, Sleep disturbances, Infectious process Instruct patient and family/caregiver on energy conservation techniques:, Frequent rests/naps, Pacing of activities, Planning ahead for special events or visits, Arranging room for maximum ease in reaching objects	12/1/2025 11:16 AM	TRACY KING RN
5	Fatigue/Weakness	2	Assess possible causes of fatigue and weakness:, History of past chemotherapy or radiation therapy, Poorly managed pain, Decreased nutritional intake, Metabolic disorders, Dehydration , Rapid tumor growth, Decreased cardiac output, Reduced oxygen transport, Fear, grief, emotional exhaustion, Medications side effects, Depression, Anemia, Sleep disturbances, Infectious process Instruct patient and family/caregiver on energy conservation techniques:, Frequent rests/naps, Pacing of activities, Planning ahead for special events or visits, Arranging room for maximum ease in reaching objects	12/13/2025 6:49 PM	TRACY KING RN
6	Fatigue/Weakness	2	Assess possible causes of fatigue and weakness:, History of past chemotherapy or radiation therapy, Poorly managed pain, Decreased nutritional intake, Metabolic disorders, Dehydration , Rapid tumor growth, Decreased cardiac output, Reduced oxygen transport, Fear, grief, emotional exhaustion, Medications side effects, Depression, Anemia, Sleep disturbances, Infectious process Instruct patient and family/caregiver on energy conservation techniques:, Frequent rests/naps, Pacing of activities, Planning ahead for special events or visits, Arranging room for maximum ease in reaching objects	12/30/2025 7:04 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Pain

Measurable Goals: Patient will report that pain is reduced to the level personally desired: _____ on a scale of 1-10

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Pain	2	Follow the WHO analgesic ladder to initiate pharmacologic interventions: Mild pain - non-opioids and adjuvants, Moderate pain - weak opioids, non-opioids, and adjuvants, Severe pain - strong opioids, non-opioids, and adjuvants, Adjuvant therapy may include: anti-depressants, anti-convulsants, steroids, anti-spasmodics, muscle relaxants, etc. Perform a thorough pain assessment with every nursing visit to determine if pain management is meeting patient's desired goal.	10/15/2025 12:33 PM	TRACY KING RN
2	Pain	2	Follow the WHO analgesic ladder to initiate pharmacologic interventions: Mild pain - non-opioids and adjuvants, Moderate pain - weak opioids, non-opioids, and adjuvants, Severe pain - strong opioids, non-opioids, and adjuvants, Adjuvant therapy may include: anti-depressants, anti-convulsants, steroids, anti-spasmodics, muscle relaxants, etc. Perform a thorough pain assessment with every nursing visit to determine if pain management is meeting patient's desired goal.	11/5/2025 2:06 AM	TRACY KING RN
3	Pain	2	Follow the WHO analgesic ladder to initiate pharmacologic interventions: Mild pain - non-opioids and adjuvants, Moderate pain - weak opioids, non-opioids, and adjuvants, Severe pain - strong opioids, non-opioids, and adjuvants, Adjuvant therapy may include: anti-depressants, anti-convulsants, steroids, anti-spasmodics, muscle relaxants, etc. Perform a thorough pain assessment with every nursing visit to determine if pain management is meeting patient's desired goal.	11/18/2025 2:16 AM	TRACY KING RN
4	Potential for Pain	2	Follow the WHO analgesic ladder to initiate pharmacologic interventions: Mild pain - non-opioids and adjuvants, Moderate pain - weak opioids, non-opioids, and adjuvants, Severe pain - strong opioids, non-opioids, and adjuvants, Adjuvant therapy may include: anti-depressants, anti-convulsants, steroids, anti-spasmodics, muscle relaxants, etc. Perform a thorough pain assessment with every nursing visit to determine if pain management is meeting patient's desired goal.	12/1/2025 11:16 AM	TRACY KING RN
5	Potential for Pain	2	Follow the WHO analgesic ladder to initiate pharmacologic interventions: Mild pain - non-opioids and adjuvants, Moderate pain - weak opioids, non-opioids, and adjuvants, Severe pain - strong opioids, non-opioids, and adjuvants, Adjuvant therapy may include: anti-depressants, anti-convulsants, steroids, anti-spasmodics, muscle relaxants, etc. Perform a thorough pain assessment with every nursing visit to determine if pain management is meeting patient's desired goal.	12/13/2025 6:48 PM	TRACY KING RN

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
6	Potential for Pain	2	Follow the WHO analgesic ladder to initiate pharmacologic interventions: Mild pain - non-opioids and adjuvants, Moderate pain - weak opioids, non-opioids, and adjuvants, Severe pain - strong opioids, non-opioids, and adjuvants, Adjuvant therapy may include: anti-depressants, anti-convulsants, steroids, anti-spasmodics, muscle relaxants, etc. Perform a thorough pain assessment with every nursing visit to determine if pain management is meeting patient's desired goal.	12/30/2025 7:04 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Nausea/Vomiting
Measurable Goals: Vomiting will be relieved or reduced.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family/caregiver to have patient remain upright or elevate HOB for 1-2 hours after eating	10/15/2025 12:34 PM	TRACY KING RN
2	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family/caregiver to have patient remain upright or elevate HOB for 1-2 hours after eating	11/5/2025 2:05 AM	TRACY KING RN
3	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family/caregiver to have patient remain upright or elevate HOB for 1-2 hours after eating	11/18/2025 2:15 AM	TRACY KING RN
4	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family/caregiver to have patient remain upright or elevate HOB for 1-2 hours after eating	12/1/2025 11:15 AM	TRACY KING RN
5	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family/caregiver to have patient remain upright or elevate HOB for 1-2 hours after eating	12/13/2025 6:48 PM	TRACY KING RN

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
6	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family/caregiver to have patient remain upright or elevate HOB for 1-2 hours after eating	12/30/2025 7:03 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Anticipatory Grief
Measurable Goals: Patient and family will express and share grief and concerns as needed in ways meaningful to them.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	No funeral arrangements.	1	Assist with funeral and/or memorial service planning.	10/16/2025 2:41 PM	ASHLEY DAVIS LMSW
2	No funeral arrangements.	1	Assist with funeral and/or memorial service planning	12/31/2025 3:39 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

CARE PLAN

Problem being care planned: Spiritual Needs

Measurable Goals: Patient and family will continue religious and spiritual practices.
Patient and family will report connections to spiritual aspects of life.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	The patient does not currently have spiritual support due to the loss of her independence. The family does desire for chaplain services to be in place and for the chaplain to support the patient and family with the following interventions.	1	Provide ample time and opportunity for patient and family to express spiritual concerns. Use touch, silence, active listening, and presence to provide spiritual support. Assist patient and family with life review. Pray with patient and/or family as requested and as appropriate. Assist patient/family with funeral planning and/or memorial service.	10/20/2025 1:00 PM	VERNON MORRISON
2	The patient does not currently have spiritual support due to the loss of her independence. The family does desire for chaplain services to be in place and for the chaplain to support the patient and family with the following interventions.	1	Provide ample time and opportunity for patient and family to express spiritual concerns. Use touch, silence, active listening, and presence to provide spiritual support. Assist patient and family with life review. Pray with patient and/or family as requested and as appropriate. Assist patient/family with funeral planning and/or memorial service.	11/5/2025 2:04 AM	TRACY KING RN

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
3	The patient does not currently have spiritual support due to the loss of her independence. The family does desire for chaplain services to be in place and for the chaplain to support the patient and family with the following interventions.	1	Provide ample time and opportunity for patient and family to express spiritual concerns. Use touch, silence, active listening, and presence to provide spiritual support. Assist patient and family with life review. Pray with patient and/or family as requested and as appropriate. Assist patient/family with funeral planning and/or memorial service.	11/18/2025 2:15 AM	TRACY KING RN
4	The patient does not currently have spiritual support due to the loss of her independence. The family does desire for chaplain services to be in place and for the chaplain to support the patient and family with the following interventions.	1	Provide ample time and opportunity for patient and family to express spiritual concerns. Use touch, silence, active listening, and presence to provide spiritual support. Assist patient and family with life review. Pray with patient and/or family as requested and as appropriate. Assist patient/family with funeral planning and/or memorial service.	12/1/2025 11:14 AM	TRACY KING RN

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
5	The patient does not currently have spiritual support due to the loss of her independence. The family does desire for chaplain services to be in place and for the chaplain to support the patient and family with the following interventions.	1	Provide ample time and opportunity for patient and family to express spiritual concerns. Use touch, silence, active listening, and presence to provide spiritual support. Assist patient and family with life review. Pray with patient and/or family as requested and as appropriate. Assist patient/family with funeral planning and/or memorial service.	12/13/2025 6:47 PM	TRACY KING RN
6	The patient does not currently have spiritual support due to the loss of her independence. The family does desire for chaplain services to be in place and for the chaplain to support the patient and family with the following interventions.	1	Provide ample time and opportunity for patient and family to express spiritual concerns. Use touch, silence, active listening, and presence to provide spiritual support. Assist patient and family with life review. Pray with patient and/or family as requested and as appropriate. Assist patient/family with funeral planning and/or memorial service.	12/30/2025 7:02 PM	TRACY KING RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

IDG UPDATE

From date: 10/23/2025
To date: 11/6/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/15/2025		ABH GEL	1/25/1 mg/ml	Apply to inner wrist Syringe	topical	Every 4 hours as needed for anxiety	anxiety	Yes	Active
	10/15/2025		acetaminophen 32 MG/ML Oral Suspension	160 mg/5ml	Take 30ml Oral Suspension	Oral Liquid	Every 4 hours as needed for pain	Moderate pain	Yes	Active
	10/15/2025		ammonium lactate 120 MG/ML Topical Cream	12%	Apply to skin Topical Cream	Topical	After shower daily	Dry skin	Yes	Active
	10/15/2025		ascorbic acid 100	100-0.15-	Take 1	po	Daily	vitamin	No	Active

		MG / biotin 0.15 MG / calcium pantothenate 5 MG / folic acid 1 MG / niacin 20 MG / pyridoxine 10 MG / riboflavin 1.7 MG / thiamine mononitrate 1.5 MG / vitamin B12 0.006 MG Oral Capsule	5-1-20-10- 1.7-1.5- 0.006 mg	capsule Oral Capsule					
10/15/2025		calcium ascorbate 500 MG Oral Tablet	500 mg	Take 1 tablet Oral Tablet	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cholecalciferol 0.025 MG Oral Capsule	0.025 mg	Take 1 tablet Oral Capsule	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cobamamide 0.1 MG / vitamin B12 5 MG Sublingual Tablet	0.1-5 mg	Take 1 tablet Sublingual Tablet	Sublingual	Daily	Vitamin b12 deficiency	No	Active
10/15/2025		diclofenac sodium 0.01 MG/MG Topical Gel [Voltaren]	1%	Apply to affected area Topical Gel	topical	Every 12 hours	mild pain	Yes	Active
10/15/2025		dorzolamide 20 MG/ML / timolol 5 MG/ML Ophthalmic Solution [Cosopt]	2-0.5%	1 drop each eye Ophthalmic Solution	Ophthalmic	Twice daily	Dry eye	Yes	Active
10/15/2025		levothyroxine sodium 0.075 MG Oral Tablet	0.075 mg	Take 1 tablet Oral Tablet	Oral Pill	Before breakfast	Hypothyroidism	No	Active
10/15/2025		loratadine 10 MG Oral Tablet [Claritin]	10 mg	Take 1 tab;et Oral Tablet	po	Daily	antihistamine	Yes	Active
10/17/2025	10/24/2025	sulfamethoxazole 800 MG / trimethoprim 160 MG Oral Tablet [Bactrim]	800-160 mg	Take 1 tablet Oral Tablet	po	Twice daily	infection	Yes	Active
Comfort Kit Medications:									
10/15/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/15/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/15/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/15/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/15/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/15/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/15/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/15/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 10/23/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:	Problem	Start Date	Latest Update

Anxiety/Agitation	10/15/2025	10/15/2025
Cognitive Loss/Dementia	10/15/2025	10/15/2025
Constipation	10/15/2025	10/15/2025
Depression	10/15/2025	10/15/2025
Diarrhea	10/15/2025	10/15/2025
Dyspnea	10/15/2025	10/15/2025
Fatigue/Weakness	10/15/2025	10/15/2025
Pain	10/15/2025	10/15/2025
Nausea/Vomiting	10/15/2025	10/15/2025
Spiritual Needs	10/20/2025	10/20/2025

Care Plan list reflects care plan orders as of 10/23/2025

Treatment
Order List:

Name	Start Date	DC Date
No Physician Orders		

Physician orders list reflects physician orders as of 10/23/2025

IDG
Narrative:

Hospice Admission Skilled Nursing Note

Patient Name: Katherine Howrigan
Age: 91
Sex/Race: Caucasian Female
Admission Date: 10/15/2025
Primary Diagnosis: Senile Degeneration of the Brain
Comorbidities: Dementia, Osteoporosis, Osteoarthritis, Hypothyroidism, Essential Tremors, Glaucoma, Alopecia
Level of Care: Routine Hospice Care
Advance Directives: OOH DNR
POA/Decision Maker: Daughter
Allergies: NKDA

FAST: 6e

Scales:

PPS: 30%

KPS: 30%

LMAC: 18.5 cm

Subjective

SN admitted the patient to hospice services at her group home. The patient's daughter provided the patient's medical history and current health status. She expressed the family's wish to focus on comfort care and to avoid further hospitalizations.

Reports indicate that over the past three months, Ms. Howrigan has exhibited progressive cognitive, functional, and physical decline, including increased weakness, poor endurance, and decreased nutritional intake.

The daughter reports the patient sleeps approximately 16–18 hours per day with frequent daytime napping. Appetite remains poor—she consumes about 25% of her meals, consisting of small portions that she rarely finishes—no reports of nausea or vomiting.

Objective

General: Elderly female, frail and cachectic in appearance, seated in her wheelchair on the porch at the time of assessment. Appears calm, no acute distress noted.

Neurological: Alert and oriented ×1. Follows simple commands inconsistently. Speech is clear but limited. Displays mild essential tremors.

Functional Status:

ADLs: Requires maximum assistance with 5/6 ADLs (previously 4/6).

Feeding: Self-feeds with utensils but requires prompting to do so.

Continence: Incontinent of bowel and bladder.

Nutrition/Weight: Appetite poor; intake 25% of three small meals per day. Evidence of weight loss and generalized muscle

wasting was observed.

Skin/Wounds: Skin clean, dry, and intact (CDI). No open areas or pressure injuries noted.

Falls: One fall reported in the past six months; no injury sustained.

Cardiopulmonary: LSCTA; respirations even and unlabored on room air; diminished breath sounds in the lower lobes—occasional shortness of breath with exertion.

GI/GU: Abdomen soft and non-tender; bowel sounds active ×4 quadrants—incontinent of both bowel and bladder.

Musculoskeletal: Limited mobility; primarily wheelchair-bound; unable to ambulate independently.

Pain: Denies pain at the time of the visit. Tylenol is available and used PRN for mild discomfort.

Mood/Behavior: Cooperative and calm; no anxiety or agitation observed. The patient refused blood pressure measurement, which the daughter states is typical behavior. SN will notify the primary RN to obtain a wrist BP cuff for future visits.

Assessment

91-year-old Caucasian female with end-stage senile degeneration of the brain, compounded by multiple comorbidities. The patient exhibits severe cognitive and functional decline, poor nutritional intake, weight loss, and increased dependence on ADLs. Prognosis poor.

Plan

Hospice Admission: The patient was admitted at the routine level of care for comfort management and symptom control.

Pain Management: Continue Tylenol PRN for mild discomfort; reassess as needed.

Wound Care: None required at present; monitor for pressure areas due to immobility.

Nutrition: Encourage small, frequent meals and nutritional supplements as tolerated.

Fall Precautions: Hospital bed, briefs, chux, and gloves provided. Reinforce safety during transfers and repositioning.

Respiratory Care: Monitor for dyspnea; provide comfort measures and oxygen PRN if indicated.

Incontinence Care: Continue using briefs to maintain skin integrity and hygiene.

Psychosocial Support: Chaplain and Social Work services to be initiated for patient and family support.

Education: Family educated on comfort and safety measures, disease progression, symptom management, DME, hospice philosophy, skin care, comfort kit use, medication regimen, and fall precautions.

Follow-up: SN reviewed the findings with the IDT and Dr. Restrepo. Patient deemed hospice appropriate. Initial POC reviewed with the patient's daughter and MPOA—all in agreement.

? Scales upon Admission: PPS/KPS 30 | LMAC 18.5 cm | DNR | Allergies: NKDA

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee:	DIEGO F RESTREPO MD
Registered Nurse:	VIRGINIA ALLAN RN
Social Worker:	ASHLEY DAVIS LMSW
Spiritual Counselor	VERNON MORRISON
Volunteer Coordinator:	VERNON MORRISON
Bereavement Coordinator:	VERNON MORRISON

Date: 10/22/2025

Electronically signed by: DIEGO F RESTREPO MD
Electronically signed by: VERNON MORRISON
Electronically signed by: VIRGINIA ALLAN RN
Electronically signed by: ASHLEY DAVIS LMSW

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

IDG UPDATE

From date: 11/6/2025
To date: 11/20/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/15/2025		ABH GEL	1/25/1 mg/ml	Apply to inner wrist Syringe	topical	Every 4 hours as needed for anxiety	anxiety	Yes	Active
	10/15/2025		acetaminophen 32 MG/ML Oral Suspension	160 mg/5ml	Take 30ml Oral Suspension	Oral Liquid	Every 4 hours as needed for pain	Moderate pain	Yes	Active
	10/15/2025		ammonium lactate 120 MG/ML Topical Cream	12%	Apply to skin Topical Cream	Topical	After shower daily	Dry skin	Yes	Active

10/15/2025		ascorbic acid 100 MG / biotin 0.15 MG / calcium pantothenate 5 MG / folic acid 1 MG / niacin 20 MG / pyridoxine 10 MG / riboflavin 1.7 MG / thiamine mononitrate 1.5 MG / vitamin B12 0.006 MG Oral Capsule	100-0.15-5-1-20-10-1.7-1.5-0.006 mg	Take 1 capsule Oral Capsule	po	Daily	vitamin	No	Active
10/15/2025		calcium ascorbate 500 MG Oral Tablet	500 mg	Take 1 tablet Oral Tablet	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cholecalciferol 0.025 MG Oral Capsule	0.025 mg	Take 1 tablet Oral Capsule	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cobamamide 0.1 MG / vitamin B12 5 MG Sublingual Tablet	0.1-5 mg	Take 1 tablet Sublingual Tablet	Sublingual	Daily	Vitamin b12 deficiency	No	Active
10/15/2025		diclofenac sodium 0.01 MG/MG Topical Gel [Voltaren]	1%	Apply to affected area Topical Gel	topical	Every 12 hours	mild pain	Yes	Active
10/15/2025		dorzolamide 20 MG/ML / timolol 5 MG/ML Ophthalmic Solution [Cosopt]	2-0.5%	1 drop each eye Ophthalmic Solution	Ophthalmic	Twice daily	Dry eye	Yes	Active
10/15/2025		levothyroxine sodium 0.075 MG Oral Tablet	0.075 mg	Take 1 tablet Oral Tablet	Oral Pill	Before breakfast	Hypothyroidism	No	Active
10/15/2025		loratadine 10 MG Oral Tablet [Claritin]	10 mg	Take 1 tab;et Oral Tablet	po	Daily	antihistamine	Yes	Active
Comfort Kit Medications:									
10/15/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/15/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/15/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/15/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/15/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/15/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/15/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/15/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 11/6/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/15/2025	11/5/2025
Cognitive Loss/Dementia	10/15/2025	11/5/2025
Constipation	10/15/2025	11/5/2025
Depression	10/15/2025	11/5/2025

Diarrhea	10/15/2025	11/5/2025
Dyspnea	10/15/2025	11/5/2025
Fatigue/Weakness	10/15/2025	11/5/2025
Pain	10/15/2025	11/5/2025
Nausea/Vomiting	10/15/2025	11/5/2025
Spiritual Needs	10/20/2025	11/5/2025

Care Plan list reflects care plan orders as of 11/6/2025

Treatment
Order List:

Name	Start Date	DC Date
No Physician Orders		

Physician orders list reflects physician orders as of 11/6/2025

IDG
Narrative:

Continue Current POC

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

- Medical Director/Physician Designee: DIEGO F RESTREPO MD
- Registered Nurse: BETTY NYINDODO RN
- Social Worker: ASHLEY DAVIS LMSW
- Spiritual Counselor: VERNON MORRISON
- Volunteer Coordinator: VERNON MORRISON
- Bereavement Coordinator: VERNON MORRISON

Date: 11/5/2025

Electronically signed by: VERNON MORRISON
Electronically signed by: DIEGO F RESTREPO MD
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

IDG UPDATE

From date: 11/20/2025
To date: 12/4/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/15/2025		ABH GEL	1/25/1 mg/ml	Apply to inner wrist Syringe	topical	Every 4 hours as needed for anxiety	anxiety	Yes	Active
	10/15/2025		acetaminophen 32 MG/ML Oral Suspension	160 mg/5ml	Take 30ml Oral Suspension	Oral Liquid	Every 4 hours as needed for pain	Moderate pain	Yes	Active
	10/15/2025		ammonium lactate 120 MG/ML Topical Cream	12%	Apply to skin Topical Cream	Topical	After shower daily	Dry skin	Yes	Active

10/15/2025		ascorbic acid 100 MG / biotin 0.15 MG / calcium pantothenate 5 MG / folic acid 1 MG / niacin 20 MG / pyridoxine 10 MG / riboflavin 1.7 MG / thiamine mononitrate 1.5 MG / vitamin B12 0.006 MG Oral Capsule	100-0.15-5-1-20-10-1.7-1.5-0.006 mg	Take 1 capsule Oral Capsule	po	Daily	vitamin	No	Active
10/15/2025		calcium ascorbate 500 MG Oral Tablet	500 mg	Take 1 tablet Oral Tablet	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cholecalciferol 0.025 MG Oral Capsule	0.025 mg	Take 1 tablet Oral Capsule	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cobamamide 0.1 MG / vitamin B12 5 MG Sublingual Tablet	0.1-5 mg	Take 1 tablet Sublingual Tablet	Sublingual	Daily	Vitamin b12 deficiency	No	Active
10/15/2025		diclofenac sodium 0.01 MG/MG Topical Gel [Voltaren]	1%	Apply to affected area Topical Gel	topical	Every 12 hours	mild pain	Yes	Active
10/15/2025		dorzolamide 20 MG/ML / timolol 5 MG/ML Ophthalmic Solution [Cosopt]	2-0.5%	1 drop each eye Ophthalmic Solution	Ophthalmic	Twice daily	Dry eye	Yes	Active
10/15/2025		levothyroxine sodium 0.075 MG Oral Tablet	0.075 mg	Take 1 tablet Oral Tablet	Oral Pill	Before breakfast	Hypothyroidism	No	Active
10/15/2025		loratadine 10 MG Oral Tablet [Claritin]	10 mg	Take 1 tab;et Oral Tablet	po	Daily	antihistamine	Yes	Active
Comfort Kit Medications:									
10/15/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/15/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/15/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/15/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/15/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/15/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/15/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/15/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 11/20/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/15/2025	11/18/2025
Cognitive Loss/Dementia	10/15/2025	11/18/2025
Constipation	10/15/2025	11/18/2025
Depression	10/15/2025	11/18/2025

Diarrhea	10/15/2025	11/18/2025
Dyspnea	10/15/2025	11/18/2025
Fatigue/Weakness	10/15/2025	11/18/2025
Pain	10/15/2025	11/18/2025
Nausea/Vomiting	10/15/2025	11/18/2025
Spiritual Needs	10/20/2025	11/18/2025

Care Plan list reflects care plan orders as of 11/20/2025

Treatment Order List:	Name	Start Date	DC Date
	TREATMENT ORDERS - Admit to Hospice: Dx: Alzheimer's Disease with late onset Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	10/15/2025	

Physician orders list reflects physician orders as of 11/20/2025

IDG

Narrative: Continue Current POC

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

- Medical Director/Physician Designee: DIEGO F RESTREPO MD
- Registered Nurse: BETTY NYINDODO RN
- Social Worker: ASHLEY DAVIS LMSW
- Spiritual Counselor: BRETT BRATCHER M.DIV
- Volunteer Coordinator: VERNON MORRISON
- Bereavement Coordinator: VERNON MORRISON

Date: 11/19/2025

Electronically signed by: BRETT BRATCHER M.DIV
Electronically signed by: VERNON MORRISON
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: BETTY NYINDODO RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

IDG UPDATE

From date: 12/4/2025
To date: 12/18/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/15/2025		ABH GEL	1/25/1 mg/ml	Apply to inner wrist Syringe	topical	Every 4 hours as needed for anxiety	anxiety	Yes	Active
	10/15/2025		acetaminophen 32 MG/ML Oral Suspension	160 mg/5ml	Take 30ml Oral Suspension	Oral Liquid	Every 4 hours as needed for pain	Moderate pain	Yes	Active
	10/15/2025		ammonium lactate 120 MG/ML Topical Cream	12%	Apply to skin Topical Cream	Topical	After shower daily	Dry skin	Yes	Active

10/15/2025		ascorbic acid 100 MG / biotin 0.15 MG / calcium pantothenate 5 MG / folic acid 1 MG / niacin 20 MG / pyridoxine 10 MG / riboflavin 1.7 MG / thiamine mononitrate 1.5 MG / vitamin B12 0.006 MG Oral Capsule	100-0.15-5-1-20-10-1.7-1.5-0.006 mg	Take 1 capsule Oral Capsule	po	Daily	vitamin	No	Active
10/15/2025		calcium ascorbate 500 MG Oral Tablet	500 mg	Take 1 tablet Oral Tablet	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cholecalciferol 0.025 MG Oral Capsule	0.025 mg	Take 1 tablet Oral Capsule	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cobamamide 0.1 MG / vitamin B12 5 MG Sublingual Tablet	0.1-5 mg	Take 1 tablet Sublingual Tablet	Sublingual	Daily	Vitamin b12 deficiency	No	Active
10/15/2025		diclofenac sodium 0.01 MG/MG Topical Gel [Voltaren]	1%	Apply to affected area Topical Gel	topical	Every 12 hours	mild pain	Yes	Active
10/15/2025		dorzolamide 20 MG/ML / timolol 5 MG/ML Ophthalmic Solution [Cosopt]	2-0.5%	1 drop each eye Ophthalmic Solution	Ophthalmic	Twice daily	Dry eye	Yes	Active
10/15/2025		levothyroxine sodium 0.075 MG Oral Tablet	0.075 mg	Take 1 tablet Oral Tablet	Oral Pill	Before breakfast	Hypothyroidism	No	Active
10/15/2025		loratadine 10 MG Oral Tablet [Claritin]	10 mg	Take 1 tab;et Oral Tablet	po	Daily	antihistamine	Yes	Active
Comfort Kit Medications:									
10/15/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/15/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/15/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/15/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/15/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/15/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/15/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/15/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 12/4/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/15/2025	12/1/2025
Cognitive Loss/Dementia	10/15/2025	12/1/2025
Constipation	10/15/2025	12/1/2025
Depression	10/15/2025	12/1/2025

Diarrhea	10/15/2025	12/1/2025
Dyspnea	10/15/2025	12/1/2025
Fatigue/Weakness	10/15/2025	12/1/2025
Pain	10/15/2025	12/1/2025
Nausea/Vomiting	10/15/2025	12/1/2025
Spiritual Needs	10/20/2025	12/1/2025

Care Plan list reflects care plan orders as of 12/4/2025

Treatment Order List:	Name	Start Date	DC Date
	TREATMENT ORDERS - Admit to Hospice: Dx: Alzheimer's Disease with late onset Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	10/15/2025	

Physician orders list reflects physician orders as of 12/4/2025

IDG Narrative:

Provide spiritual support through socialization, scripture and prayer Continue Current POC

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

- Medical Director/Physician Designee: DIEGO F RESTREPO MD
- Registered Nurse: BETTY NYINDODO RN
- Social Worker: ASHLEY DAVIS LMSW
- Spiritual Counselor: BRETT BRATCHER M.DIV
- Volunteer Coordinator: VERNON MORRISON
- Bereavement Coordinator: VERNON MORRISON

Date: 12/3/2025

Electronically signed by: VIRGINIA ALLAN RN ADON

Electronically signed by: BRETT BRATCHER M.DIV

Electronically signed by: VERNON MORRISON

Electronically signed by: DIEGO F RESTREPO MD

Electronically signed by: BETTY NYINDODO RN

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

IDG UPDATE

From date: 12/18/2025
To date: 1/1/2026

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/15/2025		ABH GEL	1/25/1 mg/ml	Apply to inner wrist Syringe	topical	Every 4 hours as needed for anxiety	anxiety	Yes	Active
	10/15/2025		acetaminophen 32 MG/ML Oral Suspension	160 mg/5ml	Take 30ml Oral Suspension	Oral Liquid	Every 4 hours as needed for pain	Moderate pain	Yes	Active
	10/15/2025		ammonium lactate 120 MG/ML Topical Cream	12%	Apply to skin Topical Cream	Topical	After shower daily	Dry skin	Yes	Active

10/15/2025		ascorbic acid 100 MG / biotin 0.15 MG / calcium pantothenate 5 MG / folic acid 1 MG / niacin 20 MG / pyridoxine 10 MG / riboflavin 1.7 MG / thiamine mononitrate 1.5 MG / vitamin B12 0.006 MG Oral Capsule	100-0.15-5-1-20-10-1.7-1.5-0.006 mg	Take 1 capsule Oral Capsule	po	Daily	vitamin	No	Active
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10/15/2025		cholecalciferol 0.025 MG Oral Capsule	0.025 mg	Take 1 tablet Oral Capsule	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cobamamide 0.1 MG / vitamin B12 5 MG Sublingual Tablet	0.1-5 mg	Take 1 tablet Sublingual Tablet	Sublingual	Daily	Vitamin b12 deficiency	No	Active
10/15/2025		diclofenac sodium 0.01 MG/MG Topical Gel [Voltaren]	1%	Apply to affected area Topical Gel	topical	Every 12 hours	mild pain	Yes	Active
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10/15/2025		loratadine 10 MG Oral Tablet [Claritin]	10 mg	Take 1 tab;et Oral Tablet	po	Daily	antihistamine	Yes	Active
Comfort Kit Medications:									
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10/15/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/15/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/15/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/15/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/15/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/15/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/15/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 12/18/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/15/2025	12/13/2025
Cognitive Loss/Dementia	10/15/2025	12/13/2025
Constipation	10/15/2025	12/13/2025
Depression	10/15/2025	12/13/2025

Diarrhea	10/15/2025	12/13/2025
Dyspnea	10/15/2025	12/13/2025
Fatigue/Weakness	10/15/2025	12/13/2025
Pain	10/15/2025	12/13/2025
Nausea/Vomiting	10/15/2025	12/13/2025
Spiritual Needs	10/20/2025	12/13/2025

Care Plan list reflects care plan orders as of 12/18/2025

Treatment
Order List:

Name	Start Date	DC Date
TREATMENT ORDERS - Admit to Hospice: Dx: Alzheimer's Disease with late onset Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	10/15/2025	

Physician orders list reflects physician orders as of 12/18/2025

IDG
Narrative:

Provide spiritual support through socialization, scripture, and prayer.

Hospice Recertification Skilled Nursing Note

Patient Name: Katherine Howrigan
Age: 91
Sex/Race: Caucasian Female
Date of Visit: 12/17/2025
Admission Date: 10/15/2025
Primary Hospice Diagnosis: Senile Degeneration of the Brain
Related Comorbidities: Dementia, Osteoporosis, Osteoarthritis, Hypothyroidism, Essential Tremors, Glaucoma, Alopecia
Level of Care: Routine Hospice Care
Advance Directives: OOHDNR
POA/Decision Maker: Daughter
Allergies: NKDA

Scales This Visit

FAST: 7c from a 6e

PPS: 30%

KPS: 30%

LMAC: 18.5 cm

Subjective

SN completed a hospice recertification visit at the patient's group home. The patient's daughter provided updates regarding the patient's current status and ongoing decline. Family continues to desire comfort-focused care only, with no hospitalizations or aggressive interventions.

Over the past several months, the patient has demonstrated continued cognitive, functional, and physical decline. The daughter reports worsening weakness, poor endurance, and significantly decreased oral intake. The patient now sleeps approximately 16–18 hours per day, including frequent daytime naps.

Appetite remains poor, with intake averaging approximately 25% of meals. Meals consist of small portions that are rarely finished. No nausea or vomiting reported. Family reports increased dependence on activities of daily living and reduced engagement with surroundings.

Objective

General: Elderly female, frail and cachectic in appearance, seated in a recliner in the living room at the time of assessment. Calm, no acute distress noted.

Neurological: Alert and oriented x1. Inconsistently follows simple commands. Speech is clear but limited. Mild essential tremors noted. The patient was changed from Lorazepam to Haloperidol due to hallucinations.

Functional Status:

ADLs: Requires maximum assistance with 5/6 ADLs (previously 4/6).

Mobility: Primarily wheelchair-bound; unable to ambulate independently.

Feeding: Able to self-feed with utensils but requires frequent prompting.

Continence: Incontinent of bowel and bladder.

Nutrition/Weight: Poor appetite with intake of approximately 25% of three small meals daily. Visible muscle wasting and ongoing weight loss were observed. LMAC remains at 18.5 cm.

Skin/Wounds: Skin clean, dry, and intact. No open areas or pressure injuries noted at this time.

Falls: Two falls were reported within the past 6 months; no injuries were sustained.

Cardiopulmonary: Lungs clear to auscultation; respirations even and unlabored on room air. Diminished breath sounds at the bilateral bases—occasional shortness of breath with exertion.

GI/GU: Abdomen soft and non-tender; bowel sounds present in all four quadrants. Incontinent of bowel and bladder. The patient has been experiencing s/s of a UTI. She has been started on Macrobid 100 mg twice daily x 10days.

Pain: Patient denies pain at the time of the visit. Acetaminophen is available PRN for mild discomfort.

Mood/Behavior: Cooperative and calm. No agitation or anxiety observed. Patient refused blood pressure measurement; daughter reports this is baseline behavior. SN to notify primary RN to obtain wrist BP cuff for future assessments.

Assessment

91-year-old female with end-stage senile degeneration of the brain demonstrating continued and progressive decline, as evidenced by worsening functional dependence, decreased oral intake, ongoing weight loss with muscle wasting, increased sleep time, and further decline in ADLs. FAST score remains at 6e with significant nutritional compromise and poor endurance. Overall prognosis remains poor, with a life expectancy of six months or less if the disease follows its expected course.

Patient continues to meet hospice eligibility criteria.

Plan

Continued Hospice Care: Patient remains appropriate for routine hospice level of care.

Pain Management: Continue acetaminophen PRN; monitor for changes in pain or discomfort.

Nutrition: Encourage small, frequent meals and nutritional supplements as tolerated.

Skin Integrity: Continue routine skin monitoring due to immobility and incontinence.

Fall Precautions: Maintain safety measures; DME in place, including hospital bed, briefs, chux, and gloves.

Respiratory Care: Monitor for dyspnea; provide comfort measures and oxygen PRN if indicated.

Incontinence Care: Continue use of briefs; reinforce pericare to maintain skin integrity.

Psychosocial Support: Continue Social Work and Chaplain involvement to support the patient and family.

Education: Family educated on disease progression, symptom management, comfort measures, DME use, medication regimen, fall precautions, and hospice philosophy.

IDT Coordination: Findings reviewed with IDT and Dr. Restrepo. Recertification and plan of care reviewed with daughter/MPOA, who remains in agreement with hospice goals.

Scales: PPS 30% | KPS 30% | LMAC 18.5 cm
Code Status: OOHNR
Allergies: NKDA

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee:	DIEGO F RESTREPO MD
Registered Nurse:	BETTY NYINDODO RN
Social Worker:	ASHLEY DAVIS LMSW
Spiritual Counselor	BRETT BRATCHER M.DIV
Volunteer Coordinator:	VERNON MORRISON
Bereavement Coordinator:	VERNON MORRISON

Date: 12/17/2025

Electronically signed by: VERNON MORRISON
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: BRETT BRATCHER M.DIV
Electronically signed by: DIEGO F RESTREPO MD

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

IDG UPDATE

From date: 12/30/2025
To date: 1/13/2026

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

No new equipment was ordered for this IDG Period

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/15/2025		ABH GEL	1/25/1 mg/ml	Apply to inner wrist Syringe	topical	Every 4 hours as needed for anxiety	anxiety	Yes	Active
	10/15/2025		acetaminophen 32 MG/ML Oral Suspension	160 mg/5ml	Take 30ml Oral Suspension	Oral Liquid	Every 4 hours as needed for pain	Moderate pain	Yes	Active
	10/15/2025		ammonium lactate 120 MG/ML Topical Cream	12%	Apply to skin Topical Cream	Topical	After shower daily	Dry skin	Yes	Active

10/15/2025		ascorbic acid 100 MG / biotin 0.15 MG / calcium pantothenate 5 MG / folic acid 1 MG / niacin 20 MG / pyridoxine 10 MG / riboflavin 1.7 MG / thiamine mononitrate 1.5 MG / vitamin B12 0.006 MG Oral Capsule	100-0.15-5-1-20-10-1.7-1.5-0.006 mg	Take 1 capsule Oral Capsule	po	Daily	vitamin	No	Active
10/15/2025		calcium ascorbate 500 MG Oral Tablet	500 mg	Take 1 tablet Oral Tablet	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cholecalciferol 0.025 MG Oral Capsule	0.025 mg	Take 1 tablet Oral Capsule	Oral Pill	Daily	Hypocalcemia	No	Active
10/15/2025		cobamamide 0.1 MG / vitamin B12 5 MG Sublingual Tablet	0.1-5 mg	Take 1 tablet Sublingual Tablet	Sublingual	Daily	Vitamin b12 deficiency	No	Active
10/15/2025		diclofenac sodium 0.01 MG/MG Topical Gel [Voltaren]	1%	Apply to affected area Topical Gel	topical	Every 12 hours	mild pain	Yes	Active
10/15/2025		dorzolamide 20 MG/ML / timolol 5 MG/ML Ophthalmic Solution [Cosopt]	2-0.5%	1 drop each eye Ophthalmic Solution	Ophthalmic	Twice daily	Dry eye	Yes	Active
10/15/2025		levothyroxine sodium 0.075 MG Oral Tablet	0.075 mg	Take 1 tablet Oral Tablet	Oral Pill	Before breakfast	Hypothyroidism	No	Active
10/15/2025		loratadine 10 MG Oral Tablet [Claritin]	10 mg	Take 1 tab;et Oral Tablet	po	Daily	antihistamine	Yes	Active
Comfort Kit Medications:									
10/15/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/15/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/15/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/15/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/15/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/15/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/15/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/15/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 12/30/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/15/2025	12/30/2025
Cognitive Loss/Dementia	10/15/2025	12/30/2025
Constipation	10/15/2025	12/30/2025
Depression	10/15/2025	12/30/2025

Diarrhea	10/15/2025	12/30/2025
Dyspnea	10/15/2025	12/30/2025
Fatigue/Weakness	10/15/2025	12/30/2025
Pain	10/15/2025	12/30/2025
Nausea/Vomiting	10/15/2025	12/30/2025
Spiritual Needs	10/20/2025	12/30/2025

Care Plan list reflects care plan orders as of 12/30/2025

Treatment Order List:

Name	Start Date	DC Date
TREATMENT ORDERS - Admit to Hospice: Dx: Alzheimer's Disease with late onset Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	10/15/2025	

Physician orders list reflects physician orders as of 12/30/2025

IDG Narrative:

Hospice Recertification Skilled Nursing Note

Patient Name: Katherine Howrigan
Age: 91
Sex/Race: Caucasian Female
Date of Visit: 12/17/2025
Admission Date: 10/15/2025
Primary Hospice Diagnosis: Senile Degeneration of the Brain
Related Comorbidities: Dementia, Osteoporosis, Osteoarthritis, Hypothyroidism, Essential Tremors, Glaucoma, Alopecia
Level of Care: Routine Hospice Care
Advance Directives: OOH-DNR
POA/Decision Maker: Daughter
Allergies: NKDA

Scales This Visit

FAST: 7c from a 6e

PPS: 30%

KPS: 30%

LMAC: 18.5 cm

Subjective

SN completed a hospice recertification visit at the patient's group home. The patient's daughter provided updates regarding the patient's current status and ongoing decline. Family continues to desire comfort-focused care only, with no hospitalizations or aggressive interventions.

Over the past several months, the patient has demonstrated continued cognitive, functional, and physical decline. The daughter reports worsening weakness, poor endurance, and significantly decreased oral intake. The patient now sleeps approximately 16–18 hours per day, including frequent daytime naps.

Appetite remains poor, with intake averaging approximately 25% of meals. Meals consist of small portions that are rarely finished. No nausea or vomiting reported. Family reports increased dependence on activities of daily living and reduced engagement with surroundings.

Objective

General: Elderly female, frail and cachectic in appearance, seated in a recliner in the living room at the time of assessment. Calm, no acute distress noted.

Neurological: Alert and oriented x1. Inconsistently follows simple commands. Speech is clear but limited. Mild essential tremors noted. The patient was changed from Lorazepam to Haloperidol due to hallucinations.

Functional Status:

ADLs: Requires maximum assistance with 5/6 ADLs (previously 4/6).

Mobility: Primarily wheelchair-bound; unable to ambulate independently.

Feeding: Able to self-feed with utensils but requires frequent prompting.

Continence: Incontinent of bowel and bladder.

Nutrition/Weight: Poor appetite with intake of approximately 25% of three small meals daily. Visible muscle wasting and ongoing weight loss were observed. LMAC remains at 18.5 cm.

Skin/Wounds: Skin clean, dry, and intact. No open areas or pressure injuries noted at this time.

Falls: Two falls were reported within the past 6 months; no injuries were sustained.

Cardiopulmonary: Lungs clear to auscultation; respirations even and unlabored on room air. Diminished breath sounds at the bilateral bases. Occasional shortness of breath with exertion.

GI/GU: Abdomen soft and non-tender; bowel sounds present in all four quadrants. Incontinent of bowel and bladder. The patient has been experiecing s/s of a UTI. She has been started on Macrobid 100 mg twice daily x 10days.

Pain: Patient denies pain at the time of the visit. Acetaminophen is available PRN for mild discomfort.

Mood/Behavior: Cooperative and calm. No agitation or anxiety observed. Patient refused blood pressure measurement; daughter reports this is baseline behavior. SN to notify primary RN to obtain wrist BP cuff for future assessments.

Assessment

91-year-old female with end-stage senile degeneration of the brain demonstrating continued and progressive decline, as evidenced by worsening functional dependence, decreased oral intake, ongoing weight loss with muscle wasting, increased sleep time, and further decline in ADLs. FAST score remains at 6e with significant nutritional compromise and poor endurance. Overall prognosis remains poor, with a life expectancy of six months or less if the disease follows its expected course.

Patient continues to meet hospice eligibility criteria.

Plan

Continued Hospice Care: Patient remains appropriate for routine hospice level of care.

Pain Management: Continue acetaminophen PRN; monitor for changes in pain or discomfort.

Nutrition: Encourage small, frequent meals and nutritional supplements as tolerated.

Skin Integrity: Continue routine skin monitoring due to immobility and incontinence.

Fall Precautions: Maintain safety measures; DME in place, including hospital bed, briefs, chux, and gloves.

Respiratory Care: Monitor for dyspnea; provide comfort measures and oxygen PRN if indicated.

Incontinence Care: Continue use of briefs; reinforce pericare to maintain skin integrity.

Psychosocial Support: Continue Social Work and Chaplain involvement to support the patient and family.

Education: Family educated on disease progression, symptom management, comfort measures, DME use, medication regimen, fall precautions, and hospice philosophy.

IDT Coordination: Findings reviewed with IDT and Dr. Restrepo. Recertification and plan of care reviewed with daughter/MPOA, who remains in agreement with hospice goals.

Scales: PPS 30% | KPS 30% | LMAC 18.5 cm
Code Status: OOHDNR
Allergies: NKDA

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee:	DIEGO F RESTREPO MD
Registered Nurse:	BETTY NYINDODO RN
Social Worker:	ASHLEY DAVIS LMSW
Spiritual Counselor	BRETT BRATCHER M.DIV
Volunteer Coordinator:	VERNON MORRISON
Bereavement Coordinator:	VERNON MORRISON

Date: 12/30/2025

Electronically signed by: VERNON MORRISON
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: BRETT BRATCHER M.DIV

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

SOCIAL WORKER PROGRESS NOTE

SOCIAL WORKER PROGRESS NOTE

Date/Time in:	12/29/2025 10:20:00 PM
Date/Time out:	12/29/2025 10:29:00 PM
Purpose of progress note:	IDG Narrative
IDG Narrative	IDG note- No concerns identified by Social Work at this time. Patient and family coping appropriately. Continue current plan of care.

Date: 12/29/2025

Electronically signed by: ASHLEY DAVIS LMSW

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in: 10/20/2025 7:00:00 PM
Date/Time out: 10/20/2025 7:02:00 PM
Purpose of progress note: IDG Narrative

IDG Narrative This chaplain was able to complete the spiritual assessment today with Katherine at her Assisted living home in Keller Texas. Upon arrival the patient was sitting in her wheel chair finishing lunch and drinking a boost. I was able to speak with the patients daughter prior to this assessment visit and she was able to render the spiritual and life history of the patient to this chaplain due to the patients inability to communicate. The patient suffers from Alzheimer's and is not able to communicate well due to its advanced stage. The patient appears well dressed and groomed. I was able to encourage the patient to drink most of her boost but she was not interested in her lunch food. The patient grew and raised her family in Texas. She has been married and has two children, a son and a daughter. She worked with the Navy as a flite attendant and later as an executive secretary. She does practice her faith in the Catholic Church but is unable to attend services due to her advanced DX. The patients daughter told me the local Catholic church does come by the ALF and offer communion to the patient periodically. The patient was not able to life reflect with me during this visit due to her Dx. I was able to share scripture with the patient from Psalm 23. I was able to sing a few hymns to the patient during this visit and played music from my phone. I was able to talk about Gods love and care for us during this visit and pray for the patient prior to its end. This chaplain will see the patient 1xm for spiritual support.

Date: 10/20/2025

Electronically signed by: VERNON MORRISON

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

SPIRITUAL VISIT NOTE

SPIRITUAL VISIT NOTE

Date/Time in:11/14/2025 10:40:00 AM

Date/Time out:11/14/2025 11:10:00 AM

Modality:In-person visit

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Others present during the visit:Mona Jones

Patient/family/caregiver's primary concern during this visit:spiritual visit

Pain Assessment

Pain rating:0

Pain scale used:Non-Verbal Scale

Location/description of pain:The patient did not show any signs of pain during this visit.

Current Mental Status

- ☒ Alert
- ☒ Forgetful

Current Behavioral Status

- *Appropriate*

☒ Patient
- *Cooperative*

☒ Patient

Describe patient's/responsible party's mood and/or affect during visit:The patient is calm. Upon arrival the patient appeared well groomed and seated in the living room. She smiled as I introduced myself and welcomed the visit.

Describe patient's/responsible party's anticipatory grief status:The patient and family remain a minimal grief status at this time.

Describe religious rituals, sacred readings, prayer, etc:I was able to share scripture and prayer during this visit

Linkage with referrals:None needed:

Teaching Provided During Visit

Topic(s) discussed (includes topic(s) and person(s) receiving instruction):This chaplain was able to share scripture with the patient during this visit from the book of John 10 and talk able the care and provision of the Lord. She is not able to carry an intelligent conversation due to her DX. I was able to sing a few hymns to the patient during this visit and pray with her prior to this visits end.

Communication

Communication with family:I was able to communicate this visit with the family post visit.

Communication with other team members (include name, title, date, time, and topic(s) discussed):None at this time.

☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Plan for next visit:Next Month

Date: 11/14/2025

Electronically signed by: VERNON MORRISON

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in:12/3/2025 8:52:00 AM

Date/Time out:12/3/2025 8:53:00 AM

Purpose of progress note:IDG Narrative

IDG Narrative Provide spiritual support through socialization, scripture and prayer

Date: 12/3/2025

Electronically signed by: BRETT BRATCHER M.DIV

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

SPIRITUAL VISIT NOTE

SPIRITUAL VISIT NOTE

Date/Time in: 12/5/2025 1:12:00 PM
Date/Time out: 12/5/2025 1:45:00 PM
Modality: In-person visit
COVID-19 screening performed? No
Reason: no symptoms
Others present during the visit: Daughter
Patient/family/caregiver's primary concern during this visit: no concerns expressed

Pain Assessment

Pain rating: 0
Pain scale used: Verbal Scale

Current Mental Status

- ☒ Alert
- ☒ Oriented

Current Emotional Status

- Expressive
 - ☒ Patient
 - ☒ Representative

Current Behavioral Status

- Appropriate
 - ☒ Patient
 - ☒ Representative
- Cooperative
 - ☒ Patient
 - ☒ Representative

Describe patient's/responsible party's mood and/or affect during visit: both patient and family (daughter) in good mood, social.
Describe patient's/responsible party's anticipatory grief status: both deny anticipatory grief
Describe religious rituals, sacred readings, prayer, etc: Daughter reports patient is Catholic and gets support from St. Elizabeth Seton Catholic Church. Open to chaplain visits for socialization, scripture and prayer
Linkage with referrals: None needed:

Teaching Provided During Visit

Topic(s) discussed (includes topic(s) and person(s) receiving instruction): Educated patient on the role of the chaplain, support and resources available to her and the family. Patient affirms understanding verbally and is accepting of future chaplain visits for socialization, scripture and prayer.

- Method(s)
- ☒ Verbal instructions
- Outcome(s)
- ☒ Verbalized understanding

Communication

☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Plan for next visit:

Provide spiritual support through socialization, scripture and prayer

Additional Information/Comments:

The patient is a 91-year-old female residing in a group home, with a primary hospice diagnosis of G30.1 - Alzheimer's disease with late onset. The patient was received sitting in a wheelchair in the living room of the group home. Patient was alert with normal verbalization. Patient showed no indication of any uncontrolled pain or discomfort. Patient was not averse to my presence and made eye contact occasionally during the visit. Attempted to facilitate life review and the patient attempted to share about her faith background. Patient daughter was present and filled in details about early life. Patient identifies as Catholic and is involved with St. Elizabeth Seton Catholic Church. Patient is open to chaplain visits for socialization, scripture and prayer. In accordance with her Catholic faith background, provided the patient with a caring pastoral presence, spiritual comfort, affirmation of faith, and we concluded our time in prayer. The patient and daughter had a positive response to the interventions, making eye contact, and being pleasant. Patient was awake in her wheelchair as I exited the area. Will continue the current plan of care with chaplain visits 1X a month for spiritual and emotional support.

Date: 12/5/2025

Electronically signed by: BRETT BRATCHER M.DIV

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in:12/16/2025 7:00:00 PM

Date/Time out:12/16/2025 7:01:00 PM

Purpose of progress note:IDG Narrative

IDG Narrative Provide spiritual support through socialization, scripture and prayer

Date: 12/16/2025

Electronically signed by: BRETT BRATCHER M.DIV

Patient: KATHERINE HOWRIGAN
Medical Record: #101996

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in:12/30/2025 8:06:00 AM

Date/Time out:12/30/2025 8:07:00 AM

Purpose of progress note:IDG Narrative

IDG Narrative Provide spiritual support through socialization, scripture and prayer

Date: 12/30/2025

Electronically signed by: BRETT BRATCHER M.DIV